South West Peninsula Board Meeting in Public

Venue: The Board Room, Aperture House, Pynes Hill, Exeter, EX2 5AZ and via Microsoft Teams
Date: 24 September 2026
Time: 09:50 – 14:15

Agenda

The quorum for meetings of the Board is:

a) Chair or Deputy Chair
b) At least two Executive Members
c) At least two Non-Executive Members
d) At least two of either the Partner Members or the Mental Health Member

1.		Introductory Items	Lead	Action	Time
1.1	SWPB26 –033	Welcome, introduction and apologies for absence	John Govett, Chair	Discussion	09:50
1.2	SWPB26 –034	Declarations of interest	John Govett, Chair	Information	09:52
1.3	SWPB26 –035	Minutes of the previous meeting held in public on 30 July 2026	John Govett, Chair	Approval	09:53
1.4	SWPB26 –036	Updates on Action Log	Molly Marcu, Chief Governance Officer	Approval	09:55
2.		**Key items & Leadership Reports**	**Lead**	**Action**	**Time**
2.1	SWPB26 –037	**Devon:** Update on maternity services at North Devon and next steps	Hannah Pugliese, Director of Women's and Children's	Assurance	10:00 – 10:30
2.2	SWPB26 –038	**Cluster:** Overview of Amos and Ockenden Reports	Hannah Pugliese, Director of Women's and Children's	Assurance	10:30 – 10:50
2.3	SWPB26 –039	**Cluster:** Patient Journeys: Understanding the experience of maternity and neonatal services	John Govett, Chair & Lucy Tuson, Director of Experience and Engagement	Information	10:50 – 11:10

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2.4	SWPB26–040	Chair's Report	John Govett, Chair	Information	11.10 – 11.20
2.5	SWPB26–041	Chief Executive Officer's Report	Susan Bracefield, Acting Chief Executive Officer	Assurance	11:20 – 11:30
		BREAK			
2.6	SWPB26–042	Reports from Partner Members and South West Peninsula Participants a) Local Authorities b) NHS Trusts and Foundation Trusts c) Primary Medical Services d) Mental Health e) Participants	NHS Cornwall and Isles of Scilly Partner Members and Participants NHS Devon Partner Members and Participants	Assurance	11:30 – 12:00
2.7	SWPB26–043	Reports from Non-Executive Chairs of Board Assurance Committees a) Audit and Risk Committee – 07 September 2026 b) South West Peninsula People. Customer Experience and Equity Committee – 03 September 2026 c) CIOS Finance, Performance and Quality Committee – 10 September 2026 d) Devon Finance, Performance and Quality Committee – 13 August and 10 September 2026	Non-Executive Chairs	Assurance	12:00 – 12:20
3.		**For Approval and Endorsement**			
3.1	SWPB26-044	**Cluster:** Winter Plan	Susan Bracefield, Acting Chief Executive	Approval	12:20 – 12:40
		LUNCH			12:40 – 13:10
4.		**Governance and Assurance Reports**	**Lead**	**Action**	**Time**
4.1	SWPB26-045	**Cluster:** Performance Exception Report	Gary Heneage, Interim Chief Finance Officer	Assurance	13:10 – 13:30

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4.2	SWPB26 –046	**Cluster:** Board Assurance Framework	Molly Marcu, Chief Governance Officer	Assurance	13:30 – 13:50
5.		**Closing Items**	**Lead**	**Action**	**Time**
5.1	SWPB26 -047	Forward Plan Overview	Molly Marcu, Interim Chief Governance Officer	Discussion	13:50 - 14:00
5.2	SWPB26 –048	Questions from Members of the Public	John Govett, Chair	Discussion	14:00 - 14:10
5.3	SWPB26 –049	Any Other Business	John Govett, Chair	Discussion	14:10 - 14:15
		Close			**14:15**

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Members

Name	Role	Organisation
John Govett	Chair	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Mark Hackett	Chief Executive (interim)	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Anne-Marie Bond	NHS Devon: Partner Member – Local Authorities	Chief Executive Officer, Torbay Council
Susan Bracefield	Acting Chief Executive	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Alison Bulman	NHS Cornwall and Isles of Scilly: Partner Member - Local Authorities	Strategic Director for Care & Wellbeing, Cornwall Council
Graham Clarke	Non-Executive Member	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Peter Collins	Chief Population Health Officer	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Simon Gittoes-Davies	Chief Finance Officer	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Gary Heneage	Interim Chief Finance Officer	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Sam Higginson	NHS Devon: Partner Member - NHS Trusts and Foundation Trusts	Chief Executive Officer, Royal Devon University Healthcare NHS Foundation Trust
Tarn Lamb	Non-Executive Member	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Phill Mantay	NHS Devon: Mental Health Member	Chief Executive Officer, Devon Partnership NHS Trust
Kim O'Keeffe	NHS Cornwall and Isles of Scilly Partner Member – NHS Trusts and Foundation Trusts	Acting Chief Executive Officer, Royal Cornwall Hospitals NHS Trust
Dr Frank O'Kelly	NHS Devon: Partner Member – Providers of Primary Care Medical Services	GP Partner, Amicus Health
Lopa Patel	Non-Executive Member	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Smitesh Patel	NHS Cornwall and Isles of Scilly: Partner Member – Providers of Primary Care Medical Services	GP, Oak Tree & Pensilva Surgery
Chris Reid	Chief Place and Transformation Officer (Chief Medical Officer)	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Debbie Richards	NHS Cornwall and Isles of Scilly: Partner Member (mental health)	Chief Executive Officer, Cornwall Partnership NHS Foundation Trust
Libby Ryan-Davies	Chief Strategic Planning and Commissioning Officer	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Martin Sykes	Non-Executive Member	NHS Cornwall and Isles of Scilly and NHS Devon Cluster

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Neil Walden	Non-Executive Member	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Patrick Weir	Chief People Officer	NHS Cornwall and Isles of Scilly
Robert Williams	Non-Executive Member	NHS Cornwall and Isles of Scilly and NHS Devon Cluster

Participants

Name	Role	Organisation
Diana Crump	NHS Devon: Voluntary Community and Social Enterprise (VCSE) Sector representative	Chair, Torbay, Plymouth and Devon VCSE Assembly
Molly Marcu	Chief Governance Officer	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Dr Eunan O'Neill	NHS Cornwall and Isles of Scilly: Director of Public Health	Interim Director of Public Health Cornwall Council
Rachel Pearce	Interim Director Transformation	NHS Cornwall and Isles of Scilly and NHS Devon Cluster
Dr Peter Roberts	NHS Devon: GP Collaborative Board Chair	GP Partner, Redfern Health Centre
Emma Rowse	NHS Cornwall and Isles of Scilly: Voluntary Community and Social Enterprise Sector representative	NHS Cornwall and Isles of Scilly Voluntary, Community and Social Enterprise Forum
Dr Lincoln Sargeant	NHS Devon: Director of Public Health	Director of Public Health, Torbay Council
Dr Stewart Smith	NHS Cornwall and Isles of Scilly: GP Collaborative Board Chair	GP Partner, St Austell Healthcare

Observer

Name	Role	Organisation
Cllr Maria Lawson	Chair of One Devon Integrated Care Partnership	Plymouth City Council

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South West Peninsula Board Register of Interests - 24 September 2026

NHS Cornwall and Isles of Scilly NHS Devon

Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
John Govett	NHS Cornwall and Isles of Scilly and NHS Devon Cluster Chair	Nil Declaration	Active	N/A	N/A	N/A
Anne-Marie Bond	NHS Devon: Partner Member - Local Authorities	Declaration of Interest	Active	Employment by Torbay Council as Chief Executive	Non-Financial Professional	Awareness of role
				Appointment as Chief Executive of the CCA		Awareness by all of the position held
Susan Bracefield	Acting Chief Executive and Chief Clinical Officer (Chief Nursing Officer)	Nil Declaration	Active	N/A	N/A	N/A
Helen Braid	Governance & Assurance Manager	Nil Declaration	Active	N/A	N/A	N/A
Alison Bulman	NHS Cornwall and Isles of Scilly: Partner Member - Local Authorites	Declaration of Interest	Active	Strategic director for Care and Wellbeing, Cornwall Council	Financial	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice
Graham Clarke	Non-Executive Member	Declaration of Interest	Active	Non-Executive Director of Medicine Discovery Catapult	Financial	Where a piece of ICB work or an item on a ICB Committee agenda item concerns an area where this person has declared an interest, they would be allowed to input their knowledge and experience but not participate in the final decision/vote.
				Non Executive Member at the Department of Health & Social Care		
				Trustee and Treasurer of the Cornwall Community Foundation	Non-Financial Professional	
				Spouse is CEO of Cornwall Partnership Foundation Trust	Indirect	
				Spouse is a Board Member of Cornwall & Isles of Scilly ICB		
				Member, Institute of Cancer Research.	Non-Financial Professional	
				Trustee of Picker Institute, a charity that advises on the design and policy for patient engagement and workforce satisfaction analysis.		
Peter Collins	Chief Population Health Officer	Declaration of Interest	Active	Spouse is Head of Clinical (Rehabilitation and Community) for NHS Supply Chain provides quality assurance for supply chain clinical products.	Indirect	Declare interest and recuse from any direct procurement decision making in relevant areas

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Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Diana Crump	NHS Devon: Voluntary, Community and Social Enterprise (VCSE) Representative	Declaration of Interest	Active	I am a freelance VCSE mentor and consultant -Crump Consultancy through which I am commissioned by Devon Communities Together (DCT) under an MOU and funded through NHS Devon to provide 3.5 days of work as the Independent Chair of The TPD VSCE Assembly. I also undertake other freelance, self employed work to support charities and social enterprises to grow their organisations through mentoring, facilitation and other forms of support.	Financial	I will declare all conflicts in relation to any specific work that has any conflict with any topic areas discussed or on agendas to be discussed. I will ensure that all interactions in relation to this board are always strictly in relation to my role as a champion for the VCSE sector. To use integrity to ensure that I put aside any allegiances to particular organisations to champion all VCSE organisations across Devon.
Mark Hackett	Interim Cluster Chief Executive Officer and Accountable Officer	Declaration of Interest	Active	Honorary Professor Swansea University	Non-Financial Professional	To be managed in line with the Standards of Business Conduct and Conflict of Interest Policies
				Wife is a Non-Executive Director at The Robert Jones and Agnus Hunt Orthopaedic Hospital	Indirect	
				Non-executive director Birmingham , black country and Solihull ICB	Financial	
				Wife is the chair (unremunerated) of Sandwell Leisure Trust in the black country.	Indirect	
Gary Heneage	Interim Chief Finance Officer	Declaration of Interest	Active	Wife is a Consultant at Circle Hospital in Reading (non-NHS)	Indirect	Will not be party to any discussions with Circle Reading.
				Sister is a partner at DWF Solicitors		Will actively exclude myself from discussions with DWF on engagement
Sam Higginson	NHS Devon: NHS Trust and Foundation Trusts Partner Member	Declaration of Interest	Active	Chair of Enfield Unity Primary Care Network	Non-Financial Professional	To be managed in line with the Standards of Business Conduct and Conflicts of Interest Policies
				Committee Member - General Charity of the Royal Devon University Healthcare NHS Foundation Trust	Non-Financial Professional	
				Chief Executive Officer Royal Devon University Healthcare NHS Foundation Trust.	Financial	

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Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Tarn Lamb	Non Executive Member	Declaration of Interest	Active	CEO at Cornwall Neighbourhood for Change Ltd	Financial	CN4C is one of the partners for WorkWell. This is mitigated as follows:- There is a separate management group to oversee the programme- This individual has no responsibility for management of the ICB relationship with CN4C or budget/contracting decisions on CN4C funding. - CN4C's level of funding, along with its delivery and performance on WorkWell, managed through lead contractor, as part of its contracting arrangements with partner organisations. - This individual has no role in decision-making regarding which organisations the ICB will partner with on WorkWell.- The ICB Workforce committee would not be involved in operational decisions on WorkWell- Active, ongoing consideration during committee/Board meetings as to whether to step away from discussions which could impact CN4C as well as ICB.
				Director of start up CIC - employment and skills services company that will provide support to people who have previously been homeless	Non-Financial Personal	Exclude from conflict situations should they arise
				Director of High Heathercombe CIC a permaculture and residential centre.	Financial	To be managed in line with the Conflicts of Interest Policy and advice from the Governance Team.
Maria Lawson	Observer	Declaration of Interest	Active	Chair of the Plymouth Health & Wellbeing Board	Non-Financial Professional	Will declare this interest again if anything on the meeting agenda relates to the Health & Wellbeing Board.
Molly Marcu	Director of Corporate Governance	Director of Blueprint Consulting	Active	Whilst the company is active it has not been trading since I started this role in June 2026	Financial	To be managed in line with the Conflicts of Interest Policy.
Phillip Mantay Braid Helen 17/09/2026 20:08:20	NHS Devon: Mental Health Member	Declaration of Interest	Active	Employed as Chief Executive Officer of Devon Partnership Trust since June 2024	Financial	Declaration of relationship prior to offering opinion or recommendation.
			Active	Children and Family Health Devon delivered services for Devon Partnership NHS Trust of which I am Chief Executive Officer		To be managed in line with the conflict of interest policy and guidance from Corporate governance

Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Kim O'Keeffe	NHS Cornwall and Isles of Scilly: Partner Member NHS Trusts and FTs	Declaration of Interest	Active	Acting Chief Executive Officer, Royal Cornwall Hospitals NHS Trust	Financial	To be managed in line with the Conflicts of Interest Policy and Corporate Governance advice.
		Declaration of Interest	Active	Honorary Professor of Nursing, University of Plymouth	Non-Financial Professional	To be managed in line with the Conflicts of Interest Policy and Corporate Governance advice.
Francis O'Kelly	NHS Devon: Partner Member - Providers of Primary Medical Services	Declaration of Interest	Ended 16/04/2026 (to be removed 10/26)	GP Partner at Amicus Health Group. As well as providing the full range of Primary Care Services the practice is also a provider of Surgical Services (dermatology, carpal tunnel, vasectomies and minor ops) and Dermatology Services. Part of the merged practice is also dispensing.	Financial	Will be managed in line with the Standards of Business Conduct Policy and advice from governance.
			Ended 16/04/2026 (to be removed 10/26)	My GP practice (Amicus Health) merged with the Mid Devon Medical Practice on 1.10.24 - Dr. Alex Degan (Medical Director for Primary Care at NHS Devon) is a partner in Mid Devon Medical Practice.	Indirect	I have declared the business of Amicus Health Group and will declare it at the start of any meeting where a conflict arises.
			Active	Blundell's School Lead Doctor.	Financial	Will be managed in line with the Standards of Business Conduct Policy and advice from governance.
				Member of the National Armed Forces Reference Group.	Non-Financial Professional	
				Attends Tiverton High School Welfare Meeting fortnightly.	Non-Financial Professional	
				Contributor to FASD Forum.	Non-Financial Professional	
				5 adopted children, 2 with recognised learning difficulties both receiving PIP. One in supported living, the other recently moved (April 2026) to supported placement with Continuing Healthcare under DOLS.	Non-Financial Personal	
				Wife is a book-keeper for STEER Education.	Financial	
				I attend a monthly online meeting of Primary Care Partner Members, which is organised and Chaired by the NHS Confederation.	Non-Financial Professional	Will be declare on Conflict of Interests at meetings.
				Member of the SW People Board.	Non-Financial Professional	To be managed in line with the conflicts of interest policy.

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Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Francis O'Kelly (Cont'd)	NHS Devon: Partner Member - Providers of Primary Medical Services	Declaration of Interest	Active	The Partnership has set up a Limited Company with the intention of buying and running Bampton Pharmacy, which is presently renting space from the Partnership Estates.	Financial	Will be declared at all meetings where there is a potential COI.
				GP Partner at Amicus Health. As well as providing the full range of Primary Care Services the practice is also a provider of Surgical Services (dermatology, carpal tunnel, vasectomies and minor ops) and Dermatology Services. Amicus Health employs General Practitioners with a Special Interest (GPSI) in both dermatology and surgery. We have GP registrars who are doing specialist modules in the practice in both these specialties. We merged with Mid Devon Medical Practice on 1.10.2024, so we are now based on 5 sites and dispense from three of them. Dr. Alex Degan (Medical Director for Primary Care at NHS Devon) is one of my Partners.	Non-Financial Professional	To be managed in line with the Conflict of Interest Policy and guidance from the Governance Team.
				Chair of The Service User Network (SUN / CEN) Task and Finish group - for those with Complex Emotional Needs on behalf of Mental Health Trusts of the SW Peninsula		To be managed in line with the Conflict of Interest Policy and guidance from the Governance Team.
Eunan O'Neill	NHS Cornwall and Isles of Scilly: Director of Public Health, Cornwall Council	Declaration of Interest	Active	Consultant in Public Health, Cornwall Council	Non-Financial Professional	To be managed in line with the Conflicts of Interest Policy and Corporate Governance advice.
Lopa Patel Braid Helen 17/09/2026 20:08:20	Non-Executive Member	Declaration of Interest	Active	My daughter is undertaking a research-based PhD funded by AstraZeneca PLC, a supplier to the healthcare sector.	Indirect	Will seek advice from the Chair / step out of the room if there are commissioning decisions related to AstraZeneca PLC
				Currently there are 12 members of my family, including my sister, nephews and nieces who work as doctors, dentists and pharmacists in the NHS, although none currently in the NHS Devon ICB commissioning region.	Indirect	I will excuse myself from any discussions related to the NHS or NHS Devon.

Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Lopa Patel (Cont'd)	Non-Executive Member	Declaration of Interest	Active	Currently Chair of equality charity, Diversity UK which is focusing on addressing inequalities, particularly for women and ethnic minorities. As a charity, Diversity UK, may receive donations from companies, organisation or individuals to help it deliver its mission. Donations are declared as per Charity Commission guidelines.	Financial	I will excuse myself from any discussions related to the NHS or NHS Devon.
				Board Member, Enterprising Women Programme at Murray Edwards College, University of Cambridge. The programme is sponsored by a number of companies, philanthropists and private donors including AstraZeneca UK, C Hoare & Co Private Bank, Innovate UK and others. The purpose of the programme is to create spin-outs from the University of Cambridge created by female postgraduates, researchers and staff. The programme currently does not accept under-graduates but this may change in the future.	Non-Financial Professional	I will excuse myself from any discussions related to the NHS or NHS Devon.
				I have been a judge for the King's Awards for Enterprise Innovation Panel since 27 November 2017. This work is carried out with The King's Awards Office, part of the Department for Business & Trade (DBT) under a confidentiality agreement with recommendations from the innovation panel being put forward to the Prime Minister's Advisory Committee. However, I have recently been asked to promote the awards on social media with text and video clips. I have also previously been asked by DSIT/DBT & the Lord Lieutenancy to take part in online and in-person events to promote the awards.		Although I do not foresee any conflict of interest, there may be business applicants from the Devon area for whom I may have to declare an interest at panel (if they are a supplier to NHS Devon ICB) to ensure impartiality at the judging stage.

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Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Lopa Patel (Cont'd)	Non-Executive Member	Declaration of Interest	Active	KPMG sponsored Diversity UK, the charity that I chair, between 2015 and 2019. I have also worked with KPMG in my previous ARC role at the IPO.	Non-Financial Professional	Advice will be sought from the Chair of the Board & Senior Independent Director in the event of a conflict of interest.
				Chair of the Finance & Performance Committee (F&PC); Member of RemCom & Member of NomCom at The National Archives (TNA), a non-ministerial department and the official archive and publisher for the UK Government.		Discussion & Review with the Chair of the Board.
Smitesh Patel	NHS Cornwall and Isles of Scilly: Partner Member - Providers of Primary Medical Services	Declaration of Interest	Active	Chair of Clinical Reference Group for CIOS ICB - directly engaging with stakeholder/provider representatives from RCHT, CFT and ICB to direct development of Palliative and EoL Care services.	Non-Financial Professional	At the discretion of the Chair
Rachel Pearce	Interim Transformation Director Devon	Declaration of Interest	Active	Vice Chair of Gambleaware. Organisation ceased on 31/3/26 but trustee oversight to continue until end December 26 before final closedown.	Non-Financial Professional	Low risk as charity not operating and all legacy issues mitigated through closedown actions.
				Trustee on Marie Curie Board - member of both finance and performance and quality committees - 3 year tenure		Will declare where appropriate at meetings and excuse myself if necessary.
Adie Perry	Deputy Chief Executive / Chief Finance Officer	Declaration of Interest	Active	Deputy Chief Executive Officer - Cornwall Partnership NHS Foundation Trust	Financial	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice.
				Trustee at Cornwall Neighbourhoods for Change (CN4C) - Voluntary Organisation.	Non-financial personal	No direct conflict anticipated but where potential conflict identified on agenda specific items, DOI to be restated and advice sought from Chair as applicable.
Chris Reid	Chief Place and Transformation Officer (Chief Medical Officer)	Declaration of Interest	Active	Director of Horneywink Ltd - Orchard.	Financial	To be managed in line with the conflict of interest policy and governance advice
				Board member - Health Innovation Network SW	Financial	

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Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Debbie Richards	NHS Cornwall and Isles of Scilly: Partner Member - Mental Health	Declaration of Interest	Active	Chief Executive Officer - Cornwall Partnership NHS Foundation Trust	Financial	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice.
				Spouse is Independent Non-Executive Member on NHS Devon ICB Board, NHS Cornwall and Isles of Scilly ICB Board, the South West Peninsula Board and Chair of the Devon ICB Finance, Performance and Quality Committee.	Indirect	To be managed in line with Conflicts of Interests Policies (CFT & ICB) and advice to be sought from Company Secretaries and Chairs as required.
				Spouse is Independent Non-Executive Member of DHSC and a member of the DHSC Audit, Risk and Assurance Committee.		No direct conflict anticipated but where potential conflict identified on agenda specific items, DOI to be restated and advice sought from Chair as applicable
				Spouse is Trustee and Treasurer of Cornwall Community Foundation	Indirect	To be managed in line with Conflicts of Interests Policies (CFT & ICB) and advice to be sought from Company Secretaries and Chairs as required.
Peter Roberts	NHS Devon: GP Collaborative Board Chair	Declaration of Interest	Active	Chair of Devon Collaborative Board (DCB)	Non-Financial Professional	To be managed in line with the conflicts of interest policy.
				Chair of The Southern Primary Care Collaborative Board		
				GP Partner The Redfern Health Centre	Financial	
Emma Rowse	NHS Cornwall and Isles of Scilly: Voluntary, Community and Social Enterprise (VCSE) Representative	Declaration of Interest	Active	Chair of NHS England South West regional VCSE group	Non-Financial Professional	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice
				Vice-chair of Perranporth PPG		
				Assistant Non-Executive Director of Cornwall Foundation Trust	Financial	
Lincoln Sargeant	NHS Devon: Director of Public Health, Torbay Council	Nil Declaration	Active	N/A	N/A	N/A
Stewart Smith	NHS Cornwall and Isles of Scilly: GP Collaborative Board Chair	Declaration of Interest	Active	Director of St Austell Healthcare limited	Financial	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice
Martin Sykes	Non-Executive Member	Declaration of Interest	Active	NED University Hospitals Bristol and Weston NHS Foundation Trust and member of their ICB Finance, Estates and Digital Committee.	Financial	Awareness and removal from potential conflict situations should they arise.
				Daughter is on the NHSE Graduate Scheme and has been placed at RDUH in a Junior Trainee Role.	Indirect	To be managed in line with the Conflict of Interest Policy and advice from the Governance Team.

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Employee	Role	Interest Type	Active / Date Ended	Interest Description	Interest Category	Mitigating Actions
Lucy Tuson	Director of Experience and Engagement	Declaration of Interest	Active	I am listed as a company director for a business run by my partner, which is a bed and breakfast accommodation.	Non-Financial Personal	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice.
Neil Walden	Non Executive Member	Declaration of Interest	Active	Member of Penwith Integrated Care Forum - public facing group	Non-Financial Professional	To be managed in line with the Conflict of Interest Policy and Corporate Governance advice.
Patrick Weir	Chief People Officer	Nil Declaration	Active	N/A	N/A	N/A
Robert Williams	Non-Executive Member	Declaration of Interest	Active	Trustee with Devon Community Foundation – newly appointed to the Board	Non-Financial Personal	To be managed in line with the Conflicts of Interest Policy and Corporate Governance advice.
				Owner/Director of Management consulting firm (DentonWilliams ltd) that has ceased trading with effect from 28th February 2026 and in the process of closing on Companies House; Previously provided services to NHS, Government bodies, commercial sector and overseas health organisations, Whilst no longer trading, retains networking connections with other companies that may provide commercial services to the NHS.	Financial	

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board meeting in public

Temporary Suspension of Maternity Services at North Devon and next steps

Date of meeting	Date report produced
24 September 2026	16 September 2026

Author(s)		Report approved by	
Name and title:	Hannah Pugliese, Director Women's and Children's	**Name and title:**	Peter Collins, Chief Population Health Officer

Executive summary

Royal Devon University Healthcare NHS Foundation Trust (RDUH) temporarily suspended acute obstetric services at North Devon District Hospital (NDDH) on 11 August 2026 following a significant deterioration in consultant obstetric and Tier 2 medical workforce availability. The Trust concluded that it could no longer provide the level of senior medical cover required to safely deliver a 24-hour obstetric service and that continuation of births at NDDH would present an unacceptable risk to women and babies

Whilst workforce fragility within North Devon maternity services had been recognised for a number of years, NHS Devon was formally advised on 23 July 2026 that workforce availability had deteriorated to a level where activation of business continuity arrangements was likely. Following notification, NHS Devon established enhanced oversight and worked closely with RDUH, NHS England Southwest, neighbouring providers, South Western Ambulance Service NHS Foundation Trust (SWASFT) and wider system partners to ensure continuity of safe maternity care, coordinate risk management and mitigate wider system impacts

Under the temporary arrangements, community midwifery, antenatal, postnatal, scanning and paediatric services continue to be provided locally, whilst births and acute obstetric activity are being delivered through alternative maternity services, predominantly at the Royal Devon and Exeter Hospital. Extensive mitigations have been implemented, including personalised care planning, transport and

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accommodation support, financial assistance and enhanced support for vulnerable women and families.

Current evidence indicates that clinical safety has been maintained throughout the period of suspension. Since implementation of the arrangements, 93 births, 182 maternity triage attendances and 28 inductions of labour have been safely managed through alternative pathways. There have been no serious incidents attributable to the service suspension, no births before arrival, and no evidence of adverse clinical outcomes associated with the temporary closure. The most significant impacts continue to relate to patient experience, increased travel requirements and the potential exacerbation of health inequalities, particularly for rural and vulnerable populations.

The service suspension has attracted significant public, political and media scrutiny, including formal consideration by Devon County Council's Health and Adult Care Scrutiny Committee. In response, NHS Devon led a system-wide Rapid Quality Review to assess the circumstances leading to the suspension, the effectiveness of risk escalation and decision-making processes, and opportunities for future improvement. The review concluded that the decision to suspend services was clinically justified, patient-safety led and necessary in the context of an acute workforce crisis. It identified a number of system-wide learning themes focused on workforce intelligence, escalation arrangements, contingency planning, communication, equality impact assessment and organisational resilience.

RDUH continues to progress workforce recovery and service restoration plans. Although substantive staffing thresholds have improved, the most recent readiness assessment concluded that all reopening criteria had not yet been fully met, resulting in a recommendation to defer reopening until workforce, onboarding and deployment requirements can be demonstrably assured. NHS Devon will continue to maintain enhanced oversight arrangements, monitor patient safety and experience, and provide regular assurance to the Board regarding restoration, recovery and wider maternity system resilience.

Whilst restoring services safely remains the immediate priority, the North Devon service suspension has reinforced the need for a longer-term approach to maternity and neonatal sustainability across the Peninsula. NHS Devon, working with system partners, will use the lessons identified through this event to shape a future model of care that delivers safe, equitable and resilient services, supported by sustainable workforce arrangements, strengthened clinical networks and improved access for rural and coastal populations. This work will be taken forward through the Local Maternity and Neonatal System and broader transformation programme, ensuring that future services are designed around the needs of women, babies and families while maintaining the highest standards of safety and quality

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:

N/A

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What is the purpose of the report?	
ASSURANCE	☒
DECISION	☒

To which Board Assurance Framework risk does the report relate?
Safety and/or Quality Failures in Commissioned Services

What are the recommendations arising from the report?
NOTE the circumstances that led to the temporary suspension of acute obstetric services at North Devon District Hospital and the actions taken to maintain safe maternity care.
TAKE ASSURANCE that enhanced system governance, quality oversight and mitigation arrangements have been implemented and that current evidence indicates maternity services have continued to operate safely throughout the period of suspension.
NOTE the findings and key learning themes arising from the Rapid Quality Review, including opportunities to strengthen workforce resilience, escalation processes, system planning and organisational learning across Devon and Cornwall maternity services.
NOTE the recommendations provided by Devon County Council Health and Adult Care Scrutiny Committee and the arrangements in place to respond to these recommendations.
REQUEST regular assurance reports on service restoration, workforce recovery, patient experience, equality impacts, quality outcomes and wider system resilience until services are safely re-established.
ENDORSE the proposed programme of work to strengthen the long-term sustainability, quality and resilience of maternity and neonatal services across Devon and Cornwall, informed by learning from the North Devon service suspension.
SUPPORT the continued development of a Peninsula-wide strategic approach to safe, sustainable maternity and neonatal services that balances quality, access, workforce sustainability and the needs of rural and coastal communities

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Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
Devon Finance Performance and Quality Committee – 10 September 2026

What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	The temporary suspension of acute obstetric services at North Devon District Hospital means pregnant women and families may need to travel further for births and urgent maternity care, while NHS Devon and partner organisations implement support measures, monitor safety, and work towards restoring local services, with particular attention to the impact on rural communities and vulnerable residents.
Quality and Safety	The temporary suspension of acute obstetric services at North Devon District Hospital was necessary to maintain safe maternity care in response to critical workforce shortages, with associated risks to access, patient experience and system capacity being mitigated through enhanced oversight, alternative care pathways and ongoing quality monitoring.
Equality, Diversity and Inclusion	The temporary suspension of acute obstetric services may have a disproportionate impact on vulnerable, disadvantaged and rural communities due to increased travel and access requirements; however, mitigation measures have been implemented to support equitable access to maternity care and reduce health inequalities.
Finances and Use of Resources	The temporary suspension of acute obstetric services will result in additional financial and resource pressures associated with patient support, workforce recruitment and retention, increased demand on alternative providers, and enhanced system oversight arrangements required to maintain safe and sustainable maternity services.
Regulation and Legal Requirements	There are no new regulatory or legal compliance issues arising from this report; however, NHS Devon

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	and RDUH must continue to demonstrate compliance with patient safety, equality, governance and business continuity requirements throughout the temporary suspension and restoration of acute obstetric services.
Information Governance (DP and Cyber)	There are no significant Information Governance (Data Protection or Cyber Security) implications arising from this report, provided that all patient information continues to be shared securely and in accordance with existing NHS information governance policies during the temporary service arrangements.
Transformation and Innovation	The report highlights the need for transformation of maternity service models and innovative workforce approaches to improve resilience, sustainability and safe access to care in rural communities.
Environment and Climate Change	No significant environmental or climate change implications have been identified; however, temporary changes to service locations may lead to a small increase in travel-related carbon emissions for patients, families and staff.

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Temporary Suspension of Maternity Services at North Devon and next steps

Introduction and background

1. North Devon (NDDH) maternity services have experienced workforce and resilience challenges for a number of years. These have included difficulties recruiting and retaining substantive consultant obstetricians and middle-grade medical staff, significant reliance on locum staffing and the challenges associated with sustaining specialist maternity services within a geographically dispersed rural population.

2. In September 2025 NHS Devon was formally notified of growing concerns regarding workforce fragility and was informed that business continuity planning was being developed by the Royal Devon University Healthcare NHS Foundation Trust (RDUH). During the subsequent period, NHS Devon continued to receive assurance that risks were being actively managed through executive oversight, workforce mitigation plans and contingency arrangements.

3. The position changed significantly during July 2026. On 23 July 2026 RDUH formally advised NHS Devon that workforce availability had deteriorated to a point where business continuity thresholds had effectively been met and that temporary suspension of births at NDDH was increasingly likely.

Current Position

4. Under the temporary arrangements, community midwifery, antenatal services, postnatal care, scanning services, paediatric services and outpatient functions continue to be delivered locally within North Devon. Births, labour ward activity, obstetric admissions, inductions of labour, caesarean sections and acute maternity triage services are being provided through alternative birthing services, predominantly RDUH services on their Eastern Site.

5. RDUH, supported by NHS Devon and system partners, has implemented extensive arrangements to minimise disruption for women and families. These include personalised birth planning, transport and accommodation support, financial assistance schemes, enhanced support for vulnerable women, translated communications and strengthened community support mechanisms.

NHS Devon Oversight and Response

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6. Following notification on 23 July 2026, NHS Devon established enhanced system oversight arrangements to coordinate planning and risk management across the wider health and care system. Tactical and strategic meetings

were established involving maternity providers, NHS England, SWASFT, neonatal services, commissioners and senior clinical leaders.

7. NHS Devon's focus throughout this period has been to ensure continuity of safe maternity care, support receiving organisations, coordinate system-wide responses to emerging risks and maintain public confidence in the safety of services.

External Scrutiny and Public Accountability

8. The temporary suspension has generated considerable public interest and concern. Devon County Council's Health and Adult Care Scrutiny Committee convened a special meeting on 17 August 2026 to consider the circumstances surrounding the service suspension and its impact on local residents.

9. The Committee requested publication of a restoration plan, a strengthened impact assessment, a comprehensive lesson learned review and a broader assessment of fragile services and system resilience across Devon.

10. A follow up meeting is convened for 24 September 2026

Media, Public and Stakeholder Interest

11. The temporary suspension has generated significant local, regional and national media attention. Coverage has predominantly focused on the impact on women and families, workforce shortages, travel distances to alternative maternity units and wider concerns regarding the sustainability of healthcare services in rural communities.

12. National coverage has framed the North Devon situation within the wider context of workforce pressures affecting maternity services across England and the challenges associated with maintaining specialist services within rural communities. Public attention increased following reports concerning the resignation of a senior Trust medical leader and ongoing debate about workforce resilience and future restoration of services.

Impact of Service Suspension

13. The temporary suspension of birth services at NDDH Maternity Unit has resulted in increased travel requirements for women, birthing people and families accessing intrapartum care, with associated impacts on anxiety, family logistics, childcare arrangements and travel costs. Feedback from service users highlights concerns regarding access to emergency care, longer journey times, communication and uncertainty regarding the future restoration of local birth services. These impacts are most significant for families living in rural areas, those on lower incomes, and those with limited access to transport.

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14. System mitigations, including additional maternity capacity at Royal Devon & Exeter Hospital (RD&E), funded transport and accommodation support, enhanced midwifery contact and ongoing stakeholder engagement, have enabled services to continue safely during the period of suspension. Monitoring and oversight arrangements remain in place through joint NHS Devon and RDUH governance processes.

15. Current Situation Report (SITREP): Since the suspension commenced on 11 August 2026, 93 births, 182 maternity triage attendances and 28 inductions of labour for women originally booked in NDDH have been managed at RD&E. There have been no serious incidents attributable to the service suspension, no births before arrival at hospital, and ambulance activity has remained within expected levels. Whilst periods of operational pressure and delays have been experienced, particularly during episodes of high regional demand, there is currently no evidence of adverse clinical outcomes associated with the temporary closure. Feedback continues to indicate increased anxiety and concerns about travel and access to care; however, women and families consistently report positive experiences of the care and support received from maternity staff.

16. Overall impact assessment: Current evidence indicates that clinical safety has been maintained through the implemented mitigations. The most significant impacts continue to relate to patient experience, increased travel burden and potential health inequalities, all of which are being actively monitored through ongoing evaluation, patient feedback and system oversight arrangements.

Rapid Quality Review and Lessons Learned

17. Recognising the significance of the temporary service change, NHS Devon led a system-wide Rapid Quality Review in accordance with National Quality Board guidance on 2nd September. The review will provide a structured opportunity to understand the circumstances leading to the suspension, assess the effectiveness of risk escalation and decision-making processes and identify lessons capable of strengthening future resilience.

18. The Review concluded that the decision was clinically led, justified on patient safety grounds, and driven by an acute workforce crisis against a backdrop of longstanding workforce fragility. The review found that Royal Devon University Healthcare NHS Foundation Trust had implemented extensive recruitment and mitigation measures over several years, but a combination of significant consultant and Tier 2 workforce losses, together with unexpected short-notice staffing changes, triggered Business Continuity Plan (BCP) thresholds and left RDUH unable to guarantee safe obstetric cover. System partners accepted that whilst there may have been opportunities to activate contingency measures earlier, there was no evidence that earlier intervention would have prevented the suspension. No organisation challenged the safety rationale for the decision, and there were no reported clinical incidents directly attributable to the service change

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19. The review identified a number of system-wide learning themes to strengthen future maternity service resilience. These include improving visibility of workforce risks across Devon and Cornwall maternity providers, strengthening mutual aid escalation processes, revising BCP triggers to better reflect Tier 2 workforce deterioration, enhancing staff psychological support and restorative supervision arrangements, and ensuring wider system engagement in future contingency planning exercises. The review also highlighted the need to strengthen communications planning, improve understanding and application of Equality Impact Assessments, consider neonatal pathway implications, and increase transparency between providers regarding emerging risks.

20. A follow-up review meeting has been scheduled to assess further learning, patient experience and the effectiveness of actions taken to improve the safety, quality and resilience of maternity services across the Peninsula

Recovery and Restoration

21. Following the temporary suspension of birthing services at NDDH on 11 August 2026 due to significant medical workforce pressures, RDUH implemented a formal programme of recruitment, workforce stabilisation and readiness assurance to support the safe restoration of services. The RDUH Board considered the recovery plan at an extraordinary meeting on 1 September 2026 and approved a proposed reopening date of 29 September 2026, subject to a number of clearly defined workforce and governance criteria being met. These included fully staffed consultant and Tier 2 rotas, a minimum substantive staffing threshold of 75% within both rotas, completion of induction and competency requirements for new staff, and a formal readiness assessment prior to reopening.

22. A robust governance process was established to oversee the reopening decision. This included ongoing workforce assurance, review of recruitment and onboarding progress, scrutiny of the sustainability and resilience of proposed rotas, and a formal Readiness Assessment undertaken through the Strategic Command Maternity Group on 14 September 2026. The assessment considered staffing establishment, deployability of newly recruited staff, induction and competency completion, contingency arrangements, and the impact of any material changes since Board approval. Particular attention was given to a significant staffing change arising on 4 September 2026 when a consultant locum supporting the proposed rota withdrew from the arrangement, requiring reassessment of the workforce model against the agreed criteria.

23. The decision-making framework is based on Board-approved criteria that required both consultant and Tier 2 medical rotas to be fully staffed on an 8/8 basis, with at least 75% substantive staffing within each rota and assurance that all staff are appropriately onboarded, assessed and deployable for independent clinical practice. The framework also requires confirmation that rota arrangements are sustainable, operationally deliverable, and supported by appropriate contingency arrangements.

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24. The readiness assessment concluded that, whilst substantive staffing thresholds had been achieved, several mandatory criteria for reopening could not yet be demonstrated as fully met. At the time of assessment, consultant staffing stood at 7 of 8 posts, with only six staff confirmed as fully deployable, and the Tier 2 workforce stood at 7 of 8 posts, with five staff confirmed as fully deployable. In addition, induction and competency processes remained incomplete for a number of newly appointed staff. Based on these findings, the Strategic Command Maternity Group recommended that reopening on 29 September 2026 should be deferred until the outstanding workforce, onboarding and deployment requirements had been satisfactorily completed and evidenced.

25. RDUH has continued to progress recruitment, onboarding and workforce mitigation actions, including agency recruitment, substantive appointments, cross-site workforce support options and competency assessment processes. The agreed next steps are for the Trust to continue enhanced oversight of consultant and Tier 2 staffing arrangements, maintain executive-level governance and risk monitoring, complete outstanding onboarding and competency requirements, and provide a further readiness assessment once all criteria have been met. Any future decision to reopen services will remain subject to formal governance processes. This includes a formal fortnightly readiness review against the agreed criteria to ensure that safe and sustainable staffing arrangements are in place.

Commissioning and Delivery of Safe Sustainable Services for the Future

26. The vision and commissioning intention for Maternity and Neonatal services across the peninsular are drafted and will be subject to is that engagement and consultation.

27. The Local Maternity and Neonatal System vision is that every woman and birthing person, baby and family receive safe, equitable, personalised and high-quality maternity and neonatal care, reducing maternal and neonatal deaths and injuries and reducing unwarranted variation in outcomes and experience and providing infants with the best start in life.

28. To achieve this NHS Cornwall and Isles of Scilly and NHS Devon cluster has set out three draft commissioning intentions for maternity and neonatal care.

29. Priority 1: Improving Safety, Quality and Clinical Outcomes
    - Secure the delivery of National Maternity and Neonatal Safety Programme
    - Implement the Maternal Care Bundle
    - Implement Core Safety Standards
    - Embed Service User Voice within Quality Improvement, Quality Surveillance and Assurance

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30. Priority 1 informs the core business of the LMNS Board to drive up standards, implement learning, reconfigure pathways and ensure quality surveillance for our system

31. Priority 2: Reduce Inequalities through family-centred, integrated neighbourhood-based care
    - Reduce inequalities through targeting enhanced wrap around support to those vulnerable to poor pregnancy outcomes
    - Deliver consistent evidence-based perinatal education and personalised family care, through the roll out of “pregnancy circles” and personalised care plans
    - Ensure pre-and post-natal care that delivers the best start in like and supports prevention of poor health outcomes
    - Strengthen the perinatal mental health and emotional wellbeing, removing local variation of offer, reducing maternal deaths and supporting parent-infant bonding

32. Priority 2 sits within the place based and neighbourhood developments

33. Priority 3: Deliver Safe, Sustainable Intrapartum and Neonatal Care
    - Develop Robust Intrapartum Care Pathways, implementing maternity triage standards and safe transfers between services
    - Consolidate and sustain birthing pathways and settings: balancing choice for home births, midwifery led centres and consultant-led units that secure services in right place and right time.
    - Strengthen Neonatal Services, with the right regional pattern of integrated family care and transitional care in maternity wards alongside the right therapeutic support to mitigate poor developmental outcomes
    - Secure sustainable services through robust peninsula wide workforce planning

34. This priority is a key focus of the Devon wide Transformation Programme, to support long term sustainability of services Devon, learning from recent events in North Devon.

Next Steps

35. Subject to consideration by the Board, NHS Devon will continue to maintain enhanced oversight arrangements for the temporary service change through existing quality, performance and recovery governance structures. The immediate actions will focus on ensuring the continued safe delivery of maternity services for North Devon residents, monitoring the impact of the temporary arrangements on women, families and staff, and providing ongoing assurance regarding patient safety, quality and experience.

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36. NHS Devon will oversee completion of the Rapid Quality and Lessons Learned Review, ensuring that findings are independently considered and translated into clear recommendations for service improvement, risk management and system resilience. The outputs of the review will be reported through the appropriate

governance committees and shared with system partners to support wider organisational learning.

37. RDUH will continue implementation of its maternity recovery programme, including consultant recruitment, workforce stabilisation measures and delivery of actions required to meet agreed restoration criteria. Progress against workforce and service restoration milestones will be monitored through dedicated programme governance arrangements and reported regularly to NHS Devon.

38. NHS Devon, working alongside RDUH, NHS England and system partners, will develop and maintain a restoration framework setting out the requirements, milestones, dependencies, risks and mitigations necessary to support the safe reintroduction of acute obstetric services at NDDH. This framework will be regularly reviewed and updated to reflect progress and emerging risks.

39. The Equality and Quality Impact Assessment will be kept under review throughout the period of service suspension. Particular attention will continue to be given to the impact on vulnerable groups, women experiencing health inequalities, those with protected characteristics and families affected by increased travel requirements.

40. NHS Devon will provide regular assurance reports to the Finance, Performance and Quality Committee and Board regarding service performance, quality outcomes, patient experience, workforce recovery, implementation of review recommendations and progress towards restoration. These reports will also include updates on actions arising from the recommendations of the Devon County Council Health and Adult Care Scrutiny Committee.

41. System partners will continue to work collaboratively to assess wider service resilience risks across Devon, ensuring that lessons identified through the North Devon maternity service suspension inform future planning, escalation arrangements, workforce strategies and business continuity processes across critical services.

42. In parallel with restoration planning, NHS Devon will work with regional and national partners to consider the longer-term sustainability of maternity and perinatal services, recognising the specific challenges of maintaining safe, resilient and high-quality services in rural and coastal areas with lower birth volumes, dispersed populations and ongoing workforce pressures.

43. The Peninsula NHS will adopt a phased approach that builds on the progress made in safety, quality improvement and population health, while focusing immediate attention on the long-term sustainability of intrapartum and neonatal services across Devon and Cornwall.

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44. The LMNS Board will continue to oversee delivery of Priority 1 (Improving Safety, Quality and Clinical Outcomes) through established quality surveillance, implementation of national safety programmes, and strengthening service user involvement in quality improvement and assurance processes. Delivery of

Priority 2 (Reducing Inequalities through Family-Centred, Integrated Neighbourhood-Based Care) will be progressed through place-based partnerships, supporting targeted interventions for vulnerable families, consistent perinatal education, personalised care planning, and strengthened perinatal mental health provision.

45. For Priority 3 (Deliver Safe, Sustainable Intrapartum and Neonatal Care). The NHS will work together with partners, clinicians, staff, service users and wider community groups to develop options that deliver sustainable, high-quality services, strengthen intrapartum and neonatal pathways, improve resilience across the peninsula, and support safe workforce models. This programme of work will be informed by learning from recent service challenges in North Devon, population need, clinical evidence, workforce availability and the requirement to maintain safe services for women, babies and families now and into the future.

Conclusion

46. Throughout this period, the priority of NHS Devon, RDUH and system partners has been to maintain safe, clinically appropriate maternity care for women and families whilst supporting staff and managing the wider system impacts. The experience has highlighted the importance of robust workforce planning, effective risk escalation, strong partnership working and transparent communication with patients, the public and stakeholders. Ongoing recovery, review and assurance activity will support the safe restoration of services and contribute to strengthening the resilience and sustainability of maternity and other critical services across Devon.

Board Recommendations

47. The South West Peninsula Board is asked to:

- **Note** the circumstances that led to the temporary suspension of acute obstetric services at North Devon District Hospital and the actions taken to maintain safe maternity care.
- **Take assurance** that enhanced system governance, quality oversight and mitigation arrangements have been implemented and that current evidence indicates maternity services have continued to operate safely throughout the period of suspension.
- **Note** the findings and key learning themes arising from the Rapid Quality Review, including opportunities to strengthen workforce resilience, escalation processes, system planning and organisational learning across Devon and Cornwall maternity services.
- **Note** the recommendations provided by Devon County Council Health and Adult Care Scrutiny Committee and the arrangements in place to respond to these recommendations.

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- **Request** regular assurance reports on service restoration, workforce recovery, patient experience, equality impacts, quality outcomes and wider system resilience until services are safely re-established.
- **Endorse** the proposed programme of work to strengthen the long-term sustainability, quality and resilience of maternity and neonatal services across Devon and Cornwall, informed by learning from the North Devon service suspension.
- **Support** the continued development of a Peninsula-wide strategic approach to safe, sustainable maternity and neonatal services that balances quality, access, workforce sustainability and the needs of rural and coastal communities

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Appendices

Appendix 1: Chronology of events and ICB involvement.

Appendix 2: Equality and population impact summary and mitigations.

Appendix 3: Rapid Quality Review

Appendix 4: Temporary service model and patient pathway changes.

Appendix 5: Media, stakeholder and public interest summary.

Appendix 6: Scrutiny Committee recommendations

Appendix 7: Fragile Services Paper

Appendix 8: Restoration

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board meeting in public

Overview of Amos and Ockenden Reports and Summary of ICB Quality Matrix

Date of meeting	Date report produced
24 September 2026	21 July 2026

Author(s)		Report approved by	
Name and title:	Hannah Pugliese, Director of Women's and Children's	**Name and title:**	Peter Collins, Chief Population Health Officer

Executive summary
The 2026 Amos Independent Investigation into Maternity and Neonatal Services in England and the Ockenden Review of Nottingham University Hospitals (NUH) conclude that despite multiple national reviews over the last decade, many of the same safety, quality and cultural issues persist across maternity services. Common themes include failure to listen to women and families, poor escalation of concerns, inequitable outcomes for ethnic minority women and disadvantaged communities, fragmented governance, workforce pressures, and inadequate organisational learning following incidents. The Amos report characterises these issues as system-wide failures requiring coordinated action from Government, NHS England, ICBs and provider trusts. Ockenden demonstrates how failures in governance, culture and leadership can lead to avoidable deaths and harm when concerns are not acted upon. NHS England has responded with a national 10-point action plan focused on safety, governance, workforce, inequalities, triage, listening to women and accountability. All systems are expected to demonstrate rapid and sustained improvement.

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
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What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

To which Board Assurance Framework risk does the report relate?

Safety failures in commissioned services.

What are the recommendations arising from the report?

NOTE the Amos and Ockenden reports and view them not as isolated provider issues but as a call for strengthened system leadership, transparency, safety culture and accountability, with a particular focus on listening to women, addressing inequalities and ensuring that learning translates into demonstrable improvement.

APPROVE the plan for next steps and receive analysis of the systems position following the national return in October 2026

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?

- The publication of the Nottingham and Amos reviews, and the associated immediate actions, were considered by the Peninsula Local Maternity and Neonatal System (LMNS) Board on 27 July 2026. The Board noted progress against regional and provider assurance requirements and supported the management of implementation through existing Peninsula LMNS governance, quality improvement and assurance arrangements. Members endorsed the use of established structures, including the Delivery and Improvement Group and the Quality, Safety and Learning Collaborative, to oversee delivery and assurance activity, avoiding duplication and ensuring alignment with existing system priorities.
- Reference to the reports was included in the Chief Executive Officer's report to the South West Peninsula Board in July 2026.
- South West Peninsula Management Group – via correspondence on 03 August 2026
- Devon Finance, Performance and Quality Committee – 13 August 2026
- Cornwall and Isles of Scilly Finance, Performance and Quality Committee – 10 September 2026

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What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	The reports highlight the importance of listening to women and families, improving patient experience, and strengthening engagement with Maternity and Neonatal Voices Partnerships (MNVPs). There are implications for public confidence in maternity services and for improving outcomes for women, babies and families across Devon, Cornwall and the Isles of Scilly.
Quality and Safety	Significant implications. The reports identify persistent concerns relating to patient safety, clinical governance, incident learning, maternity triage, workforce pressures and escalation processes. The proposed actions are intended to strengthen safety, quality oversight and continuous improvement across maternity and neonatal services.
Equality, Diversity and Inclusion	Significant implications. Both reports highlight inequalities in maternal and neonatal outcomes, particularly for women from Black and ethnic minority communities, disadvantaged groups, and vulnerable families. The system is expected to prioritise reducing inequalities and ensuring equitable access, experience and outcomes.
Finances and Use of Resources	No additional funding or dedicated resources have been identified to support implementation of the recommendations. Delivery will be undertaken within existing resources, which may impact organisational capacity and place additional demands on provider and system teams.
Regulation and Legal Requirements	The reports create additional assurance and oversight expectations for provider trusts, ICBs and system partners. Compliance with NHS England requirements, implementation of national actions and adherence to statutory quality and safety duties will require ongoing monitoring.
Information Governance (DP and Cyber)	No direct information governance or cyber security implications are identified within this report. Any data used for assurance, performance monitoring or inequalities analysis must continue to be managed in accordance with existing information governance requirements.

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Transformation and Innovation	The reports support transformation of maternity and neonatal services through strengthened governance, improved learning systems, enhanced use of patient experience, greater focus on inequalities, and the implementation of new national standards and improvement approaches.
Environment and Climate Change	No direct environmental or climate change implications are identified within this report.

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Overview of Amos and Ockenden Reports and Summary of ICB Quality Matrix

Key Findings from Amos and Ockenden

Women and families are not consistently listened to

1. Both reviews identify repeated examples where concerns raised by women and families were dismissed, contributing to avoidable harm and loss. Listening to women is described as a core patient safety issue rather than solely an experience measure.

Persistent inequalities

2. Black women remain significantly more likely to die during pregnancy and childbirth, while outcomes for babies from Black and Asian communities remain poorer than for White populations. The reports conclude that racism, discrimination and inequalities are maternity safety issues requiring systematic intervention.

Governance and accountability remain weak

3. The Nottingham review found longstanding failures in leadership, board scrutiny, governance and incident management. Amos identifies fragmented national accountability and unclear responsibilities across organisations

Learning from incidents is insufficient

4. Families often experience lengthy, defensive investigations and limited evidence that lessons are implemented. Reviews frequently identify the same themes without sustained improvement.

Workforce and culture are major contributors

5. Staff report workforce shortages, inadequate training opportunities, bullying cultures, poor psychological safety and fear of speaking up. These factors directly affect patient safety.

Maternity triage requires urgent improvement

6. Both reviews highlight maternity triage as a critical point of risk where concerns are often missed or delayed. National standards and board-level oversight are now required.

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Key Findings from Amos and Ockenden

Area	Provider Trusts	ICBs	NHS England
Safe delivery of maternity and neonatal care	A/R	C	I
Clinical governance and board oversight	A/R	C	I
Safe staffing and workforce deployment	A/R	C	C
Maternity triage implementation	A/R	C	A
Listening to women and families	A/R	C	A
MNVP engagement	R	A/R	C
Addressing health inequalities	R	A/R	A
System planning and commissioning	C	A/R	C
System assurance and escalation	C	A/R	A
National standards and policy	I	C	A/R
Martha's Rule rollout	R	C	A/R
National monitoring and benchmarking	I	C	A/R
Workforce funding and national programmes	C	C	A/R
Implementation of review recommendations	R	A/R	A

Key:
A = Accountable
R = Responsible
C = Consulted
I = Informed

7. Based on responsibilities outlined in the Amos recommendations and NHS England action plan.

Risks for Devon, Cornwall and the Isles of Scilly

8. The national findings create several risks for the Clusters maternity system:

Assurance Risk

9. Boards may receive assurance on compliance and performance but not have sufficient insight into women's lived experience, cultural issues or early warning signals. This was a key feature in Nottingham.

Inequalities Risk

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10. Devon, Cornwall and the Isles of Scilly must assure itself that outcomes are understood across:

- deprived communities

- ethnic minority populations
- vulnerable families
- rural and coastal populations

11. The reports demonstrate that inequalities can become ingrained and invisible without active scrutiny.

Safety and Escalation Risk

12. Triage and escalation arrangements must be demonstrably safe, supported by clear metrics and regular review.

Reputation and Public Confidence Risk

13. The national attention generated by Ockenden and Amos means Boards must be able to demonstrate visible progress and transparency. Public confidence is directly linked to evidence of learning and improvement

Board Assurance Questions

14. **Safety**

- How do we know women are being listened to and concerns acted upon?
- What evidence demonstrates that maternity triage is safe and responsive?
- What are our current maternity safety risks and mitigation plans?
- Can we demonstrate compliance with the NHS England 10-point plan?

15. **Governance**

- Is there clear executive accountability for maternity and neonatal services?
- Are medical, nursing, operational and midwifery leaders jointly accountable?
- How are maternity risks escalated through organisational and system governance?

16. **Learning**

- What themes emerge from incidents, complaints and claims?
- Can we evidence changes implemented following investigations?
- How do we know improvements are sustained?

17. **Inequalities**

- What are the differences in outcomes and experiences by ethnicity, deprivation and geography?
- What actions are in place to reduce inequalities?
- How are MNVPs and communities influencing service design?

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18. **Workforce and Culture**

- Are staff confident to raise concerns?
- What is the evidence regarding bullying, discrimination and psychological safety?
- How are we supporting recruitment, retention and professional development

Work already undertaken

19. Assurance was requested by the NHSE Southwest Regional Quality Group in June 2026 against the Amos/Ockenden 10-point plan and the Fuller enquiry mortuary recommendations.

20. Reporting templates were returned by all Trusts, and a high-level summary of responses is provided below. The board should note that there are challenges around workforce, triage & homebirth provision within the peninsula system.

Summary requirement	System trust action summary
Roll out of Martha's Rule	• Implementation leads & governance identified. • Elements of the rule are already established in some units.
Listen to women and their families using real time and transparently reported outcomes and experience and clinical audit data that are acted upon at board level	• Patient reported outcomes already forming part of standard governance but plans in place to strengthen these. • All units work closely with their Maternity & Neonatal Voices Partnership leads.
Dual board-level accountability between Medical Directors and Chief Nursing Officers for maternity and neonatal care This should also be implemented at system level.	Where not already in place, Chief Nursing and Medical Officers will attend key governance & safety meetings. Where this is in place, it will be strengthened.
'Amos into action' staff listening exercise	*Awaiting feedback progress update as this is an NHSE action* However, all peninsula Trusts have staff listening & engagement routes in place.
Addressing inequalities in maternity and neonatal outcomes (Perinatal Equity &	*Available by end 2026- staff to attend once launched.* Various approaches to equity currently in place (e.g. specific leadership roles & forums). These

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Anti-discrimination Programme)	will be further developed in line with requirements & learning.
Ensuring 24/7 safety and responsiveness of maternity and neonatal services (Service model & workforce) Commissioners must ensure that the perinatal service model aligns to local demographic needs. They must also address local gaps and siloed working.	Trusts regularly present workforce intelligence through existing governance structures, however gaps in workforce establishment, resilience and sustainability remain. Perinatal workforce is a risk within the peninsula. The Peninsula Board is asked to commit to this action through the development of updated commissioning intentions.
Trusts to review their homebirth services	*Awaiting national Homebirth Standards* There is a variable level of homebirth service across the peninsula. Trusts are undertaking assessments, enhancing training and reviewing service models. There is a known challenge across the country around births 'outside of guidance', which should be a recognised part of the homebirth service review work.
Responding to patient safety incidents, complaints and concerns with humanity, candour and a trauma-informed approach	Trusts follow national guidance through PSIRF and the Perinatal Quality Oversight Model, however, recognise the need to enhance trauma informed approaches and training where this is not already in place. MNVP engagement has either taken place or is planned regarding the complaint's procedure in each organisation.
Deliver safe and effective triage in maternity services	All Trusts are reviewing their services against the requirements of the National Maternity Triage Specification. There are issues with triage present in all Trusts, identified either through recent CQC inspections and/or Trust internal risk processes. These issues include process, escalation and estates and significant work is underway or planned to address this in each organisation.
Post death care	Fuller inquiry recommendations reviewed, action plans outlined and Human Tissue Authority governance in place. As of the end of July 2026, some actions remained outstanding with plans in place to complete.

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National Return

21. On 25 September 2026 Trusts will submit a formal baseline assessment to the Southwest Perinatal Team (NHSE). On 31 December a progress update will follow.

22. Trust boards must sign off both submissions and ensure that board oversight includes regular review of risks, progress, evidence & gaps. This should be undertaken through existing governance arrangements.

23. The assurance provided should confirm that delivery is on track with key risks, concerns and dependencies escalated appropriately. The Trust boards may delegate this oversight but must maintain accountability.

24. Further detail around this assurance process, including the required template can be found here NHS England » Maternity and Neonatal 10 Point Plan assurance process.

Next Steps

25. **Within 3 Months**

- A Cluster-wide gap analysis against the Amos recommendations and NHS England 10-point plan will be completed following the submission of Trust self-assessment on 24 September 2026.
- Local triage arrangements will be reviewed including assurance methodology and board reporting arrangements.
- Executive and board accountability structures at provider and system level will be re-reviewed. Significant work has already taken place to review and strengthen governance at provider and system level as part of the ICB merger and the Devon trusts participation in the national Perinatal Safety Support Programme.
- Review maternity inequalities dashboard and agree priority improvement actions. Work to address inequalities is already underway but further due diligence will be undertaken to ensure the work is fully data and evidence informed.

26. **Within 6 Months:**

- Establish routine board reporting of women's experience alongside clinical outcomes.
- Undertake a system review of workforce, escalation and resilience arrangements. This work is in progress.
- Review implementation of Martha's Rule across maternity and neonatal services.
- Strengthen processes for learning from incidents, complaints and claims.

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27. **Within 12 Months:**

- Provide system assurance to the South West Peninsula Board on implementation of Amos and Ockenden recommendations.
- Demonstrate measurable improvement in safety metrics, women's experience and inequalities outcomes.
- Align local maternity strategy with the forthcoming National Maternity and Neonatal Action Plan.

Recommendation

28. The South West Peninsula Board is asked to:

- **Note** the Amos and Ockenden reports and view them not as isolated provider issues but as a call for strengthened system leadership, transparency, safety culture and accountability, with a particular focus on listening to women, addressing inequalities and ensuring that learning translates into demonstrable improvement.

- **Approve** the plan for next steps and receive analysis of the systems position following the national return in October 2026

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board meeting in public

Voice of local people: Understanding the experience of maternity and neonatal services

Date of meeting	Date report produced
24 September 2026	9 September 2026

Author(s)		Report approved by	
Name and title:	Keri Ross, Head of Experience Lucy Tuson, Director of Experience and Engagement	**Name and title:**	Rachel Pearce, Chief Strategic Planning and Commissioning Officer

Executive summary

This paper introduces the **"Voice of Local People"** programme, a new approach to embedding patient experience, inequalities and outcomes as a routine and visible part of Board discussions across NHS Cornwall and Isles of Scilly and NHS Devon. The aim is to ensure that strategic decisions are informed by a clear understanding of how local people experience care and whether services are improving outcomes for those who use them, particularly communities experiencing the greatest health inequalities.

The programme is underpinned by a strategic framework that will be used consistently at future Board meetings. In time it will align with the NHS England patient experience agenda, the Core20PLUS5 approach to reducing health inequalities, the NHS 10 Year Health Plan. The framework will enable the Board to build a cumulative understanding of experience across major care pathways and assess whether commissioning intentions are delivering meaningful improvements for residents across Devon, Cornwall and the Isles of Scilly.

Maternity and neonatal care is the first pathway presented through this approach. Drawing on national maternity surveys, Healthwatch reports, Maternity and Neonatal Voices Partnership (MNVP) feedback, complaints data and local engagement insight, the review provides a comprehensive picture of the

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experiences of people and families who use maternity services across the peninsula.

The evidence shows that maternity experiences across Devon, Cornwall and the Isles of Scilly are more similar than different. Families consistently value safe, compassionate and personalised care delivered by professionals who listen, communicate clearly and involve them in decisions. Positive experiences are closely linked to continuity of care, trusted relationships with staff and feeling informed throughout pregnancy, birth and the postnatal period. However, the review identifies several important challenges. Geography is one of the strongest determinants of maternity experience and access. Rurality, transport availability, distance from specialist services and, for Isles of Scilly residents, reliance on mainland services, all have a significant impact on how care is experienced.

Across all areas, communication emerged as the most consistent theme in patient feedback. Families report receiving conflicting advice, not always understanding clinical decisions and feeling insufficiently informed about next steps in care. Postnatal care is identified as the weakest part of the maternity pathway, with many women reporting lower levels of support once they return home. Access to health visiting, mental health support, feeding advice and specialist services are recurring concerns.

The review also highlights that significant inequalities remain hidden within generally positive overall outcomes. Women and families experiencing deprivation, rural isolation, disability, mental health challenges, domestic abuse, language barriers and social exclusion often face multiple and overlapping barriers to accessing and benefiting from maternity care. There are currently also notable gaps in the voices currently represented within engagement activity, particularly among young parents, ethnic minority communities, Gypsy, Roma and Traveller communities, and people with learning disabilities.

Several strategic opportunities are proposed for Board consideration, including priorities for strengthening postnatal support, improving access and continuity of care, addressing geographical inequalities, enhancing community partnership working and ensuring the voices of underrepresented groups are routinely heard. The paper also recommends establishing a Devon and Cornwall Maternity Community Partnership and convening a Peninsula-wide Maternity Summit to support future service development, which aligns with the national NHS wide conversation ‘Your Voice: Building a Better Future for Maternity and Neonatal Care’ which launches in September.

The Board is asked to note the findings, support the further development of the Voice of Local People programme, agree the proposed next steps, and ensure that future Board discussions routinely consider patient experience, inequalities and outcomes alongside quality, workforce and financial performance information.

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If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

To which Board Assurance Framework risk does the report relate?
N/A

What are the recommendations arising from the report?
NOTE the findings from the maternity experience review and associated inequalities analysis.
Discuss and **AGREE** the next steps outlined, which includes the opportunity to Develop a Devon and Cornwall Maternity Community Partnership and convene a Devon Maternity Summit to engage women, families, communities and partners as part of long-term approach to discuss and consider the future needs for maternity and neonatal pathway and services
AGREE how the Board should be updated on how local maternity improvement priorities align with the national maternity improvement programme, the AMOS review recommendations and the NHS 10 Year Health Plan.
SUPPORT the continued development of the "Voice of Local People" programme and agrees that future Board discussions should routinely consider patient experience, inequalities and outcomes alongside quality, workforce and financial information.
INTRODUCE the metrics for future engagement in relation to CORE20PLUS5

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
People, Customer Experience and Equity Committee on 3 September 2026

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What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	The report strengthens the role of residents, patients, families and communities in shaping strategic decision-making by introducing the Voice of Local People framework. It highlights the importance of understanding lived experience, particularly for rural communities, island residents, young parents and those experiencing health inequalities. The proposed Maternity Community Partnership and Maternity Summit would provide additional opportunities for community involvement and co-design of future services.
Quality and Safety	The report identifies several areas directly linked to quality and safety, including potential access to intrapartum care, communication, continuity of care, postnatal support and workforce resilience. It highlights that geography can influence access to safe care and that patient experience should be considered alongside clinical outcomes, safety indicators and performance measures. The framework will support routine Board oversight of quality, safety, experience and inequalities.
Equality, Diversity and Inclusion	Equality and reducing health inequalities are central to the report. The analysis highlights the disproportionate impact of deprivation, rurality, disability, mental health needs, ethnicity, domestic abuse and social exclusion on maternity experiences and outcomes. The proposed approach aligns with the NHS Core20PLUS5 framework and seeks to increase engagement with seldom-heard communities, ensuring services are shaped by the experiences of people who currently face the greatest barriers to good outcomes.
Finances and Use of Resources	The report does not seek approval for new expenditure at this stage. However, future priorities arising from the findings may require investment decisions relating to postnatal support, engagement activity, community partnerships, workforce resilience and reducing geographical inequalities. Improved understanding of patient experience may support more effective targeting of resources towards interventions that deliver greatest benefit and value.
Regulation and Legal Requirements	The report supports compliance with NHS England expectations regarding patient and public involvement, reducing health inequalities, patient experience and

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	quality improvement. It also supports delivery of recommendations from the Amos Review, the NHS 10 Year Health Plan and the Integrated Care Board's statutory responsibilities to involve patients and the public in planning and service improvement. Future engagement activity will need to continue to meet relevant consultation and public involvement requirements where applicable.
Information Governance (DP and Cyber)	No significant information governance or cyber security implications arise directly from this report. Any future use of patient stories, filmed experiences, personas, engagement data or feedback will need to comply with UK GDPR, Data Protection Act requirements, confidentiality standards and NHS information governance policies. Appropriate consent processes should be maintained for all patient experience material
Transformation and Innovation	The report represents a significant transformation in how the Board considers patient experience as part of strategic decision-making. It introduces a systematic and longitudinal approach for incorporating lived experience, personas, journey mapping and inequalities analysis into Board discussions. The report also recognises opportunities arising from digital innovation, including the implementation of the Epic electronic patient record, whilst recognising that digital approaches must not disadvantage those who require non-digital access routes.
Environment and Climate Change	There are no direct environmental or climate change implications arising from the recommendations within this report.

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Voice of local people: Understanding the experience of maternity and neonatal services

Introduction

1. NHS Cornwall and Isles of Scilly and NHS Devon's Board have set out an ambition that the experience, needs and outcomes of local people should become a routine and visible part of Board discussions and decision-making.

2. This approach is intended to shift the Board discussion from "what services are doing" to "what local people experience". Over time, the Board will build a cumulative understanding of how people experience care across major pathways, where inequalities exist, and whether commissioning decisions or service improvements are resulting in better outcomes and experiences for local people.

3. To support this ambition, this paper sets out a 'voice of local people' initiative supported by a strategic framework as a standing component of future Board meetings and presents the first experience for maternity and neonatal care for initial discussion.

4. The 'voice of local people' programme focuses on:
   - Understanding services from the customer perspective.
   - Identifying strategic priorities that could improve experiences and outcomes.
   - Bringing the lived experience from our communities into Board discussions through experience narratives, personas and journey mapping.
   - Monitoring experience, outcome, safety and equity measures that matter most to local people – with a particular focus on people experiencing the most health inequalities
   - Understanding the impact of decisions on people experiencing the poorest health outcomes and greatest inequalities.
   - Ensuring the Board maintains clear sight of how commissioning intentions are being experienced by local people.

5. Underpinned by a framework which is designed to ensure that discussions about commissioning, transformation, quality and performance are informed not only by operational, financial and clinical information, but also by the experiences and outcomes of the people who use our services. In time, this framework will support delivery of the NHS England patient experience requirements and ensure national requests such as the Baroness Amos' Independent Investigation into Maternity and Neonatal Services are delivered upon and that our Board routinely reviews experience, outcomes, inequalities and evidence of improvement from the perspective of the voice and experience of local people.

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6. The framework will align directly with the commissioning intentions for Devon and Cornwall and Isles of Scilly and utilise the NHS Core20PLUS5 approach to ensure a particular focus on reducing health inequalities and improving outcomes for underserved communities. The approach will provide a consistent mechanism across future Board meetings, creating a cumulative picture of customer experience and system improvement over time and the impact it drives in improving overall experience for our residents across CIOS and Devon.

Mapping the patient journey: maternity and neonatal services

Context

7. This is the first in a series of pathway-based experience reports demonstrating how experience insight can be used alongside quality, safety and performance data.

8. To support Board discussion, each pathway review is accompanied by a visual presentation drawing on local insight, patient journeys, population personas, experience mapping and video content.

9. The aim is to bring to life the maternity and neonatal patient journey for further discussion by our Board so it can go on to inform further strategic planning across maternity and neonatal services in Devon, Plymouth, Torbay, Cornwall and the Isles of Scilly in the future.

10. National maternity survey findings, local NHS reports, Healthwatch feedback, complaint themes and engagement insights from across Devon, Plymouth, Torbay, Cornwall and the Isles of Scilly have informed the approach. Annex 1 provides a detailed breakdown of source data and evidence.

The key facts and figures from our maternity and neonatal care experience across Cornwall, the Isles of Scilly and Devon

11. Collectively, maternity services across Devon, Cornwall and the Isles of Scilly support approximately 12,500 to 13,500 births each year.

12. Around 4,000 births occur in Cornwall and the Isles of Scilly, with a further 8,500 to 9,500 births occurring across Devon's maternity providers.

13. While this represents a relatively small maternity population compared with many urban systems, the geography covered is extensive, spanning large rural areas, coastal communities and the unique challenges associated with providing maternity care for residents of the Isles of Scilly.

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Locality	Main Provider	Approx. Annual Births
Exeter and East Devon	Royal Devon University Healthcare (RD&E)	~3,000
North Devon	North Devon District Hospital	~1,200-1,500
Plymouth	University Hospitals Plymouth (Derriford)	~4,000
Torbay and South Devon	Torbay Hospital	~2,000
Cornwall and Isles of Scilly	Royal Cornwall Hospitals (Treliske)	~3,500-4,000

14. Evidence from national maternity surveys and successful continuity-of-care models demonstrates that women consistently value personalised, relationship-based care, regardless of geography. The challenge for future service design is therefore how to create a sustainable model that maintains continuity, local accessibility and meaningful engagement with the communities it serves.

15. Due to our location, maternity services operate across organisational and geographical boundaries with some women and birthing people receiving care outside their immediate area, particularly for specialist services.

16. Importantly, patient flows do not always align with county or Integrated Care Board boundaries. For some communities in North Cornwall, particularly around Bude and the surrounding area, maternity services in North Devon may be geographically closer than services in Truro. As a result, service changes in Devon can directly affect the experience of Cornwall residents.

17. The maternity population across Devon, Cornwall and the Isles of Scilly is relatively small, geographically dispersed and predominantly White British, compared with England as a whole.

18. While often perceived as affluent areas, all three geographies contain significant inequalities linked to deprivation, housing affordability, transport access, disability, rural isolation and access to public services.

19. Maternity services in Devon and Cornwall are operating within a changing demographic landscape. Nationally, birth rates have fallen by approximately 20% over the past decade, creating new challenges for workforce planning, service configuration and financial sustainability.

20. Between January 2022 and June 2026, all trusts in Devon, Cornwall and the Isles of Scilly have seen significant increases in the rate of service users from minority ethnic groups.

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NHS trust	January 2022 to December 2022	July 2024 to June 2025	Percentage increase
Royal Cornwall Hospitals	3.2% 120 out of 3,705	6.2% 215 out of 3,450	92.4%
Royal Devon University Healthcare	4.7% 200 out of 4,235	9.1% 390 out of 4,280	92.9%
Torbay and South Devon	4.2% 80 out of 1,905	8.0% 140 out of 1,755	90.0%
University Hospitals Plymouth	6.0% 180 out of 2,995	13.4% 405 out of 3,030	122.4%

The figures in the above table are from the national Maternity and Neonatal Equalities dashboard using the metric "EthnicGroupMotherAtBirth".

The current maternity and neonatal care experience across Cornwall, the Isles of Scilly and Devon

21. Across the peninsula, nearly 350 responses to the 2025 CQC maternity survey consistently identify the same factors as defining a positive maternity experience:

    - Being listened to
    - Feeling safe
    - Kind and compassionate staff
    - Continuity of care
    - Clear information and communication
    - Involvement in decision-making
    - Support for partners and families
    - Access to perinatal mental health support
    - Good postnatal care and feeding support

22. These themes are consistently reflected in national CQC maternity surveys and local Maternity and Neonatal Voices Partnership (MNVP) engagement across both Devon and Cornwall.

23. The most important, shared challenge across Devon, Cornwall and the Isles of Scilly is geography. For many families, maternity experience is influenced as much by travel times, transport availability, housing pressures and access to services as by clinical care itself.

24. However, the nature of these challenges differs across the peninsula. In Devon, access challenges are increasingly linked to service resilience and maintaining equitable access across a large provider landscape.

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25. In Cornwall, distance from specialist services remains a significant factor, particularly for rural and coastal communities.

25. For the Isles of Scilly, maternity experience is uniquely influenced by the need to access mainland services, with travel, accommodation and weather-related disruption often forming part of the maternity journey.

26. As maternity experiences do not always align with organisational boundaries, the impact of service changes, including the current temporary suspension of births and face-to-face maternity triage at North Devon District Hospital during 2026 therefore had implications not only for Devon families but also for some women and birthing people in North Cornwall who may previously have relied on those services because of shorter travel times and easier accessibility.

What local insight tells us

27. Importantly, many of the themes identified through patient experience data are not unique to one organisation or locality. Similar issues relating to communication, continuity, postnatal support, access and travel are reported consistently across Devon, Plymouth, Torbay, Cornwall and the Isles of Scilly.

28. This suggests that many opportunities for improvement require a whole-system response rather than action by individual organisations alone.

What local insight tells us	What it means for services
Women consistently praise midwives, obstetricians and support staff. Compassion, professionalism and reassurance remain the strongest positive themes across all local providers.	**Staff remain the greatest strength** of maternity services despite significant workforce pressures.
Continuity of care is inconsistent. Many women value seeing the same midwife or small team, but report having to repeat their story to different professionals throughout pregnancy and after birth.	**Better continuity** improves confidence, personalised care and safety.
Feeling listened to is one of the biggest drivers of experience. Positive experiences are associated with women feeling their concerns were taken seriously, while negative experiences often involve feeling dismissed or not believed.	**Listening and shared decision-making** are critical quality indicators.
Postnatal care receives the lowest satisfaction. Families often report delays obtaining help on wards, feeding	**Postnatal care** remains the greatest opportunity for improvement.

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support, pain relief and discharge information once the baby is born.	
Communication varies between teams and settings. Women want clearer explanations, consistent information and better communication between hospital, community midwives, GPs and health visitors.	**Better coordination** would improve experience and reduce anxiety.
Access can be challenging in rural and coastal communities. Long travel distances, transport, appointment availability and accessing specialist services are recurring concerns, particularly across Devon, Cornwall and the Isles of Scilly.	**Geography** continues to shape inequalities in maternity experience.
Partners and families want greater involvement. Families value flexible visiting, involvement during labour and clear communication throughout the maternity pathway.	**Family-centred care** remains an important expectation.
Mental health support has improved but remains variable. More women report being asked about their mental health, although access to ongoing perinatal support still differs between areas.	**Early identification** is improving, but equitable access remains important.
People understand pressures on services. Many women recognise staffing shortages and distinguish between excellent staff and organisational pressures.	**Workforce shortages** are increasingly visible to patients.

Intersectionality

29. Intersectionality recognises that people's experiences of maternity care are shaped by multiple overlapping social, economic and personal factors. Rather than viewing inequalities through a single lens, such as deprivation or ethnicity alone, an intersectional approach considers how characteristics such as poverty, rurality, age, disability, maternal and infant mental health and social exclusion interact to influence access, experience and outcomes. This approach can help identify communities whose needs may be overlooked within traditional engagement and service design processes.

Intersectional Group	Potential Experience Risk
Rural + low income	Travel, access, continuity
Young parent + mental health deed	Isolation, confidence, support
Learning disability + deprivation	Understanding information, advocacy
Domestic abuse + pregnancy	Safety, disclosure, trust
Traveller community + rurality	Access, continuity, inclusion

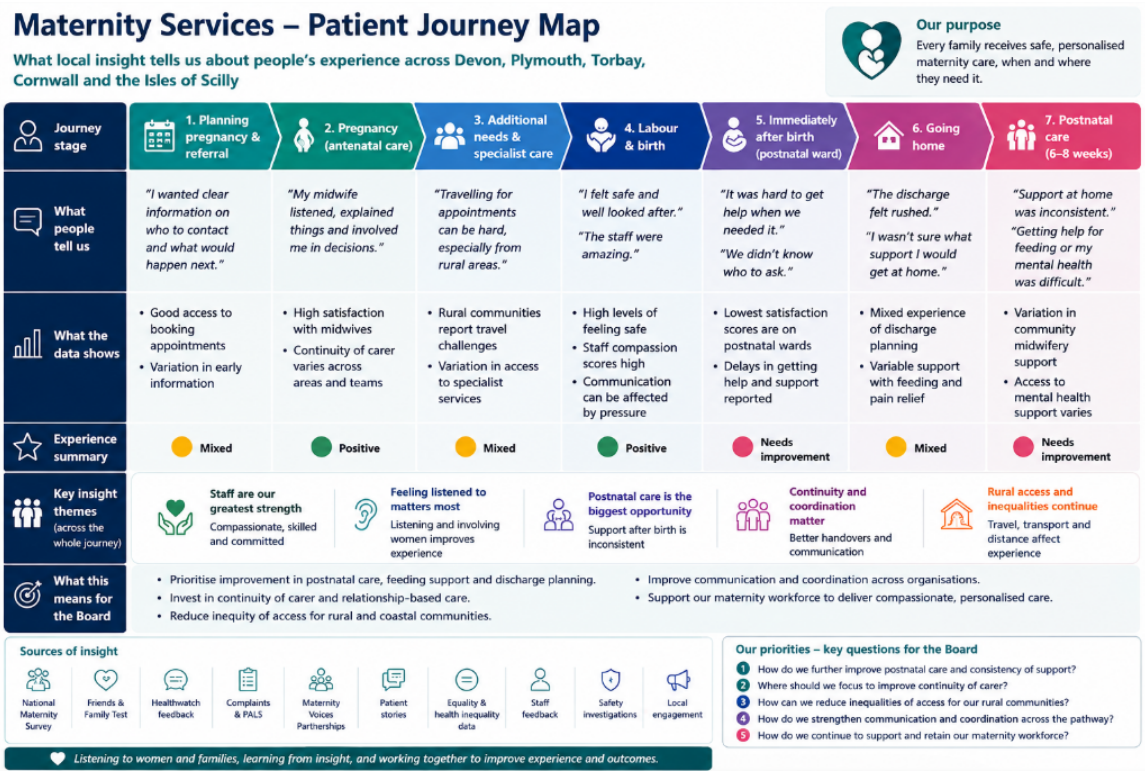
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Ethnic Minority woman + rurality	Cultural safety, representation
Mental health need + low income	Accessing support, postnatal wellbeing

30. Future maternity services and support should prioritise building partnerships with trusted community organisations to better understand the needs of women and birthing people whose voices are least likely to be heard, particularly those living in deprived coastal communities, young parents, women experiencing mental ill health, people with learning disabilities, domestic abuse survivors, Gypsy, Roma and Traveller communities and minority ethnic women living in rural areas.

Patient journey map

31. Across the locations of Cornwall, Deon and Isles of Scilly we have mapped out the general patient journey map, which highlights key touchpoints and areas for improvement – which are highlighted in Board presentation that accompanies this paper:



Patient experience films

32. As part of the presentation that accompanies this paper, we include personal examples taken from a series of patient films that illustrate the maternity and neonatal care patient journey and their experiences.

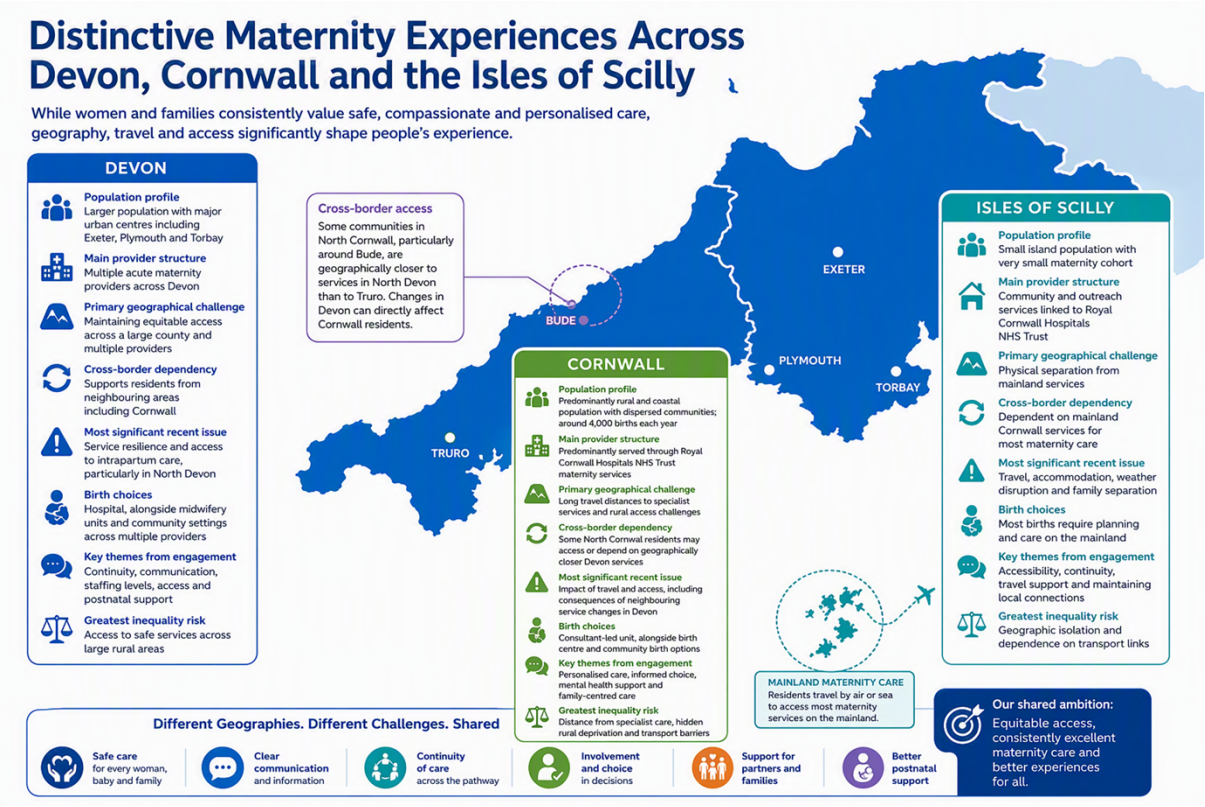
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The full set of films are available to watch in full:

- Film - New mums talk honestly about breastfeeding and support from the Plymouth-wide Infant Feeding Team
- Film - Teen mum Gabbie turned her life around with support from the Family Nurse Partnership
- Film - Perinatal peer support groups offering a safe, nurturing and supportive space for new mums
- Film – Cornwall's post birth support - https://vimeo.com/1221440357?fl=pl&fe=sh
- Film – Wild Young Parent - (436) RANT Club Music Video by WILD Young Parents in Cornwall - with subtitles - YouTube

Distinctive features by location

33. The diagram below illustrates that while women and families consistently tell us that the fundamentals of good maternity care are the same regardless of where they live, the practical experience of accessing services differs significantly across Devon, Cornwall and the Isles of Scilly. Geography, travel, service resilience and access to specialist care emerge as some of the strongest determinants of experience.



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Maternity and neonatal personas

34. A dedicated persona programme supports a better understanding of the needs, preferences, behaviours and barriers experienced by different resident groups and communities. This enables more targeted communications, engagement activities and service improvements, helping ensure that services are designed around the people who use them.

35. The programme has the potential to improve public participation, trust and access to services by ensuring that resident voices are reflected in planning and delivery.

36. Three maternity specific personas have been developed to test the various elements of the maternity and neonatal pathways locally, and what the potential impacts of these might be for local people. These personas are developed by using real-time, representative data from Devon and Cornwall populations.

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MATERNITY AND NEONATAL PERSONAS

Different people. Different needs. One system that works for all.

HANNAH



Age: 26 | Pregnant with first child
Ilfracombe

Household	Lives with partner
Housing	Social rented housing
Employment	Working full-time
Transport	Relies mainly on a car
Digital confidence	Comfortable online but does not want digital to be the only route to care
Support	Partner is important; wider family support is geographically distant

Key consideration:
Travel distance and maintaining personal access to care.

EMMA



Age: 31 | Parent of a 4-month-old baby
St Austell

Household	Lives with partner and baby
Housing	Owns home with mortgage
Employment	Maternity leave from administrative / customer-service role
Transport	One-car household
Digital confidence	Generally digitally confident but occasionally affected by poor connectivity
Support	Parents live elsewhere in Cornwall and can help occasionally

Key consideration:
Balancing childcare, finances and access to postnatal support.

AMINA



Age: 30 | Pregnant with first child
Plymouth

Household	Lives with partner
Housing	Private rented flat
Employment	Full-time Healthcare / care-sector administrator
Transport	Relies on public transport
Digital confidence	Moderate. From Nigeria, first language is Yoruba not English
Support	Partner, no local family support

Key consideration:
Accessible communication, transport and culturally appropriate support.



Hannah, 26

Ilfracombe

Hannah lives with her partner in social housing on the North Devon coast. She works full-time and is expecting her first baby.

Although comfortable using online services, Hannah values face-to-face appointments and reassurance from healthcare professionals.

Her wider family lives outside the area, so she relies heavily on her partner for support.

What matters most to Hannah

- Local services whenever possible
- Consistency and continuity of care
- Clear communication
- Travel that is practical and affordable
- A choice of digital and face-to-face appointments

Potential barriers

- Longer journeys for appointments
- Additional travel costs
- Limited family support nearby
- Concern about changes that increase travel distance



Emma, 31

St Austell

Emma is currently on maternity leave and lives with her partner and baby in a home they own with a mortgage.

Although financially stable, reduced maternity pay and future childcare costs are creating pressure.

Her parents live elsewhere in Cornwall and can help out occasionally.

What matters most to Emma

- Easy access to postnatal support
- Breastfeeding and infant-feeding advice
- Flexible appointments around childcare
- Practical information for returning to work
- Support for emotional wellbeing

Potential barriers

- Childcare and travel arrangements
- Reduced household income
- Finding support outside normal hours
- Worry about returning to work



Amina, 30

Plymouth

Amina works in a healthcare administration role and lives with her partner in privately rented accommodation.

She is originally from Nigeria and speaks Yoruba as her first language. Health information can sometimes be difficult to navigate.

She has no local family support so relies mainly on her partner.

What matters most to Amina

- Clear, easy to understand information
- Inclusive and culturally sensitive care
- Good public transport links
- Opportunities to ask questions and get reassurance
- Feeling listened to and respected

Potential barriers

- Reliance on public transport
- Complex or unclear information
- Language and communication challenges
- Limited local support network

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Eperience differences across the peninsula

37. The maternity experience evidence is strikingly consistent: people value being listened to, clear information, continuity, kindness, involvement in decisions and confidence that services are safely staffed.

38. CQC's 2025 maternity survey provides an important local benchmark. Women who gave birth between 1 and 28 February 2025 (and January, where trusts did not have a minimum of 300 eligible births in February) were invited to participate.

39. Labour/birth experiences were broadly in line with national levels at all Devon and Cornwall providers.

40. Postnatal care was weaker: Royal Devon scored **6.7/10** and was rated somewhat worse than expected; Plymouth and Torbay both scored **7.3/10**. Royal Cornwall also scored **7.3/10**. At Royal Devon, partner length of stay scored only **3.6/10** and was worse than expected and which relates directly to family experience, emotional wellbeing and support during the immediate postnatal period.

41. The biggest inequality that is starting to emerge is that some people can access safe care during labour and birth more easily than others because of where they live.

42. While the themes are similar, there are some local nuances:

    - **Devon (Royal Devon)** – Recent survey results show performance above the national average in several areas, particularly around antenatal communication, involvement in decisions and support from midwives. Women and birthing people particularly valued having time to ask questions and feeling informed.
    - **Plymouth (University Hospitals Plymouth)** – Feedback commonly highlights skilled clinical teams and confidence in emergency care, but some women describe inconsistent communication, longer waits and variable postnatal support.
    - **Torbay** – Feedback often reflects positive relationships with maternity staff, personalised care, particularly around birth planning and being listened to.
    - **Cornwall and Isles of Scilly (Royal Cornwall Hospitals)** – Feedback shows people generally value local maternity teams but highlight the impact of travelling long distances for specialist appointments and concerns about maintaining local access for more remote communities.

43. National 2025 maternity survey qualitative evidence confirms these themes, while Devon Maternity and Neonatal Voices Partnership (MNVP) work specifically emphasises welcoming environments, accessibility, mental-health support, inclusive information and personalisation.

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44. The Cornwall Maternity and Neonatal Voices Partnership MNVP has had a particularly strong impact locally, where it has directly driven up the quality of care and has implemented initiatives that are now being evaluated as improving clinical and social outcomes – this includes the role of Pregnancy circles, which is explained further in this report.

The issues people raise most often

Communication is the biggest recurring theme

45. The strongest and most consistent theme across Devon and Cornwall is communication.

46. Families frequently report:
    - inconsistent advice from different professionals
    - not always understanding why tests, observations or interventions are taking place
    - inadequate explanation of options and choices
    - feeling uninformed about next steps in care.

47. In Cornwall, more than 30% of comments related to communication, consistency of advice and informed consent.

Postnatal care feels less strong than antenatal and birth care

48. A common finding nationally and locally is that postnatal support often does not match the quality of antenatal or intrapartum care. With more people feeling less supported once they are home with their baby

49. Women and birthing people describe:
    - feeling less supported once home
    - wanting more face-to-face contact
    - difficulties accessing health visitors
    - variation in breastfeeding and infant-feeding advice
    - challenges accessing specialist support.

Access and geography remain major challenges

50. This issue is particularly pronounced in rural Devon, Cornwall and the Isles of Scilly. Families report concerns about:
    - travel distances for appointments
    - transport, car ownership and parking
    - access to 24/7 transport options
    - accessing specialist services
    - attending postnatal appointments shortly after discharge
    - inequalities between urban and rural areas.
    - for families from the Isles of Scilly, having to go to mainland (Cornwall) to give birth

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Capacity and staffing pressures are visible to service users

51. While staff are often praised, families notice when services are stretched. Feedback includes:
    - delays in appointments
    - feeling rushed
    - longer waits for specialist support
    - difficulty accessing perinatal mental health services or infant feeding support due to demand.

Choice and informed decision-making

52. People value personalised care but some feel they are not always presented with all available options around:
    - place of birth
    - induction of labour
    - pain relief
    - infant feeding support
    - postnatal pathways.

53. Importantly, these concerns are often experienced together rather than in isolation. For some women, birthing people and families, factors such as rurality, poverty, poor transport links, housing insecurity, limited family support, language barriers, disability, mental health needs or low confidence in navigating services can combine to create multiple barriers throughout pregnancy and early parenthood.

54. The cumulative effect of these intersecting challenges can significantly influence access to care, experience of services, ability to make informed choices and ultimately outcomes for both parent and baby. Understanding and addressing this intersectionality is essential if we are to reduce inequalities and give every child the best possible start in life.

Impacts of demographic, geographical, and socioeconomic factors geography and rural/island isolation

55. Women and families experiencing deprivation, rural isolation, disability, mental health challenges, domestic abuse, language barriers and social exclusion often face multiple and overlapping barriers to accessing and benefiting from maternity care. Some of the key factors and their impacts are indicated below.

Transport and travel times

- **Rural and remote areas:** Families living in rural Cornwall, North Devon, or remote coastal communities face significant travel times to acute obstetric units (e.g., Derriford Hospital in Plymouth or Royal Cornwall Hospital in Truro).
- **Isles of Scilly:** Most birthing parents on the Isles of Scilly must travel to the mainland (typically Cornwall) weeks prior to their due date to ensure safe

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delivery care, creating added stress, logistical hurdles, and temporary separation from local support networks.

- **Temporary unit closures:** Rural maternity units do occasionally experience temporary closures or reduced midwife-led services due to staffing shortages, forcing longer journeys to central hospitals.

Ethnicity and language barriers

- **Access to translation:** Healthwatch and local LMNS assessments highlight that ethnic minority women and migrants face major hurdles accessing interpreter and translation services. Some report reliance on digital translation apps or family members rather than professional interpreters.
- **Equity gaps:** National and local data reveal that Black, Asian, and mixed-ethnic women experience disproportionately higher risks of adverse outcomes and report feeling less listened to or validated during clinical encounters.

Socioeconomic deprivation and background

- **Deprived urban areas:** In higher-deprivation areas such as Plymouth and parts of Torbay, national data indicates lower satisfaction with dignity, respect, and postnatal support.
- **Digital inclusion and transport costs:** Families experiencing economic deprivation report greater difficulty attending routine appointments due to public transport costs and lack of digital devices for virtual appointments or online notes

Age and mental health

- **Perinatal mental health:** National and local CQC survey findings indicate that while routine mental health screening during pregnancy has improved, women with pre-existing or perinatal mental health needs are significantly less likely to report feeling supported with kindness and understanding post-birth.
- **Younger mothers:** Younger birthing parents frequently express feeling judged or needing tailored communication and accessible peer support networks

Key findings

1. Average maternity outcomes across Devon, Cornwall and the Isles of Scilly can appear positive while significant inequalities remain
2. Geography remains one of the strongest determinants of maternity experience
3. Rurality affects access, experience and outcomes and should be considered a health determinant rather than simply a demographic characteristic
4. Postnatal care remains the most consistent improvement opportunity
5. Staffing, service resilience and organisational sustainability directly affect patient experience
6. Hidden deprivation linked to housing, transport and rural isolation can create significant barriers to care
7. Disability, neurodiversity and mental health needs require increasingly personalised and accessible approaches

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8. Digital services create opportunities but must always be complemented by non-digital alternatives
9. The Isles of Scilly experience is distinct and requires separate consideration because travel and geography influence every stage of the maternity pathway
10. The strongest improvement opportunity is combining universal maternity provision with proportionately greater support for families experiencing multiple disadvantages
11. Maternity experiences do not always align with administrative boundaries. Changes within one maternity system may directly affect neighbouring populations, particularly in North Cornwall

What is making a difference already

Pregnancy circles in Cornwall

56. Some areas in Cornwall are now offering Pregnancy Circles as an alternative to routine care, if preferred. Instead of short, individual midwife appointments, Pregnancy Circles offer care and discussion in a small group with people who are due around the same time and who live in similar areas.

57. They combine clinical care with education and peer support in a relaxed, welcoming environment, focusing on continuity of care, shared learning, and building community. Pregnancy Circles have a range of benefits, including:
    - Improved experience: Women report feeling more informed, confident, and connected.
    - Better outcomes: Research shows group antenatal care can improve satisfaction and engagement.
    - Peer support: Sharing experiences reduces isolation and promotes emotional wellbeing.

Electronic patient record (EPR)

58. Across the Devon system, hospital trusts have moved to a shared electronic patient record through Epic, with Royal Devon University Healthcare NHS Foundation Trust introducing this in 2020, and University Hospitals Plymouth NHS Trust and Torbay and South Devon NHS Foundation Trust both going live earlier this year.

59. As Epic becomes embedded across Devon, there are opportunities to improve how information is shared between clinical teams and how women and birthing people access information and communicate with services. Royal Devon has reported positive feedback from its use of the My Care patient portal, particularly its ability to communicate directly with women and birthing people and provide information through a digital platform.

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60. As uptake of My Care increases across Torbay and South Devon and University Hospitals Plymouth, the system will be able to better understand the extent to which these benefits can be replicated across Devon.

15 steps for maternity in Cornwall

61. 15 steps is a toolkit developed and published by NHS England to be used by MNVPs to support them to elevate the voices of the service users and allow them to explore collaborative working to review, explore and design services within maternity units.

62. In 2022, Kernow MNVP, Royal Cornwall Hospitals, Healthcare Cornwall and others visited three local maternity units to make observations and gather experiences using photos, written notes, voice notes and videos.

63. A range of findings and examples were collated in a report, including good practice and where improvements were needed that influenced a series of recommendations that were taken to Royal Cornwall Hospitals' board.

Learning from maternity and neonatal best practice

64. Evidence from UK maternity services shows that positive experiences are strongly associated with continuity of care, effective communication, involvement in decision-making and trusted relationships with professionals. Women report better experiences when they feel listened to, respected and supported by a known midwife or small team throughout pregnancy, birth and the postnatal period.

65. A notable example is the Albany Midwifery Practice in South East London, which served highly disadvantaged and ethnically diverse communities in Lewisham, Deptford, Peckham and Southwark. The service provided relationship-based care through small caseload midwifery teams who supported women throughout their maternity journey, often in community and home settings.

66. Despite working in areas of significant deprivation, the practice achieved lower intervention rates, higher breastfeeding rates and better perinatal outcomes than national averages. Women and birthing people consistently reported feeling known, valued and empowered in decisions about their care.

67. The lessons for Devon and Cornwall are clear. Maternity services should prioritise continuity, personalised care and meaningful engagement with local communities.

68. For rural and coastal populations, where isolation, inequality and access challenges can affect experience and outcomes, relationship-based models of care may be particularly beneficial. Strengthening continuity teams, expanding community-based services and embedding the voice of women and families through co-design and Maternity and Neonatal Voices Partnerships (MNVPs) could improve both experience and outcomes.

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69. The evidence suggests that women value services where they are treated as partners in care and supported by professionals who know them as individuals.

What more could be done?

Population inequalities and gaps in data

70. We need to further consider the experiences, working with partners and connecting with established organisations, of people that we aren't currently hearing from, including:
    - People living in deprivation
    - Young parents
    - Experiences of maternal and infant mental health
    - Learning disabilities
    - People living in rural communities
    - Ethnic minority women
    - Impact on people facing domestic abuse
    - People from local Gypsy, traveller and Roma communities

Group	What we often know	What we often don't know
People living in deprivation	Outcomes, smoking rates, breastfeeding, service use	Experience of accessing services and decision-making
Young parents	Birth rates and service contacts	What support feels most helpful
Perinatal mental health	Referral and treatment activity	Women's lived experience of navigating support
Learning disabilities	Numbers known to services	Accessibility of maternity information and environments
Rural communities	Travel times and geography	Impact of travel on choices and experience
Ethnic minority women	Outcome inequalities nationally	Local experiences of inclusion, trust and cultural safety
Domestic abuse survivors	Safeguarding data	Experience of disclosure and support pathways
Gypsy, Roma and Traveller communities	Very limited routine data	Almost entirely absent from most maternity engagement

71. Reducing maternity inequalities requires more than analysing outcomes data; it requires understanding the experiences of communities whose voices are often underrepresented in traditional engagement approaches.

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72. Particular attention should be given to people living in deprivation, young parents, women experiencing perinatal mental health challenges, people with learning disabilities, rural communities, ethnic minority women, domestic abuse survivors, and Gypsy, Roma and Traveller communities.

73. Working with NHS colleagues who provide dedicated support and alongside trusted voluntary, community and faith sector organisations can help services reach groups that are less likely to participate in surveys or formal consultations. Shaping future maternity service and support should especially reflects the needs of all communities rather than only those who are most easily heard.

As part of the ICB and system engagement we propose introducing the following suggested metrics for future engagement:

74. Reported alongside standard maternity performance metrics:
    - % of engagement participants from Core20 communities
    - % of feedback from young parents
    - % of feedback from rural communities
    - Number of engagement activities delivered through community partners
    - Number of seldom-heard groups represented in service redesign
    - Evidence of changes made as a result of lived experience

75. Whilst maternity services engage routinely through MNVPs, Healthwatch and local engagement activity, there remain groups whose experiences are underrepresented in current insight. Future reports will therefore seek to strengthen the voice of communities who experience the greatest inequalities and poorest outcomes.

Conclusion

76. The maternity experiences of Devon, Cornwall and the Isles of Scilly are more similar than they are different. Across the peninsula, families consistently value safe, compassionate, personalised care delivered by staff who listen, communicate clearly and involve them in decisions.

77. However, the defining difference between the three geographies is the nature of their access challenge:

    - Devon's challenge is maintaining resilient maternity services across a large and complex provider landscape
    - Cornwall's challenge is providing equitable access across a dispersed rural population while recognising cross-border dependence on neighbouring services, particularly in North Cornwall
    - The Isles of Scilly's challenge is overcoming the practical realities of island geography and reliance on mainland maternity care

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78. Taken together, the evidence suggests that future maternity planning should increasingly consider Devon, Cornwall and the Isles of Scilly as an interconnected maternity system, while recognising the distinct experiences and inequalities faced by each community.

Board presentation, discussion and next steps

79. The purpose of presenting a patient journey, persona and filmed experience is not to provide anecdotal evidence, but to make strategic data more meaningful. By bringing together quantitative evidence, qualitative insight and individual experiences, the Board can better understand how policy, commissioning decisions and service design are experienced by women, babies and families in practice.

Strategic opportunities emerging from this maternity and neonatal care experience review

80. The Board may wish to note several strategic opportunities:

Insight	Strategic question
Geography increasingly shapes experience and access	How do we ensure equitable access regardless of where someone lives?
Postnatal care remains the weakest aspect of experience	How do we redesign support beyond birth?
Workforce fragility can rapidly affect access and confidence	How do we strengthen resilience across Devon services?
Women value continuity, communication and relationships	How do we organise services around people rather than organisations?
Public interest in maternity services is growing	How do we involve communities in shaping future models of care?

81. The evidence suggests that Devon's greatest maternity challenges are not solely clinical but relate to access, equity, service resilience and experience.

The Board is invited to consider 4 key areas:

1. What should be our overarching system maternity improvement priorities be for the next 12 months?

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82. Should our focus be on:
- Improving access and consistency of maternity triage.
- Strengthening continuity across the whole maternity pathway.

- Addressing postnatal care, which remains the weakest element of the experience.
- Reducing geographical inequalities in access to safe maternity care.

2. How do we respond to the challenges of rural maternity care?

83. The evidence highlights rurality as a key determinant of maternity experience, particularly where travel times, transport availability, workforce fragility and service disruption affect women's ability to access care.

84. The Board is asked to consider:
    - What principles should underpin maternity provision in rural and coastal communities?
    - How do we balance safety, sustainability and local access?
    - What does a modern community-focused maternity model look like for Devon?

3. Are we doing enough to hear and act on lived experience?

85. Particular consideration should be given to:
    - Rural communities.
    - Communities experiencing deprivation.
    - Disabled and neurodivergent women and birthing people.
    - Ethnic minority communities.
    - Young parents and those experiencing multiple disadvantage.

4. How do we involve the system and public in shaping future maternity and neonatal services?

86. A national NHS-wide conversation on maternity and neonatal care opens in September 2026, giving system colleagues the opportunity to share views and help shape improvements.

87. The findings will help inform the National Maternity and Neonatal Taskforce's action plan and provide valuable evidence for the Modern Service Framework, so that future decisions are grounded in the realities of staff, services and families.

88. Organisations across our system are being encouraged to support participation and promote this important opportunity.

89. Given the current public interest in maternity provision across our Cluster, which reflects the strength of feeling local communities have about maternity care.

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90. The Board may wish to consider further opportunities locally for a more open conversation about:
    - The challenges facing maternity services
    - Workforce sustainability
    - Women's expectations and choices
    - Future service models that could be utilised through the emerging neighbourhood health model

91. We propose two actions follow this Board discussion:

1) Develop a Devon and Cornwall Maternity Community System Partnership

A partnership bringing together:

- MNVP representatives
- VCSE organisations
- Family Hub leads
- Community arts organisations
- Perinatal mental health charities
- Maternity clinicians
- Local authority representatives

Its role would be to:

- gather lived experience continuously rather than episodically;
- use creative methods to engage seldom-heard communities;
- provide regular insight reports to Boards and Integrated Care Boards;
- ensure future service redesign is shaped by community voice alongside clinical and financial evidence.

2) Convene a system Devon maternity summit

Given:

- the known challenges of delivering maternity services in rural communities;
- increasing public interest and media attention surrounding maternity services;
- NHS England's 10 Year Health Plan direction towards neighbourhood and community-based care;
- the findings of the AMOS review regarding leadership, culture, listening to women and strengthening accountability;

the Board may wish to sponsor a **Devon Maternity Summit** bringing together:

- Women and families
- MNVP representatives
- VCSE organisations
- Local authorities
- Public health teams
- Primary care
- Community providers
- Acute trusts
- NHS commissioners
- Professional and clinical leaders

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The purpose would be to:

- Discuss and provide feedback on the experiences and challenges facing maternity care
- Hear about high performing systems and innovative approaches to rural maternity provision from the UK and internationally

This could go onto to shape:

- The co-design of future maternity and neonatal pathways, commissioning intentions and priorities with women and families rather than for them.
- Build a shared system response to the AMOS review and national maternity improvement requirements.

What should success look like by 2027?

92. Potential measures could include:
    - Improved access to maternity triage.
    - Reduced variation in experience between localities.
    - Improved postnatal experience scores.
    - Increased continuity of carer/team.
    - Improved equity indicators.
    - Stronger public confidence and involvement in service design

Future development of the 'Voice of Local People' Programme

93. Maternity and neonatal care is the first pathway presented through the Voice of Local People framework.

94. Future Board meetings will receive similar thematic reviews covering other major pathways and population groups, enabling the Board to build a longitudinal understanding of people's experiences across the health and care system.

Future pathways for review include:

- Long term conditions including diabetes, heart and lung conditions
- Keeping older people well in their neighbourhood using the NHS 10 year plan three shifts – prevention, digital and community
- Health Inequalities and Intersectionality
- Mental Health, Learning Disability and Neurodiversity
- Timely and effective access to primary care services (GP, pharmacy and dental)

95. A consistent methodology will be used for each review, combining experience data, outcome measures, inequalities analysis, patient journeys, personas and lived experience.

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Recommendations

96. The South West Peninsula Board is asked to:

- **Note** the findings from the maternity experience review and associated inequalities analysis.
- Discuss and **agree** the next steps outlined, which includes the opportunity to Develop a Devon and Cornwall Maternity Community Partnership and convene a Devon Maternity Summit to engage women, families, communities and partners as part of long-term approach to discuss and consider the future needs for maternity and neonatal pathway and services.
- **Agree** how the Board should be updated on how local maternity improvement priorities align with the national maternity improvement programme, the AMOS review recommendations and the NHS 10 Year Health Plan.
- **Support** the continued development of the "Voice of Local People" programme and agrees that future Board discussions should routinely consider patient experience, inequalities and outcomes alongside quality, workforce and financial information.
- **Introduce** the metrics for future engagement in relation to CORE20PLUS5

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Annex 1

Maternity and neonatal sources and evidence base:

This paper refers to publicly available evidence available to August 2026.

National sources

- Care Quality Commission (CQC) Maternity Survey 2025
- NHS Maternity Survey Benchmark Reports 2025
- NHS England Three-Year Delivery Plan for Maternity and Neonatal Services
- Office for National Statistics population and fertility data
- National Maternity and Perinatal Audit publications
- Maternity Services Data Set (MSDS)
- MBRRACE-UK maternal mortality reports

Cornwall and Isles of Scilly evidence

Royal Cornwall Hospitals NHS Trust

- Maternity service model and service information
- Birth centre and community provision
- Isles of Scilly maternity pathways
- Annual maternity activity

Kernow Maternity and Neonatal Voices Partnership

- Annual Report 2023-24
- Annual Report 2024-25
- Community engagement reports
- Isles of Scilly engagement work
- 15 Steps maternity reviews
- User feedback and co-production activity

Devon evidence

- Devon Joint Strategic Needs Assessment (JSNA)
- Devon maternity system data
- Devon Maternity and Neonatal Voices Partnership engagement
- CQC maternity survey results for Devon providers
- Publicly reported maternity service operational updates

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Interpretation

This paper is provided as a strategic engagement and planning tool rather than a description of every individual experience. Experiences vary according to geography, health status, disability, deprivation, transport access, ethnicity, age, family support and personal circumstances.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board meeting in public

Chair's report

Date of meeting	Date report produced
24 September 2026	16 September 2026

Author(s)		Report approved by	
Name and title:	John Govett, Chair	**Name and title:**	N/A

Executive summary
This report provides an update from the NHS Cornwall and Isles of Scilly and NHS Devon Integrated Care Board Cluster Chair.

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

To which Board Assurance Framework risk does the report relate?
N/A

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What are the recommendations arising from the report?
NOTE the contents of the report

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
N/A

What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	N/A
Quality and Safety	N/A
Equality, Diversity and Inclusion	N/A
Finances and Use of Resources	N/A
Regulation and Legal Requirements	N/A
Information Governance (DP and Cyber)	N/A
Transformation and Innovation	N/A
Environment and Climate Change	N/A

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Chair's report

Chief executive recruitment

1. An advert for our permanent cluster chief executive (CEO) went live in mid-August. Applications are open until Friday 18 September, with interviews and panels to commence after that. I will keep you updated as this progresses.

Options appraisal – Cornwall and Isles of Scilly

2. Cornwall and Isles of Scilly (CIOS) have for some time now been in the national vanguard of delivering the government's left shift of services from the acute into our local communities and thus the movement towards neighbourhood and communities has long been a Cornwall mindset and in practice. The options appraisal on 9 September 2026 was facilitated by The Value Circle and brought together the NHS ICB and NHS Provider, Primary Care chairs and chief executives in Cornwall along with key Devon Trust chief executives who provide key services to our residents in CIOS.

3. The meeting was very productive and the robust and constructive challenges to each other demonstrated our mature ability to discuss difficult issues and work towards a conclusion in the interest of the population we serve. The chief executives were asked by the chairs to bring back to another facilitating meeting in the middle of October, their collective views on way forward. A particular interest was how the providers along with the ICB in CIOS would work together to ensure this winter our patients receive the best possible service that we could deliver to them. It does not take contracts and commissioning in order to collaborate and if there is a genuine desire - which there appears to be to move resources into the community and into neighbourhoods – then the opportunity of winter will demonstrate a seriousness of each of us to deliver what we say is important and will improve the outcomes for our population and patience.

4. The paper I referred from the CEOs needs to be available for our 29 October 2026 board meeting.

North Devon hospital and peninsula maternity

5. On the agenda for the public meeting this month is a material set of papers considering a range of issues around our maternity services in our South West Peninsula.

6. I am grateful that at the start of our board meeting the; chair of the Devon County Council health and scrutiny committee, the North Devon primary care Co-Chair of their GP collaborative, along with the vice chair of NHS Royal Devon University Exeter Trust board will be attending our Board session on maternity. The purpose of the agenda items is to collectively look forward to ensure our maternity services are configured in such a fashion that, where we

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can, such suspension of services does not happen again, despite the national problems of accessing a rural maternity workforce.

7. The plan is to spend the first public agenda item on looking forward with regard particularly to the North Devon maternity unit in Barnstaple.

8. The second public agenda item will cover a paper looking into the South West Peninsula wide impact of the Ockenden and Amos maternity reports.

9. The third public agenda item will be the first ever Customer-Experience approach review by our ICB, looking to the Customer/Patient journey mapping of our services and trying to bring the voice of the patient experience across all the “touch-points” with our services into the boardroom. This first review of customer journey mapping will be focused on our maternity services.

Looking to the future – merger / legislation

10. It is expected during the month of September that NHS England will make their announcement on whether ICBs across England are to merge in April 2027 or in April 2028. We also await the final details of the emerging Health Act (replacing the Health & Care Act 2022). Therefore, in the private session of the board we will begin the discussion of what this may mean for the running of the ICB boards and their constitution from April 2027.

11. We are also considering how, we as a commissioner, delegate to neighbourhood delivery entities (e.g. multi neighbourhood providers (MNPs)) so that local delivery can be done in such a fashion at community level that it is inclusive of a range of delivery partners from NHS trusts to primary care to councils to VCSE and other partners for each place.

12. Regardless of whether we merge or not, it is now highly likely ICB Board changes will have to be made from April 2027 due to the Health and Care Act 2022 being repealed and the introduction of a new Health Act.

13. We will be looking as a result of the introduction in the autumn of this year of the Cornwall Council strategic authority – and looking at some point in 2027/28 towards the election of a Devon wide Mayor – we need to reconsider the way in which the NHS and social care/councils works across the South West Peninsula and particularly with a view to the wider determinants of health e.g. housing, jobs, education etc.

14. I have therefore taken the time to meet (in small discussion groups) with virtually all the NHS CIOS and NHS Devon ICB board members about key ICB board governance proposals for this board meeting in private. I have arranged to meet Devon Council leaders and Council CEOs at their request, as they would wish to meet together with myself and the ICB chief executive & ICB Chief Governance Officer in a plenary session, on 4 November 2026.

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Your Voice: Building a Better Future for Maternity and Neonatal Care

15. I would like to encourage colleagues and partners to support Your Voice: Building a Better Future for Maternity and Neonatal Care, an NHS-wide conversation that opens on 21 September 2026 and will run for three weeks. This follows the publication of the Independent National Maternity and Neonatal Investigation led by Baroness Amos and is designed to ensure that the experiences, insights and expertise of frontline staff help shape the future of maternity and neonatal services.

16. Your Voice is an NHS-wide opportunity for an open conversation for all maternity and neonatal staff to help shape real and sustainable change in services. Building on feedback from more than 9,000 staff who contributed to the national investigation, the conversation aims to gather the frontline intelligence needed to inform the National Maternity and Neonatal Taskforce's action plan and support meaningful improvements for women, babies, families and staff. Participants will be able to contribute anonymously and in their own time, helping to ensure future decisions are grounded in the realities of delivering care.

17. As maternity services are a key focus of this Board meeting, I encourage all organisations across our system to support participation and promote this important opportunity. The stronger the contribution from our local maternity and neonatal workforce, the better placed we will be to influence national discussions and ensure future improvements reflect the needs, experiences and aspirations of both staff and the communities we serve. Your Voice communications toolkit - soft launch updated 15092026

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board meeting in public

Chief Executive's report

Date of meeting	Date report produced
24 September 2026	10 September 2026

Author(s)		Report approved by	
Name and title:	Sam Cush, Communications Manager	**Name and title:**	Susan Bracefield, Acting Interim Chief Executive

Executive summary
This report provides an update from the NHS Cornwall and Isles of Scilly and NHS Devon Integrated Care Board Cluster Chief Executive Officer.

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

To which Board Assurance Framework risk does the report relate?
N/A

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Regulation and Legal Requirements	N/A
Information Governance (DP and Cyber)	N/A
Transformation and Innovation	N/A
Environment and Climate Change	N/A

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Chief Executive's report

North Devon District Hospital maternity services

1. Births at North Devon District Hospital (NDDH) were temporarily suspended by the Royal Devon University Healthcare NHS Foundation Trust (RDUH) on Tuesday 11 August, due to unsafe levels of medical staffing.

2. The temporary suspension, which is despite extensive efforts to maintain services, affects births and face to face triage only.

3. Other services including community midwifery, scans, birth planning, ultrasound clinics and antenatal outpatient clinics will remain in Northern Devon.

4. The Trust has contacted more than 300 women and birthing people who are 34 weeks pregnant or more, as well as those identified as vulnerable or clinically high risk, to discuss their individual circumstances and support available.

5. A dedicated helpline for pregnant women and birthing people is available – 01271 335 773 (9am-5pm, Monday – Friday) and a 24/7 maternity triage line for those who require urgent medical advice – 01392 406 616.

6. The Trust has made progress on recruiting to consultant and registrar rotas, however they still have further work to do to meet thresholds and to complete the comprehensive onboarding and induction programme all new staff must undertake.

7. Clinical and operational assurance confirmed that the trust has not been able to meet the reopening threshold rota coverage to reopen birthing services safely.

8. Despite this latest position, the trust remains committed to reopening birthing services at NDDH as soon as possible. They will undertake their next readiness assessment on Monday 28 September 2026 and, if needed, every two weeks after that.

9. The trust's maternity web pages are being updated regularly with updates and answers to frequently asked questions.

Winter planning

10. NHS England requires every integrated care board (ICB) to develop a robust, clinically led and deliverable winter plan, tested through regional exercises and approved by local boards before submission by 30 September 2026.

11. Regional NHS England guidance is for separate plans for Cornwall and Isles of Scilly and Devon, supported by a cluster-level cover paper.

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12. An executive winter lead and senior responsible officer have been appointed, and a task-and-finish group is coordinating plan development across the cluster.

13. Provider organisations have submitted draft winter plans and are refining operational arrangements with local partners. ICB plans are expected to be substantially complete by September.

14. The plans aim to:
    a. Establish clear escalation triggers and response actions
    b. Define exceptional measures for periods of extreme pressure
    c. Strengthen governance and rapid decision-making processes
    d. Improve collaboration across providers, places, neighbourhood teams and primary care
    e. Enhance admission avoidance and care closer to home
    f. Ensure clear accountability and command-and-control arrangements across the system.

15. A key theme is shifting from reactive operational management to proactive, intelligence-led demand management. This approach aims to:
    a. Identify emerging pressures earlier
    b. Support proactive intervention through the system co-ordination centre
    c. Improve system-wide coordination and resilience
    d. Allow the ICB to maintain its strategic commissioning role rather than day-to-day operational flow management.

16. Following local and regional reviews, the final plans and Board assurance statements are now presented to the Board on 24 September 2026 for approval.

System operational and financial performance

17. The performance exception report sets out key challenges within the Devon and Cornwall system in relation Urgent and Emergency Care, Referral to Treatment, Cancer 62 day performance and diagnostics treatment which are being actively addressed by providers ahead of winter

18. Whilst Devon and Cornwall system is projecting delivery of the forecast outturn position for the 2026/27 period, there are material risks to delivery, specifically relating to provider cost improvement plans, with mitigations currently being robustly reviewed.

19. The Board will continue to maintain oversight of delivery, in conjunction with the Finance, Performance and Quality Committees.

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ICB Annual Assessments 2025/26

20. Both NHS Cornwall and Isles of Scilly and NHS received their 2025/26 Annual Assessments from NHS England in August.

21. The assessments are based on the statutory assessment criteria set out by NHS England and the evidence available for that period including the ICBs' annual reports and accounts, the NHS Oversight Framework (NOF) contextual metrics and year-end data for 2025/26, feedback from partners including local Health and Wellbeing Boards (HWBs), and regional oversight and assurance information.

22. The accompanying letters from NHS England recognises that the operating context has continued to evolve since the end of the assessment period, including the progression of cluster arrangements and the transition towards a stronger strategic commissioning role for ICBs. These developments provide important context but fall outside the scope of this assessment.

23. NHS England remains committed to working with us and our partners to support continued improvement, delivery and transformation as these arrangements mature.

24. High level summaries of each organisations' assessments are shown below.

NHS Cornwall and Isles of Scilly

25. NHS Cornwall and Isles of Scilly's assessment highlights positive achievements including improved elective care performance, strong delivery of learning disability annual health checks, improvements in aspects of primary care access, positive mental health outcomes in selected services and successful delivery of a planned financial breakeven position.

26. The ICB also met its planned efficiency programme, remained within agency spending limits and achieved compliance with the Mental Health Investment Standard. Governance and assurance arrangements were considered effective and the organisation continued to meet its statutory responsibilities in a complex operating environment.

27. At the same time, NHS England identifies a number of continuing pressures. Urgent and emergency care performance, ambulance handover delays, diagnostics and aspects of cancer performance remained below plan, while community services, workforce resilience and some mental health and community pathways continued to experience fragility. Significant health inequalities persist, particularly in relation to deprivation, cancer diagnosis and stroke admissions. Quality, safety and patient experience were assessed as mixed, with variation in access and outcomes across services.

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28. Overall, NHS England concludes that NHS Cornwall and Isles of Scilly has made measurable progress in several priority areas but that improvement has not yet been achieved at the scale or consistency required across the system. The assessment calls for stronger grip, sharper delivery and more sustained improvement during the year ahead.

NHS Devon

29. NHS Devon's assessment notes positive progress in several areas, including learning disability annual health checks exceeding the 75% ambition, improved bowel screening coverage, strengthened mental health performance in a number of pathways, continued development of neighbourhood-based models of care, and delivery of the ICB's planned £8 million surplus and efficiency targets. Governance arrangements were found to be robust, with a satisfactory internal audit opinion and evidence of strong partnership working across the system.

30. However, NHS England identifies significant ongoing challenges. Urgent and emergency care performance, elective recovery, diagnostics and elements of cancer performance remained below expected levels. Long-standing health inequalities persisted and, in some cases, worsened, particularly in relation to cardiovascular outcomes. Quality and safety concerns remain in some services, including children's services, neurodiversity pathways and maternity-related outcomes. Workforce engagement indicators also deteriorated during the year.

31. The assessment concludes that NHS Devon has demonstrated commitment to partnership working, statutory responsibilities and system leadership, but that progress has not yet been delivered at the pace or consistency required across all priority areas.

32. NHS England's expectation is that NHS Devon demonstrates stronger delivery, sustained improvement and reduction in the risk of future formal intervention during 2026/27.

Readiness for planning rounds

33. Planning principles for 2027/28 have been developed and are included within the September Board papers. The current working assumption is a flat cash settlement for providers.

34. Development of the 2027/28 Commissioning Intentions is progressing, with work underway to align priorities with population need, financial sustainability, strategic commissioning objectives, quality outcomes and patient experience.

35. A detailed planning timetable is being developed. Current indications are that national planning submissions are likely to be required during December 2026 and March 2027, although final guidance is awaited.

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36. Planning will be undertaken at both system and place level, ensuring consistency with overall financial and strategic objectives whilst reflecting local population and service needs. Quality metrics, patient experience insights and population health intelligence will inform the planning process throughout.

37. Work is underway to align planning, finance, performance, quality, and commissioning processes to support a more integrated and deliverable plan.

38. Early focus is being given to identifying the key transformation, productivity and service redesign opportunities required to deliver a financially sustainable plan whilst maintaining and improving quality and outcomes.

39. Planning assumptions and risks will continue to be refined as national guidance emerges, with regular updates provided to the Board.

40. The ICB Planning Team will lead the development of the 2027/28 plan, supported by a multi-disciplinary team drawn from across the organisation's directorates. This planning structure will be stood up from mid-September to coordinate planning activity, ensure organisational alignment and support delivery of a robust and integrated system plan.

Devon neighbourhood health engagement

41. I am delighted to share that we have launched a Devon-wide conversation on neighbourhood health this month to help shape the future of health and care. Building on the Devon 10 Year Plan engagement last year, we want to hear from staff, patients and local communities to understand their experiences and views on how health and care services can work better together.

42. The national vision for neighbourhood health, set out in the 10 Year Health Plan for England and national Neighbourhood health framework, is to bring together GPs, community services, mental health, social care, voluntary organisations and local partners to provide more joined-up support in local communities. This aims to help people stay healthier for longer, access more care closer to home, reduce the need for hospital treatment where appropriate, and ensure services are shaped around the needs of local communities.

43. Creating a neighbourhood health service is not something the NHS can do alone. It depends on health services, social care, local councils, community groups and local people working together to improve health and wellbeing. That's why organisations from across Devon are supporting the conversation and encouraging people to get involved.

44. Residents can take part in the 'Let's talk Neighbourhood health' engagement programme by completing a survey, going along to an event, or joining a local feedback group or the new Devon People Panel, among other opportunities, to make sure their voice, previous experiences and opinions are heard.

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45. The engagement events are taking place across the country throughout September and October:
    - Friday 11 September – Plymouth Leadworks
    - Wednesday 23 September – Tavistock Town Hall
    - Friday 25 September – Paignton Library
    - Monday 28 September – Exeter Community Centre
    - Monday 19 October – Roundswell Community Centre, Barnstaple.

46. I would encourage everyone to get involved, share their views and help shape the future of health and care in Devon. More information is available on our website.

Senior leadership appointments

47. We are pleased to confirm four appointments to our senior leadership structure, which means the majority of posts are now filled.

48. Molly Marcu has been appointed as our permanent chief governance officer, having been undertaking a similar role on an interim basis since the end of June.

49. Mark Sowden and Jason Brooke have been appointed as directors of workforce strategy, planning and change in the chief people officer directorate. Mark and Jason have worked in similar roles for NHS Devon and NHS Cornwall and Isles of Scilly respectively for several years and therefore bring great expertise and leadership to the posts.

50. Richenda Mason has been appointed as our new medical director (regulation and transformation). Richenda has extensive experience, most recently as medical director for Practice Plus Group in Plymouth. She also held a deputy director role at Royal Cornwall Hospitals NHS Trust.

51. Congratulations to Molly, Mark, Jason and Richenda.

52. There are five remaining unfilled posts, of which recruitment is underway for four of them – chief pharmacist, director of commissioning, place director for Cornwall and Isles of Scilly, and place director for south Devon.

53. The process for the fifth and final post, director of nursing and allied health professionals (operations and transformation), is ongoing.

ICBs restructure update

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54. We continue to make progress in implementing the nationally required changes to Integrated Care Boards (ICBs), which require all ICBs to reduce running costs and strengthen their role as strategic commissioners focused on improving population health outcomes and reducing health inequalities.

55. I am pleased to share that we have begun implementing our new staff structure, following an extended consultation period on the People Impact Assessment (PIA).

56. The extended consultation, which ran from 15 – 29 July, provided colleagues with the opportunity to:

    a. Raise any questions, queries or challenges relating to their proposed PIA outcome
    b. Request consideration under acting up/secondment arrangements where individuals had been acting up or undertaking an internal secondment for 12 months or more (as at 15 July 2026), in line with the agreed organisational change principles
    c. Raise any outstanding structural queries relating to statutory functions.

57. In total, 236 pieces of feedback were received, all of which was carefully reviewed and considered by the people teams and relevant senior managers.

58. The process has involved constructive challenge, detailed discussion and a collective effort to balance a range of factors, including organisational effectiveness, affordability, feedback from colleagues and recognised trade unions, and the need to establish a clear and coherent structure for the future.

59. Whilst no structure of this scale and complexity will meet every expectation, we believe it represents the most balanced and considered position possible, considering the breadth of feedback received and the constraints that we are operating within.

60. We recognise that this has been a difficult period for our colleagues. As we move into the implementation phase, we remain committed to working openly and collaboratively to ensure colleagues feel informed, supported and treated fairly, regardless of individual outcomes.

New Cornish film encourages people with diabetes to protect their foot health

61. NHS Cornwall and Isles of Scilly, working in partnership with Cornwall Partnership NHS Foundation Trust (CFT) Podiatry Services and members of the Long-Term Conditions Expert Reference Group, has launched a new educational video to help people living with diabetes look after their feet and prevent serious complications.

62. The new film, Happy Feet: Small Steps Make a Big Difference, highlights the importance of daily foot care and encourages people to seek help early if they notice any changes or concerns.

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63. Diabetes can affect circulation and reduce sensation in the feet, making it harder to notice injuries and slowing the body's ability to heal. Without prompt treatment, seemingly minor problems such as cuts, blisters or infections can develop into more serious complications. Diabetes remains one of the leading causes of lower limb amputations, many of which can be prevented through good self-care and early intervention.

64. While the number of people with diabetes in Cornwall has risen in the last decade by 44% (from 26,000 to 37,500), the number of diabetic amputations has reduced by 22% (from 134 to 104).

Community Gateway and Community Hubs Shortlisted for the 2026 HSJ Awards

65. I am pleased to share that our trailblazing Community Gateway and Hub Network (CGHN) has been shortlisted for the prestigious Health Service Journal (HSJ) Awards 2026.

66. We and healthcare volunteering charity Helpforce, along with local VCSE partners, have gained national recognition for the development, evaluation and promotion of the county's trailblazing Community Gateway and Hub Network (CGHN).

67. The CGHN – a joint ICB project with healthcare volunteering charity Helpforce, along with local voluntary, community and social enterprise (VCSE) partners – has been shortlisted in two categories: Excellence in Narrowing the Health Inequalities Gap and Neighbourhood and Community Care Innovation of the Year.

68. The programme brings together NHS Cornwall and Isles of Scilly Integrated Care Board, Volunteer Cornwall, Age UK Cornwall and Isles of Scilly, Cornwall Voluntary Sector Forum and Cornwall Council, more than 50 community hubs and wider local voluntary and community sector organisations and social care partners. These include Pentreath, Cornwall Neighbourhoods for Change, The CHAOS group and Cornwall Rural Community Charity.

69. Together, partners have developed a county-wide model that helps people access practical, social and wellbeing support closer to home. The Community Gateway helpline is open 8am to 8pm, 365 days a year, while community hubs provide trusted local spaces where people can connect, get advice, take part in activities and access wider support before needs escalate.

70. Independent evaluation by Helpforce, found the programme supported 340,000 attendances in 12 months, with 88% of people reporting they felt less isolated. Confidence in managing health rose from 59% to 76%, and 99% of beneficiaries said their expectations were met or exceeded. Analysis also estimated more than £11.6 million avoided NHS costs over a 12-month period through enabling people get the right support earlier and closer to home.

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South West Peninsula Board Meeting in public

Local Authority Partner Member Update for Cornwall and the Isles of Scilly

Date of meeting	Date report produced
24 September 2026	4 September 2026

Author(s)		Report approved by	
Name and title:	Alison Bulman, Strategic Director for Safe and Caring Communities, Cornwall Council	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
Key highlights include: Period 1 performance is positive in several areas, while focused action is needed to widen choice, strengthen provider quality and sustain capacity across the five continuous-improvement themes.

Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

Key recommendations and actions requested
NOTE the content of this report.

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Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	
Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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Local Authority Partner Member Update for Cornwall and the Isles of Scilly

Introduction and Background

1. This report provides an update from the perspective of the Cornwall and Isles of Scilly Local Authority Partner Members of the South West Peninsula Board.

2. For the purposes of clarity, the Local Authority members are Cornwall Council and Council of the Isles of Scilly.

3. The content of this report focusses on the areas of most significance in terms of key performance measures for each organisation as well as key achievements and events from the last period. The report also highlights any emerging issues or risks from the perspective of the Local Authorities.

Access and choice

4. Direct Payment uptake reached 28.0% in July, just below the 29% ambition, while uptake among carers reached 80.1%, slightly above target. Clearer guidance, the Kernow Wallet and Directory, and the new carers service aim to widen choice and control.

Safeguarding

5. Risk was reduced or removed in 87.8% of concluded Section 42 enquiries, just above the 87.5% target but below the latest national average. Training, audit, better outcome recording and review of cases where risk remains will support improvement.

Prevention and independence

6. Care-home admissions remain well below target for younger and older adults. A further 68.2% of people receiving long-term support live at home or with family, above the 64% target. Reablement, supported living and home-based care will help sustain independence.

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Health and care integration

7. Emergency admissions among people aged 65 and over, and delays after patients are ready for discharge, compare favourably with wider benchmarks. Ageing, demand growth and limited community capacity remain risks. Better Care Fund activity will support prevention, discharge and recovery at home.

Workforce and provider quality

8. Workforce turnover improved from 26.7% to 20.2%, outperforming national, regional and peer comparisons. Provider quality is weaker: 79.1% are rated Good or Outstanding, below England at 81.7% and the South West at 86.1%. Workforce delivery and provider assurance remain priorities.

Governance and outlook

9. The framework connects performance, audit, risk, feedback and transformation delivery across all five themes, supporting an “always ready” approach. The direction is positive, but several measures are provisional, annual or published in arrears, so trends need continued scrutiny.

Recommendations

10. The South West Peninsula Board is asked to **note** the content of this report.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

NHS Trusts and Foundation Trusts Partner Member Update for Devon

Date of meeting	Date report produced
24 September 2026	16 September 2026

Author(s)		Report approved by	
Name and title:	Sam Higginson, Chief Executive Officer, Royal Devon University Healthcare NHS Foundation Trust	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
This report provides an update from the perspective of the One Devon Integrated Care System NHS Trust and Foundation Trust Partner Members of the South West Peninsula Board.

Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

Key recommendations and actions requested
NOTE the contents and let us know if they require further briefing on any of the items within.

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Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	
Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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NHS Trusts and Foundation Trusts Partner Member Update

Introduction and Background

1. This report provides an update from the perspective of the One Devon Integrated Care System NHS Trusts and Foundation Trusts Partner Members of the South West Peninsula Board.

2. For the purposes of clarity the NHS Trusts and Foundation Trusts members are Devon Partnership NHS Trust, Royal Devon University Healthcare NHS Foundation Trust, Torbay and South Devon NHS Foundation Trust, University Hospitals Plymouth NHS Trust and South Western Ambulance Service NHS Foundation Trust.

3. The content of this report focusses on the areas of most significance in terms of key performance measures for each organisation as well as key achievements and events from the last period. The report also highlights any emerging issues or risks from the perspective of the NHS Trusts and Foundation Trusts.

Performance and planning

4. Significant focus remains across the system on delivering to our financial and elective plans in a challenging national context, and under sustained operational pressure. Many organisations both regionally and nationally, have faced a challenging period operationally because of the extended period of exceptionally warm weather across the Summer which has included separate red and amber alerts for heat, and six heatwaves. These have both contributed to and combined with sustained high levels of urgent care demand to place sustained pressure on services.

5. As a health system we are working towards finalising our collective winter plans to support patients and teams into the next period of expected demand.

6. We are working together to maximise opportunities to support shared communications, including media and social media activity, to help encourage the public to make informed choices about accessing healthcare and using services appropriately.

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The Royal Devon

7. Good progress continues to be made towards our **financial recovery** with our reported position in line with the plan trajectory signed off with NHS England and delivery of our savings programme. The trajectory of improvement increases in the second half of the year to achieve a breakeven position and risk of circa £14m remains against the yearend forecast but we are continuing to work on mitigation plans. We are currently working on the actions needed to ensure we continue to deliver what we said we would deliver and build further recovery actions to ensure we have options to manage the residual risk.

8. A meeting is being held with NHS England during September where we will be presenting the forward trajectory and reaffirming our continued commitment to financial recovery whilst protecting operational performance.

9. On the basis of performance in Q1 2026/27 the Royal Devon has been assessed as successfully moving from **Oversight Framework Segment 3 in Q4 2025/26 to Segment 2**, following publication by NHS England on Friday 11 September. Whilst recognising the number of changes made nationally to the metrics within the framework for 2026/27, the Trust's performance across a number of individual domains including in relation to finance, patient safety, and people have each been assessed as having materially improved. The Trust's position within national league tables has accordingly improved by 54 places, from 86 out of 134 Trusts nationally at the end of Q4 2025/26 to 32 out of 134 Trusts at the end of Q1 2026/27.

10. The Trust is reviewing both the progress made to date as well as the challenge remaining to deliver our operational plan commitments by the end of March 2027. Following recent agreement with commissioners of both the Trust's Urgent Care Plan, and of the necessary actions to address the elevated levels of referral growth experiences in Q1, the Trust is refocussing on actions to drive delivery of these commitments.

Torbay and South Devon

11. Torbay and South Devon continues to operate in a highly challenging financial and operational environment. Delivery of the 2026/27 financial plan depends on managing risks of more than £10m, while performance across Referral to Treatment pathways and urgent and emergency care is materially below the commitments made at the start of the year.

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12. In response, the Trust is reviewing its delivery approach and refocusing its Plan for Better Care priorities on the areas requiring the greatest improvement. These will be supported by detailed recovery plans, strengthened oversight and regular reporting through the monthly performance report. Planning arrangements for 2027/28 have also commenced.

University Hospitals NHS Trust

13. **System oversight and improvement**: University Hospitals Plymouth NHS Trust (UHP) has improved its rating and is now in NHS Oversight Framework (NOF) Segment 3 following the Quarter 1 2026/27 update (formerly Quarter 4). The Trust has also improved its national position from 105/134 acute trusts to 92/134. Average metric score of 2.59. This improved from 2.64 in Quarter 4. This reflects an improvement across a range of performance measures and demonstrates continued progress over the period.

14. **Financial target:** following discussions with NHSE and the ICB, the UHP Board approved a revised breakeven plan at its May 2026 meeting. The overall breakeven plan is now based on a CIP target of £91.6m (c8% of turnover). At month 5 the Trust is on plan for the year with a deficit position of £19.3m to date but faces significant delivery risks around achievement of the year end breakeven position. These are primarily around CIP delivery of the full plan and an impact of a higher than planned run rate going into 2026-27. The Trust has established a Financial Recovery Plan and Board to oversee actions to secure the delivery of the plan. These actions include increased workforce controls and CIP deliver assurance actions.

15. **Improving quality of care:** UHP continues to develop and deliver a focused quality improvement programme centred on patient safety, maternity improvement, reducing corridor care and improving patient flow. Following publication of the national Maternity and Neonatal Investigation report, the Trust Board have approved the self-assessment and subsequent actions within maternity services with the insight meeting with NHSE scheduled for October. Recent CQC inspections across a number of core services are underway and positive initial feedback from the Radiotherapy IRMER inspection provide assurance regarding the quality of care being delivered across the organisation.

16. **Regulatory inspection:** The Care Quality Commission carried out an unannounced inspection of the following core services on Wednesday 26 and Thursday 27 August.
    1. Children and Young People
    2. End of Life Care (including Mount Gould Hospital)
    3. Critical Care
    4. Surgery (action plan follow up)

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17. The outcome report is awaited.

SWAST – Devon

18. The South Western Ambulance Service continues to work closely with ICB colleagues and wider system partners, as the demand and pressures continue heading towards the winter months.

19. In July 2026, for Devon, the South Western Ambulance Service experienced significantly higher demand compared with July 2025, driven in part by seasonal population growth and increased visitor numbers. Total incidents rose from 21,939 to 23,496 (an increase of 7.1%).

20. Despite higher demand, performance improved through greater use of alternative care pathways. Hear & Treat activity increased from 15.84% in July 2025 to 22.7% in July 2026 (an increase of 6.9%), enabling more patients to be managed remotely and reducing demand for face-to-face responses. As a result, the proportion of patients conveyed to Emergency Departments fell from 43.4% to 39.6%.

21. Category 2 response performance remained challenged by sustained high demand; however, increased clinical assessment, greater use of Hear & Treat, and higher referrals to community, GP and specialist services helped mitigate pressures and target frontline resources towards patients with the greatest clinical need.

One Devon EPR

22. The One Devon EPR programme celebrated a major milestone in July with the successful Epic go-live at University Hospitals Plymouth on 23 July. The Royal Devon went live in Eastern services in 2020 and Northern services in 2022.

23. Torbay and South Devon NHS Foundation Trust went live in April this year and, with UHP now live, staff across all three organisations can access patient health and care information quickly and securely through a single electronic patient record.

24. The focus of the programme has now transitioned from implementation to optimisation. Teams across the three trusts are working collaboratively to refine workflows, embed new ways of working and realise the full benefits of the shared system. The ability for clinicians to access patient information quickly and securely across organisational boundaries is already improving joined-up care and supporting safer, more efficient decision-making.

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25. Patient adoption of MY CARE, Epic's patient portal, continues to grow across Devon and the portal is enabling patients to access key health information, view appointments and engage more actively in their care.

Royal Devon University Healthcare

26. **Birthing services at North Devon District Hospital (NDDH)** continue to be suspended and will not reopen on Tuesday 29 September as previously anticipated.

27. Despite extensive efforts on a national recruitment campaign, an assessment of staffing levels and rota coverage today confirmed we have been unable to meet the agreed staffing threshold needed to safely reopen services later this month.

28. We remain fully committed to reopening birthing services at NDDH, but we will only do so when we can be confident the service is safe, sustainable and not at risk of closing again at short notice.

29. The Trust will undertake the next readiness assessment on Monday 28 September and, if needed, every two weeks after that.

27. **Specialist neuro-rehabilitation service recognised for outstanding care**: the Royal Devon has welcomed the findings of the latest CQC inspection of Mardon Neuro-rehabilitation Centre, which has confirmed the service continues to be rated Good overall, with an Outstanding rating for Caring.

28. The specialist inpatient service supports people recovering from stroke, acquired brain injury and other complex neurological conditions, helping patients maximise their independence and quality of life through intensive multidisciplinary rehabilitation.

29 The assessment highlighted the expertise and dedication of staff and found patients and families consistently received compassionate, person-centred care. Inspectors heard examples of staff going above and beyond to support patients and carers, building strong relationships and tailoring rehabilitation to individual needs and goals.

30. **Pathology service redesign**: separate programmes of work have been ongoing at the Royal Devon and Torbay and South Devon (TSDFT) trusts to address longstanding estate constraints and associated safety risks in pathology services.

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31. As part of the first phase of the transformation programme, the Royal Devon Blood Sciences and Microbiology services are scheduled to move to the new CSL at Gadeon House in summer / autumn 2027 and the plan was to maintain services across both acute sites until this time.

32. Following the recent Royal Devon voluntary redundancy scheme, and with service viability and patient safety as a priority, it has become necessary to move Microbiology services at NDDH to RD&E (Wonford) from Monday 2 November.

33. Consolidating the service on one site will support workforce resilience and enable the service to operate more effectively and Royal Devon clinicians, GP practices and patients should notice little or no difference to the service they receive.

34. **Investment and improvements at NDDH:** the Royal Devon is investing in improvements to urgent and emergency care services across northern Devon to help more patients receive timely care, closer to home where appropriate and in the setting best suited to their needs.

35. As part of a wider programme to improve urgent care services a new, expanded same day emergency care (SDEC) facility at North Devon District Hospital is being developed, funded through external capital investment from NHS England, to enhance urgent care services at NDDH and help ensure they remain fit for the future.

36. Alongside this we have received funding from NHS England to deliver the follow improvements at NDDH:

    - £1mil to create additional treatment space in the emergency department for patients with minor injuries and illnesses, helping to reduce waiting times and improve patient flow
      £2mil capital investment to upgrade our electrical infrastructure to future proof NDDH, supporting planned expansions such as the single MRI and the NIHR research hub.

37. **Heavitree surgical hub taking shape**: work on a new surgical hub at Heavitree is moving up a level, with floor two construction now well underway. £20m of NHS England funding specifically for projects that help trusts reduce waiting lists, lift productivity and recover performance to pre-pandemic levels has been secured to help build the hub.

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38. Once complete, it will have two new theatres specialising in high-volume, medium-complexity day surgery, and extra recovery space. It will also increase surgical capacity, allowing us to see an extra 3,000 patients a year, while freeing up theatre capacity in Wonford to focus on higher-complexity and urgent surgery.

39. **Trust-wide supported internship programme**: the Royal Devon is bringing its supported internship offer together into one Trust-wide programme, delivered in partnership with Exeter and North Devon Colleges Group.

40. The programme builds on established approaches, including the highly successful Project SEARCH initiative. Project SEARCH supported the training and development of many young people with an Education, Health and Care Plan, and 75% remain employed by the Trust. The programme strengthens our commitment to creating meaningful routes into employment for autistic young people and young people with learning disabilities.

41. **Lord Mayor of Exeter and HM Lord Lieutenant Devon visits:** we've been honoured to welcome Gemma Rolstone, Lord Mayor of Exeter and David Fursdon, Lord Lieutenant Devon to RD&E (Wonford) during August and September. Both met with Sam Higginson, Chief Executive Officer and the Lord Mayor also met Emergency Department colleagues in recognition of 999 Emergency Day.

42. **Cancer patient experience:** the Royal Devon ranked 11th equal with Mersey and West Lancashire Teaching Hospitals NHS Trust. Patients gave the Royal Devon an average rating of care of 9.1 out of 10. Of the 59 questions, the Trust scored above the expected average for 35, an increase of 11 questions compared to the 2024 survey. No questions scored below the expected range.

Torbay and South Devon

43. Improving urgent and emergency care and eliminating corridor care remain among the Trust's highest operational priorities. A national Getting It Right First Time review in August identified priorities including a GP-led Urgent Treatment Centre within the Emergency Department, stronger front-door streaming, better use of frailty services, improved length of stay and strengthened clinical and operational leadership. The review team will return in October to assess progress and winter readiness. There is also a need to continue to work with partners including the ICB and SWAST in relation to very high levels of demand.

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44. The **Adult Social Care transition programme** continues to progress across governance, workforce, finance, operational planning and enabling workstreams. The Trust, Torbay Council and NHS Devon are working together to support a safe and orderly transition to new arrangements from 01 April 2027, with continuity of care and support for colleagues remaining central to the programme.

45. **Estate resilience, fire safety and emergency preparedness** remain key strategic priorities. Recent incidents at Torbay Hospital, Allerton Ward and the former nurses' home at Teignmouth Hospital, together with a major multi-agency evacuation exercise and wider fire safety reviews, have reinforced the importance of investment, effective risk management, regular training and continuous learning. Debriefs have highlighted the professionalism of colleagues and identified further improvements that are being taken forward.

46. The Trust continues to make progress across its **capital programme**. The new day theatre opened in July, refurbishment of existing day theatres is underway, and work continues to expand oral and maxillofacial surgery, modernise cardiac catheter laboratories and improve maternity facilities. The Cove, the newly refurbished birthing suite at Torbay Hospital, is now open and includes a new birthing pool and dedicated accessible wet room.

47. Work continues on the proposed **relocation of Cellular Pathology services** to Gadeon House in response to significant estates and infrastructure challenges in the current laboratory accommodation. The associated business case and engagement programme are progressing. Torbay and South Devon and Royal Devon are also exploring whether co-location could create opportunities for future collaboration, but no decisions have been taken.

48. **Neighbourhood working** continues to gather momentum across Torbay and South Devon. The South Local Care Partnership has approved an Action Learning Set approach to help partners turn shared ambitions into practical action, while the Trust is supporting Devon ICB's Let's Talk Neighbourhood Health engagement and running its own neighbourhood conversations as part of the Annual Members' Meeting on 30 September.

News in brief

49. For the fourth consecutive year, Torbay and South Devon has been recognised among the best performing trusts nationally for cancer patient experience. Patients rated their overall experience at 9.1 out of 10, with no areas below the expected national range.

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50. The 2025 Adult Inpatient Survey showed an improvement in overall experience from 8.2 to 8.4 out of 10. Results across all 51 questions were in line with the national benchmark, with positive feedback on dignity, respect, compassion, ward cleanliness and confidence in clinical staff.

51. The Radiotherapy Department has retained ISO 9001:2015 accreditation following an external assessment, with no new non-conformities identified. The Trust has also achieved reaccreditation as an NHS Veteran Aware organisation.

52. The Trust has secured £400,000 from the Torbay Medical Research Fund to provide three pre-doctoral and three doctoral fellowships over the next three years. A further £88,000 of National Institute for Health and Care Research funding is strengthening pharmacy aseptic services and clinical trials capacity.

53. The Trust has launched a Greener Torbay Action Group to coordinate environmental improvement activity and has appointed an Energy and Decarbonisation Manager to support the programme.

University Hospitals Plymouth

54. One step closer to delivery of a new **Emergency Care Building**: UHP marked a significant milestone in the construction of its new Emergency Care Building (ECB), with partners, staff and contractors coming together to celebrate the building reaching its highest point at the start of September. The traditional "topping out" ceremony took place at Derriford Hospital, recognising the progress made on one of the Trust's most important infrastructure projects.

55. The milestone signifies the completion of the building's main structural framework and marks the transition to the next phase of construction. Due for completion in April 2029, the Emergency Care Building will provide 4 clinical floors of modern, purpose-built facilities designed to support the delivery of urgent and emergency care services for patients across Plymouth, South Devon and Cornwall. The development forms a key part of the Trust's ambition to improve patient experience, enhance clinical environments and support future healthcare needs for the growing population it serves.

56. **AGM and Careers Showcase**: UHP will hold a Careers Showcase alongside its Annual General Meeting (AGM) on Saturday 26 September. Plymouth continues to face challenges around youth unemployment, with rates remaining higher than many areas of the country. The Careers Showcase aims to help demonstrate the breadth of opportunities available locally and highlight the rewarding careers that exist within the NHS in a bid to inspire local people, particularly younger people thinking about potential careers.

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57. From the traditional routes of nursing, midwifery and medicine, to pharmacy, research and development, healthcare sciences, administration, estates, digital and other support roles, visitors will be able to meet staff from across the Trust and learn about different routes into employment, training and apprenticeships. More information online

Recommendations

58. The South West Peninsula Board is asked to **note** the contents and let us know if they require further briefing on any of the items within.

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South West Peninsula Board Meeting in public

Providers of Primary Medical Services Partner Member Update for Cornwall and the Isles of Scilly

Date of meeting	Date report produced
24 September 2026	10 September 2026

Author(s)		Report approved by	
Name and title:	Dr Smitesh Patel, Primary Care Medical Services Partner, GP	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
This report provides an update from the perspective of the providers of Cornwall and Isles of Scilly Integrated Care System Primary Medical Services Members of the South West Peninsula Board. Key highlights include: • General practice continues to navigate the 2026/27 contract implementation and BMA collective action, supported by Kernow Health CIC (KHCIC) and the GP Collaborative Board in managing OPEL status and resilience. • Wide variations in same-day urgent appointment data (sitting at a system average of 75.4% against the 90% target) are being actively addressed through standardised Ardens mapping guidance.

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- Dental contract reforms introduce an 8.2% urgent care mandate alongside recent contract awards, golden hellos, and the decommissioning of the unreliable central dental waiting list.
- Community pharmacy and optometry are expanding their clinical footprint through Pharmacy First, digital electronic referral pathways (DERS), and active integration within Integrated Neighbourhood Teams (INTs).

Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

Key recommendations and actions requested
NOTE the content of this report.

Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	
Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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Providers of Primary Medical Services Partner Member Update for Cornwall and the Isles of Scilly

Background

1. This report provides an update from the perspective of the Cornwall and Isles of Scilly Integrated Care System providers of primary medical services member of the South West Peninsula Board.

2. For the purposes of clarity the providers of primary medical services are primary medical services contract holders responsible for the provision of essential services, within core hours to a list of registered persons for whom the NHS Cornwall and the Isles of Scilly Integrated Care Board has core responsibility.

3. The content of this report focusses on the areas of most significance in terms of key performance measures for each organisation as well as key achievements and events from the last period. The report also highlights any emerging issues or risks from the perspective of the providers of primary medical services.

Introduction

4. This paper provides an update on primary care developments in Cornwall and the Isles of Scilly since the July 2026 report. It is structured around the four primary care pillars: general practice, dentistry, community pharmacy, and community optometry. It also reflects national NHS changes and local system developments affecting primary care medical services, while recognising the need to protect and properly resource all parts of primary care rather than general practice alone. The emphasis is on recent changes, successes, and areas requiring further development

General Practice

5. As the ICB transitions to a more strategic role, it increasingly relies on established providers to deliver enhanced functions. In Cornwall and the Isles of Scilly, general practice benefits from a well-established GP Collaborative Board and a scale provider, Kernow Health CIC (KHCIC), which supplies infrastructure and coordinates resilience support, demand-capacity initiatives, primary care hubs, and patient risk stratification. Furthermore, KHCIC is coordinating OPEL status reporting to support struggling practices and deploy available workforce capacity.

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6. Implementation of the 2026/27 GP contract continues alongside ongoing collective action, digital service expansion, and evolving national access guidance. BMA collective action remains live, focusing on refusing unsupported shared care requests alongside ongoing friction over data sharing. Meanwhile, national consultations on neighbourhood contract models ran through September 2026.

7. GP Appointments Data (GPAD), online consultation metrics, and telephony data to understand demand. However, significant data quality issues persist regarding appointment mapping.

8. Under the 2026/27 GP contract, practices must ensure that patients identified as clinically urgent are managed on the same day, measured via the 'General Consultation Acute' (GCA) slot category.

9. While June 2026 integrated performance reports recorded a Cornwall and Isles of Scilly position of 75.4% (progressing toward the 90% target by March 2027), wide variations across practices (ranging from 23% to 100%) largely reflect inconsistent appointment mapping rather than true variation in clinical access.

10. KHCIC and the Collaborative Board are actively supporting practices to standardise mapping via Ardens reports and targeted guidance.

Positive Changes

- Structured resilience support deployed for practices experiencing high demand or workforce gaps.
- Enhanced utilisation of data to evaluate access, appointment mapping, and workload dynamics.
- Strengthened collaboration between neighbourhood workstreams, estates, and primary care planning.

Needs Further Development

- Standardising GPAD and appointment mapping consistency across all practices and PCNs.
- Improving workforce resilience in practices facing persistent vacancies.
- Aligning neighbourhood Key Performance Indicators (KPIs) and financial drivers with primary care delivery realities.

Dentistry

11. Dentistry remains access-constrained, particularly for routine adult NHS care and patients with complex transport or additional needs. Reform efforts continue to focus on urgent access, prevention, and outreach capacity. Mobile and outreach dental services remain visible assets for rural and coastal communities, and local commissioners are aligning dental pathways with

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neighbourhood health teams to better support vulnerable children, frailty, dementia, and end-of-life care.

12. NHS Dental Contract Reform (April 2026): Introduces a shift toward prevention and consistency, mandating an 8.2% urgent care requirement and a standard £75 Band 4 payment per urgent course of treatment. While meeting urgent demand, this presents financial and capacity rebalancing challenges for Cornwall and the Isles of Scilly (CIOS) due to previous local rates (£105) and dispersed rural geography.

13. New Procurements & Contract Awards: Four procurement exercises successfully awarded six new NHS dental contracts across the county (including locations in Truro, Newquay, Camborne, and Bodmin), with West Carclaze (St Austell) market engagement slated for September 2026.

14. Workforce & Prevention: The ICB approved 36.3 WTE Golden Hello applications (12.7 WTE currently in post). Preventative initiatives include national supervised toothbrushing programmes and Smile Together CIC delivering the Mouth Care Matters scheme in care homes.

15. Dental Electronic Referral System (DERS): Paper referrals to the community dental service ceased on 31 March 2026, followed by full system rollout to the Royal Cornwall Hospital Trust on 13 July 2026

16. Dental Waiting List Decommissioning: The ICB recommends decommissioning the unreliable Devon and Cornwall central dental waiting list—which fails to meet NHS standards, lacks regular review, and creates unrealistic patient expectations—and replacing it with clear signposting to urgent care pathways and the NHS "Find a Dentist" tool.

17. Orthodontic Contract Termination: Following serious patient safety risks and financial concerns, the ICB terminated the River Practice Ltd NHS orthodontic contract in Truro and Penzance, successfully securing an alternative provider to ensure continuity of care for active patients.

Positive Changes

- Mobile and outreach dental services are increasingly embedded in local delivery.
- Prevention remains a core pillar in strategic planning.

Needs Further Development

- Routine adult NHS access remains fragile across several localities.
- Ongoing recruitment and retention constraints limit overall capacity.
- Integration between dental services and integrated neighbourhood teams requires formalisation.

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Community Pharmacy

18. Community pharmacy continues to expand its clinical footprint through Pharmacy First, minor illness management, vaccination campaigns, and medicines optimisation. Pharmacy teams are increasingly participating in Integrated Neighbourhood Team (INT) meetings and multidisciplinary case reviews for frail patients.

Community Optometry

19. Community optometry continues to contribute effectively to urgent triage, digital electronic referrals (e-Referral Service), and disease prevention. Optometrists are increasingly engaging with INTs where visual impairment intersects with frailty and falls prevention.

Summary

20. Strategic alignment across Cornwall and the Isles of Scilly emphasises neighbourhood working, prevention, digital access, and integrated models of care.

21. System Interface Group continues to successfully bridge primary and secondary care boundaries under GP leadership and KHCIC resourcing

22. However, risks remain regarding cumulative under-investment across primary care sectors, necessitating that future neighbourhood models are adequately resourced rather than expected to absorb demand without investment.

23. General practice continues to adapt to contract and access changes, with data quality enhancement and neighbourhood integration taking priority. While dentistry, pharmacy, and optometry demonstrate steady progress, targeted investment, workforce support, and structural integration remain essential. Aligning neighbourhood KPIs, financial incentives, and estate strategies will ensure the entire primary care system is appropriately resourced to serve Cornwall and the Isles of Scilly.

Recommendations

The South West Peninsula Board is asked to **note** the content of this report.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

Providers of Primary Medical Services Partner Member Update for Devon

Date of meeting	Date report produced
24 September 2026	10 September 2026

Author(s)		Report approved by	
Name and title:	Frank O'Kelly, Primary Care Partner Member	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary

This report provides an update from the perspective of the providers of One Devon Integrated Care System Primary Medical Services Member of the South West Peninsula Board.

Key highlights include:

1. Ongoing improved productivity across most of Primary Care, month on month, year on year.
2. Devon Primary Care Collaborative (GP, Dental, Optometry, Pharmacy, and the VCSE) continues towards becoming a legal entity and plans to be a significant part of the developing Devon Healthcare system in 26/27. A further Outline Business Case is in Development.
3. The position of Devon Pharmacies as explained by the LPC should be noted.
4. The issues around the introduction of EPIC in Plymouth highlight the co-dependency of all parts of the Healthcare system and that problems in one part can and do have a significant impact on other parts

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Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

Key recommendations and actions requested
DPCC - Devon Primary Care Collaborative (MPOD + VCSE) is still working on the formation of a legal entity/ CIC. The business case is being further refined. **ONGOING SUPPORT** of the Board for this development is requested.
Noting that Neighbourhoods are the delivery model of the future healthcare system investment in them, at the scale of at least 1% of the ICB spend for 2027/28, is essential, if we are to transform and improve outcomes for the people of Devon.

Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	
Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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Providers of Primary Medical Services Partner Member Update for Devon

Introduction and Background

1. This report provides an update from the perspective of the One Devon Integrated Care System providers of Primary Medical Services of the Southwest Peninsula Board.

2. For the purposes of clarity, the providers of Primary Medical Services are Primary Medical Services contract holders responsible for the provision of essential services, within core hours to a list of registered persons for whom the NHS Devon Integrated Care Board has core responsibility.

3. The content of this report focusses on the areas of most significance in terms of key performance measures for each organisation as well as key achievements and events from the last period. The report also highlights any emerging issues or risks from the perspective of the providers of Primary Medical Services.

Devon Primary Care Collaborative (DPCC)

4. DPCC progress has stalled for the moment, in terms of creating a legal entity to go live in 2026/27, as another Outline Business Case is being progressed. Until there is clarity on how this will be funded progress is held back. There are 5 pillars making up DPCC, which are the VCSE, + MPOD (Medicine (GP), Pharmacy, Optometry and Dentistry).

5. DPCC will become a Community Interest Company (CIC) and will grow by iteration, so will deliberately be lean to start and will evolve significantly in the first three years.

6. The new delivery model of Neighbourhood Health requires a Devon wide Primary Care Organisation, at scale, which understands community delivery because that is core business.

7. If we are to rebalance the healthcare delivery model within Devon to one that can deliver Neighbourhood Health at scale this entity is needed. There has, for many years, been a power imbalance in the Devon System with Primary Care not at the table when major strategic decisions are being made. This DPCC would nullify this imbalance.

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GP Update

Access

8. 851,194 appointments were completed in July, which is a 2.8% increase on the same month last year. There were the same number of working days in July 2026 (23) compared to July 2025 (23).

9. This equates to 37,008 appointments on average per working day.

10. This equates to 658 appointments per 1,000 people in Devon, nationally this figure is 534 appointments per 1,000 people in England.

Other GP headlines

11. University Hospitals Plymouth NHS Trust (UHP) Pathology links issue: Implementation of EPIC across UHP caused a critical incident with the transmission of pathology results to UHP-facing GP practices. Commencing in August this (as at early September) is not fully resolved but being worked through. The impact on GP practices is being assessed by the ICB for resource support. There should be learning drawn from this incident.

Local Medical Committee (LMC) Update

12. August and most likely September has been dominated by the UHP transition to EPIC. This resulted in significant problems with the electronic transfer of Pathology results to General Practice within the Hospital Footprint.

13. This resulted in a transfer of risk and significant workload to General Practice, in a problem not of their making but resulted in General Practice having to sort it out. That risk has yet to be fully mitigated, and reimbursement arrangements remain outstanding with routine work impacted throughout those practices facing UHP. Both LMCs have felt it appropriate to raise this further with NHSE regionally given the prolonged impact.

14. This focus on EPIC has revealed county wide discontent with the output documents it produces which will likely become the subject of further work.

15. A significant issue remains over provision of IV bisphosphonates for Devon patients. Each acute (initially UHP, now spreading to all) has cited a lack of capacity to provide this service for eligible patients. General Practice cannot offer this to patients however they hold the risk for fragility fractures that might arise. This issue remains to be resolved.

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Dental Update

National Context

16. **N**ational dental contract reform in progress – initial changes introduced from April 2026; includes focus on urgent care with mandate (providers to deliver 8.2% of total contract value as unscheduled/urgent care) and complex care pathways (targets adults with severe, active gum disease / extensive dental decay).

Local Developments

17. NHS Devon aims to increase dental activity (routine and urgent / unscheduled) by 10% for 2026/27. So far, dental providers have delivered a total of 290,200 UDAs year-to-date (April - July 2026), which equates to an increase of approximately 1.5% from the same period in 2025/26.

18. Urgent dental care activity is increasing – for 2025/26, Devon saw a 5.4% increase from 2024/25. The first increase year on year since before 2019/20.

19. Devon has secured substantial additional NHS dental capacity through recent procurement activity (including 157,000 UDAs and 27,250 urgent and unscheduled care appointments). Devon services will go live from November 2026/27.

20. Devon continues to support recruitment and retention of NHS dentists through the Dental Recruitment Incentive (“Golden Hello”) - 8 applications have been recruited in total to date, as well as other workforce initiatives and postgraduate training support.

21. There is no update from the LDC for this report.

Improving dental provision to vulnerable populations in Devon is a strategic priority for the ICB team for 2026/27

22. Newly commissioned NHS dental contracts have an explicit requirement for providers to consider innovative service models to best cater for vulnerable populations in their respective areas.

23. NHS Devon has approved funding for a 24-month paediatric dentistry pilot with Peninsula Dental Social Enterprise (PDSE) in Plymouth, commencing in September 2026. This pilot is expected to provide around 2,400 appointments and support approximately eight hundred children, particularly those from deprived and vulnerable communities.

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24. NHS Devon is commissioning a child-friendly dental practice in North Devon, also due to commence in September 2026.

25. Plans to expand the two current services in Plymouth and Exeter providing dental care to people experiencing homelessness.

Pharmacy Update

National Context

26. The national team are in the process of updating the national Pharmacy First and Pharmacy Contraception Services to introduce prescribing into the existing pathways. Five new prescribing-only pathways for additional minor ailments within the Pharmacy First Service will also be added. NHS England is currently developing the clinical criteria, service specifications, and operational arrangements with the aim to launch Autumn 2026.

Local Developments

27. NHS Devon aims to increase the community pharmacy clinical service activity by 20% for 2026/27; Devon exceeded local targets (year-on-year increase of 40%) last year and saw a 46% increase in clinical service activity within community pharmacy in 2025/26 (includes national Pharmacy First, Contraception & Hypertension Services). Devon is currently on track for target achievement in 2026/27.

28. NHS Devon continues to fund a Community Pharmacy PCN Engagement Lead Network, managed by Community Pharmacy Devon, to support cross-sector system-working within Primary Care.

Local Pharmacy Committee (LPC) Update

29. Devon's community pharmacy network has experienced a significant reduction in capacity over recent years, with the number of pharmacies across the Devon ICB footprint falling from 222 in March 2023 to 198 currently, representing the permanent closure of twenty-four pharmacies.

30. Alongside these closures, there has been a substantial loss of pharmacy opening hours (4,000 per month) reflecting the financial and operational pressures facing contractors as they seek to remain viable while continuing to support local populations.

31. Despite this reduction in infrastructure, community pharmacies in Devon continue to deliver a high volume of clinical services; however, performance varies considerably between services.

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32. Services that are driven directly by community pharmacies, such as Pharmacy First clinical pathways, hypertension case finding and contraception services, generally perform well, demonstrating that pharmacies have capacity and capability to provide additional clinical care.

33. In contrast, services that rely on referrals from other parts of the NHS, particularly Minor Illness and Discharge Medicines Service referrals, remain significantly underutilised in Devon although both services have the potential to support the left shift to community-based services, improved outcomes for patients and reduction in readmissions to hospital.

34. The key reasons for lower referral activity in Devon compared to other counties within the Southwest include limited public awareness of pharmacy services, reluctance, or inconsistency in referral practices from relevant healthcare professionals, a lack of embedded electronic referral processes, and the challenges associated with Devon's large rural geography, which can reduce access and engagement.

35. This means that the local system is failing to maximise the opportunities offered by the nationally commissioned services to encourage the left shift and build resilience within community pharmacy which remains in a fragile place.

36. The factors above suggest that the main barrier in Devon to increased activity is not pharmacy capacity as often quoted, but the need for stronger referral pathways, greater public promotion, and improved integration between community pharmacy, general practice, and other NHS services.

37. NHS England will shortly be commissioning an Independent Prescribing service from community pharmacy.

Optometry Update

38. First joint Devon & Cornwall LOC / ICB Liaison meeting to be held on 11 September 2026

39. Devon is progressing with a local enhanced service to ensure access to an NHS funded sight-test within the community, for all patients who have undergone uncomplicated cataract surgery (who do not qualify for an NHS funded sight test under the GOS contract).

40. There is no update from the Local Optometry Committee (LOC) this month.

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Recommendations

- DPCC - Devon Primary Care Collaborative (MPOD + VCSE) is still working on the formation of a legal entity/ CIC. The business case is being further refined**.** **Ongoing support** of the Board for this development is requested.

- Noting that Neighbourhoods are the delivery model of the future healthcare system investment in them, at the scale of at least 1% of the ICB spend for 27/28, is essential, if we are to transform and improve outcomes for the people of Devon.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

Mental Health Partner Member Update for Cornwall and the Isles of Scilly

Date of meeting	Date report produced
24 September 2026	30 July 2026

Author(s)		Report approved by	
Name and title:	Colm Owens, Chief Medical Officer, Cornwall Partnership NHS Foundation Trust	**Name and title:**	Adie Perry, Deputy CEO, Cornwall Partnership NHS Foundation Trust

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
The Mental Health Act reforms represent the most significant change to mental health legislation in a generation. While implementation will be phased over a number of years, national guidance is clear that successful implementation will depend upon expansion of specialist workforce groups, additional clinical capacity, stronger legal and administrative infrastructure, and significantly enhanced community provision for people with learning disability and autistic people. Discussion within Devon and Cornwall already highlights workforce pressures affecting Section 12 provision, Mental Health Act Assessment (MHAA) capacity, Responsible Clinician (RC) availability, tribunal processes and community alternatives to hospital admission. These pressures pre-date implementation and may therefore be exacerbated by the reforms. This paper proposes a system-wide approach to workforce impact assessment and planning. It argues that future workforce requirements should not be viewed solely through the lens of consultant psychiatrist numbers, but rather across three interconnected domains:

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- Mental Health Act statutory capacity (Responsible Clinicians, Approved Clinicians, Section 12 doctors and tribunal activity).
- Mental Health Act governance and administrative infrastructure.
- Community alternatives for people with learning disability and autistic people.

The Board is asked to support development of a collaborative Devon and Cornwall workforce impact model during 2026/27 to inform future commissioning, provider planning and national investment discussions.

Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

Key recommendations and actions requested
NOTE the significant workforce implications associated with Mental Health Act reform.
RECOGNISE that workforce consequences extend beyond psychiatry into governance, legal, administrative, community and learning disability/autism services.
SUPPORT development of a Devon and Cornwall Mental Health Act Workforce Impact Assessment during 2026/27.
REQUEST that providers and ICB colleagues establish a joint task-and-finish group to produce a system-wide workforce model and implementation plan.
USE the outputs to inform future workforce, commissioning and financial planning discussions across the Peninsula.

Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	

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Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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Mental Health Partner Member Update for Cornwall and the Isles of Scilly

Mental Health Act Reform: Strategic Workforce Implications for Devon and Cornwall

Background

1. The Mental Health Act reforms seek to strengthen patient rights, reduce unnecessary detention, improve therapeutic benefit, increase opportunities to challenge detention and treatment, and limit circumstances in which people with learning disability or autism may be detained under Section 3 of the Act. National implementation documentation anticipates a phased delivery programme extending over approximately ten years due to the scale of workforce and infrastructure change required.

2. National organisations, including the Royal College of Psychiatrists, have consistently highlighted that implementation cannot be absorbed within existing workforce capacity and will require additional clinical time across a range of professional groups. Particular areas identified nationally include:
   - Increased Mental Health Tribunal activity.
   - Statutory Care and Treatment Plans.
   - Advance Choice Documents.
   - Greater involvement of Nominated Persons and advocates.
   - Increased evidential requirements for Community Treatment Orders.
   - Expansion of community provision for people with learning disability and autistic people.
   - Strategic Context in Devon and Cornwall

3. The Devon and Cornwall system already experiences significant challenges related to Mental Health Act implementation.

4. Recent system discussions have identified:
   - Delays in Mental Health Act Assessments associated with shortages of Section 12 doctors and second-doctor availability.
   - Concerns regarding sustainability of current Responsible Clinician capacity.
   - Delays affecting tribunal activity and statutory processes.

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- System risks associated with Mental Health Act assessment pathways and workforce fragility.

5. In parallel, providers are already exploring alternative Approved Clinician models, including Multi-Professional Approved Clinician (MPAC) roles, to address limitations in traditional medical workforce supply and to increase Responsible Clinician capacity within community services.

6. These existing pressures suggest that Mental Health Act reform is likely to act as a force multiplier for workforce demand rather than creating entirely new areas of activity.

Anticipated Workforce Implications

Responsible Clinician Capacity

7. The most immediate consequence of reform is likely to be increased demand for senior statutory decision-making.

8. Changes to detention review arrangements, tribunal access, Care and Treatment Plans, consultation requirements and documentation standards are expected to increase the amount of RC time required per detained patient. National workforce reviews undertaken for the Royal College of Psychiatrists have highlighted these impacts as areas of significant concern.

9. For Devon and Cornwall this may manifest as:
   - Increased demand for consultant psychiatrist time.
   - Greater need for Approved Clinician and MPAC development.
   - Additional pressure on community RC functions, including CTO management.
   - Increased requirements for specialist legal and tribunal preparation.

10. The risk is that, without additional capacity, statutory functions begin to displace direct clinical care.

Mental Health Act Administration and Governance

11. While workforce discussions frequently focus on psychiatrists, reform also carries major implications for governance infrastructure.

12. Additional tribunal activity, statutory Care and Treatment Plans, Nominated Person arrangements, advocacy engagement and monitoring requirements will increase demand on:

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- Mental Health Act offices.
- Governance teams.
- Legal services.
- Medical secretarial support.
- Data and reporting functions.

13. There is a risk that inadequate investment in administrative infrastructure could undermine compliance regardless of clinical staffing levels.

Learning Disability and Autism Services

14. Potentially the most significant strategic consequence of reform relates to learning disability and autism services.

15. The reforms will mean that people with learning disability and autistic people will generally no longer be detained under Section 3 unless a co-occurring mental disorder meets detention criteria. National implementation guidance recognises that this change is dependent upon substantial expansion of community services and support infrastructure.

16. This has potentially profound implications for Devon and Cornwall.

17. Future demand may include expansion of:
    - Intensive community support services.
    - Positive Behaviour Support.
    - Specialist psychology.
    - Learning disability nursing.
    - Autism specialist practitioners.
    - Dynamic Support Register functions.
    - C(E)TR infrastructure.
    - Housing, provider support and social care interfaces.

18. The workforce consequences of reform may therefore be greater in community services than within inpatient care.

Proposed Workforce Impact Modelling Approach

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19. To support future planning, it is proposed that a Devon and Cornwall workforce impact model be developed jointly by providers and commissioners.

20. The model should assess:
    - Statutory Activity
    - Detained patient activity.
    - Tribunal volumes.
    - CTO caseloads.
    - Renewal activity.
    - Mental Health Act Assessments.
    - Section 136 activity.
    - Workforce Baseline
    - Consultant psychiatrist capacity.
    - Approved Clinician capacity.
    - MPAC capacity.
    - Mental Health Act administration.
    - Learning disability and autism workforce.
    - Crisis and liaison services.
    - Scenario Planning

21. Three scenarios are proposed:
    - Low impact
    - Central impact
    - High impact

22. This would allow the Collaborative to distinguish between:
    - Minimum workforce required for legal compliance.
    - Workforce required to deliver reforms safely.
    - Workforce required to fully deliver intended population benefits.

Key Risks

23. Without proactive planning there are risks of:
    - Increased delays in Mental Health Act processes.
    - Reduced availability of Responsible Clinicians.

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- Growing tribunal backlogs.
- Increased legal and regulatory exposure.
- Failure to realise intended benefits of reform.
- Increased pressure on already stretched learning disability and autism services.
- Unplanned financial growth requirements emerging through crisis escalation rather than strategic investment.

Recommendations

24. The South West Peninsula Board is asked to:

- **Note** the significant workforce implications associated with Mental Health Act reform.
- **Recognise** that workforce consequences extend beyond psychiatry into governance, legal, administrative, community and learning disability/autism services.
- **Support** development of a Devon and Cornwall Mental Health Act Workforce Impact Assessment during 2026/27.
- **Request** that providers and ICB colleagues establish a joint task-and-finish group to produce a system-wide workforce model and implementation plan.
- **Use** the outputs to inform future workforce, commissioning and financial planning discussions across the Peninsula.

Conclusion

25. Mental Health Act reform represents a major opportunity to improve rights, therapeutic care and patient outcomes. However, delivery of these ambitions will depend upon significant workforce investment and service redesign.

26. For Devon and Cornwall, the most important strategic consideration is not simply whether there are enough Responsible Clinicians to undertake additional statutory duties. Rather, it is whether the system develops the community infrastructure, learning disability and autism support, governance capacity and multidisciplinary workforce necessary to make the reforms workable in practice.

27. A coordinated system-wide workforce plan will therefore be essential if the Collaborative is to deliver both compliance with the new legislation and the wider ambitions that underpin it.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

Representative of the Voluntary, Community, and Social Enterprise (VCSE) sector within Cornwall and Isles of Scilly and within Devon

Date of meeting	Date report produced
24 September 2026	09 September 2026

Author(s)		Report approved by	
Name and title:	Diana Crump, CEO, Torbay, Plymouth and Devon VCSE Assembly Emma Rowse, CEO, Cornwall VSF	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
This report provides an update from the perspective of the VCSE sector within the Devon Integrated Care System / Cornwall and Isles of Scilly Integrated Care System. Key highlights include: An evidence based paper written in collaboration between Active Devon and Active Cornwall outlining a unique opportunity for Devon and Cornwall to become a national leader in integrating physical activity into health and care systems that are 'fit for the future. Alongside reading their full report (Appendix 1) we would request that all board members watch the short accompanying video which

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summarises the key ask and offer. Integrating Physical Actvity Into Health Systems | Videos & Movies on Vimeo

The national work that the peninsula VCSE are supporting and engaging with to learn from other areas and spread our good practice more broadly across the UK. This includes the research, led by the University of Plymouth which explains the factors behind the economic and social problems of coastal regions and the development of the CESA data portal. Centre for Coastal Communities - University of Plymouth

Examples of new and innovative ways that are joining up our VCSE infrastructure and delivery work, sharing learning and scaling programmes which have originated in one of the ICB areas across the peninsular. This includes a proposal to expand the 'Gateway' meetings to cover both Devon and Cornwall for the benefit of citizens across the peninsular and the governance, accountabilities, groups and networks set up within the newly formed independent TPD VCSE Assembly to support the ICB delivery of the 10 year plan and neighbourhood health and wellbeing development.

Committees that have previously discussed/agreed the report, and outcomes of that discussion

N/A

Key recommendations and actions requested

ENDORSE the development of a Devon and Cornwall & Isles of Scilly NHS Physical Activity Shared Vision and supporting Commissioning Framework as part of the wider prevention and neighbourhood health agenda and in support of Cornwall & Isles of Scilly and Devon ICB Joint Forward Plan (1) (2). Specifically, the purpose is to:

- Recognise physical activity as a strategic intervention in population health approaches, reducing inequality and for preventing and treating long term conditions
- Highlight the contribution and potential of community, voluntary sector in this work, and partnerships with public health and public leisure services in delivering health outcomes through physical activity.
- Support and build on a collaborative Devon and Cornwall approach across NHS, Local Authority Public Health, VCSE and Active Partnership organisations to strengthen the role of physical activity within health and care system (Appendix 1)

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ENDORSE the setting up of 'Peninsula Gateway Meetings' as a means of spreading best practice across Devon and Cornwall. This successful model has the potential to embed a better understanding of the VCSE potential in supporting delivery of the NHS plan across the peninsula. This will provide a safe place where strategic opportunities and systemic barriers can be openly discussed between senior ICB and VCSE Leads with the aim of developing a shared plan to capitalise on the VCSE potential whilst addressing the barriers. The outcomes of these conversations will augment data and insights which can be used to help shape ICB commissioning intentions regarding VCSE development and delivery of services across the whole peninsula.
Continue to **SUPPORT** the exploration of innovative ways of ensuring the VCSE are an equal partner in the governance processes, design and delivery of services and support.
Continue to **PUSH FORWARD** with its ambition to increase investment into the VCSE with a focus on sustainable, longer-term funding support the left shift.

Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	
Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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Representative of the Voluntary, Community, and Social Enterprise (VCSE) sector within Cornwall and Isles of Scilly and within Devon Update

Introduction and Background

1. This report provides an update from the perspective of the VCSE sector within Cornwall and Isles of Scilly and Devon.

2. For the purposes of clarity, this sector covers the Local Authority areas of Cornwall Council, Council of the Isles of Scilly, Devon County Council, Torbay Council and Plymouth City Council.

3. The content of this report focusses on the areas of most significance in terms of key performance measures for each organisation as well as key achievements and events from the last period. The report also highlights any emerging issues or risks from the perspective of the VCSE within Cornwall and the Isles of Scilly and within Devon.

National/Regional VCSE work

Alliance 42

4. We are very active within the regional VCSE Infrastructure network, ALLiance42 which connects the VCSE alliances across England with NHSE national and regional and DHSE giving direct access to knowledge about the hundreds and thousands of VCSE organisations operating in local systems.

- VSF CEO is the SW regional lead and part of the leadership group. The aim being to influence the need for a stronger VCSE voice across all programmes of NHSE work including Neighbourhood Health as well as advocating for the importance of investing in VCSE infrastructure, collaboration, capacity building and workforce.

- The TPD VCES Assembly CEO continues to work at national and regional level in supporting the SW regional commissioning development programme in conjunction with Annie Tysom, Head of Commissioning Development, System Oversight Co-ordination, and others at NHSESW

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Centre for Coastal Communities

5. As a result of the VCSE involvement in the development of the work led by the University of Plymouth, the TPD VCSE Assembly CEO attended a reception at the House of Lords hosted by Lord Bassam of Brighton in relation to the challenges facing Britains Coastal Communities. This research explains the factors behind the economic and social problems of coastal regions and shows how policy reform can create dynamic coastal communities. The development of the CESA data portal provides an innovative, engaging and easy to use online resource providing access to data relating to the socio-economic characteristics of Englands coastal and non-coastal communities which were collected during an ESRC funded research project. For more information: Centre for Coastal Communities - University of Plymouth

Peninsular VCSE Infrastructure development

6. Devon and Cornwall VCSE leaders are committed to work collaboratively, delivering together where this makes sense for our population and sector and learning from each other's successes and failures to ensure best practice.

Peninsula Gateway Meetings

7. We would like to spread the good practice from Cornwall in relation to working collaboratively at strategic level to develop our shared aspirations by convening a series of 'Peninsula Gateway Meetings'. This would provide a safe space for a small group of interested ICB Executives, non-executive directors and VCSE leaders to ensure that the ICB is capitalising on the potential and value of the VCSE across the peninsula.

8. The ICB's **Head of VCSE Development** across Devon and Cornwall has been appointed (Darin Halifax). The purpose of this post is to work across NHS, VCSE and communities to develop a network of shared understanding to support the transformation of the current system to prevent ill health and reduce health inequalities. The post-holder will work in a matrix and flexible way across multiple areas including Population health, neighbourhood development and strategic commissioning to support the left shift and delivery of the NHS 10 year plan.

9. The individual and complimentary **VCSE infrastructure investments** into the CIoS and Devon systems will put the VCSE across the peninsula at the heart of the 10 year plan shifts of sickness to prevention, hospital to community and analogue to digital. Key areas of focus for the infrastructure investment will be:

- Leadership and advocacy – strategic engagement and representation through coordinated and transparent routes, including sourcing and remunerating VCSE orgs to participate.

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- Partnerships and collaboration – convening thematic and other conversations with the widest possible groups of VCSE organisations to support both VCSE and public sector work.

- Capacity building – small grant scheme, workforce skills and wellbeing funding

- Communication – information on funding opportunities, jobs, sector updates, events, marketing and campaigns

- Impact and insight - support for the sector on measurement tools, evaluation, social return on investment and impact reporting, capacity building training for teams and orgs

VCSE Provider Collaborative

10. Building on work developed in 2024 in CIOS, Cornwall VSF and TPD Assembly are working with colleagues in Bristol, North Somerset and South Gloucester (BNSSG) to design and deliver a provider collaborative function. In recognition of the ICB's shift to a strategic commissioning function, the VCSE Provider Collaborative will offer a straightforward mechanism through which new investment can be commissioned from the VCSE. By delegating collaboration and partnership development, codesign and sometimes contract management functions to an experienced body or consortium with sector-specific expertise, the NHS can streamline administrative processes, align activities more closely with population health needs, reduce overhead expenditure, and improve service quality and outcomes. This proposal is underpinned by a commitment to population-based commissioning and the principles of integrated and personalised care that promote preventive, community-led solutions at lower cost and with higher impact. One of the aims will be to increase the number of VCSE organisations involved in public sector delivery and offer an equitable, transparent and supportive way for VCSE organisations to collaborate on solutions and service.

Devon Specific VCSE Infrastructure developments

ICB Investment in Devon VCSE Infrastructure

11. Following Board approval of £5.94m of funding over 5 years, good progress is being made to develop the TPD VCSE Assembly further as a separate, independent legal entity with a mandate to support the delivery of the NHS plan across Devon:

- Full consultation with VCSE sector regarding the governance structure resulting in agreement for a Charitable Incorporated Organisation (CIO) with free membership open to VCSE organisations across the County who would have full voting rights.

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- Collaborative working with the ICB to embed TPD VCSE Assembly across strategic, place and neighbourhood levels within the ICB.

- MOU signed with ICB Q1 and Q2 funding now received. Processes to be put into place to prevent further delays in release of funding moving forwards

- Interim Trustees appointed. TPD VCSE Assembly Board fully functioning. Advisory Committee in place consisting of 15 VCSE CEOs who each have a mandate to represent various interests and place-based agendas. (See Appendix 2 for structure and governance arrangements)

- CEO and Finance and Ops Directors appointed (Diana Crump and Julia Boas respectively)

- Administrative and co-ordination team recruited and inducted

- Companies House registration completed. Charity Commission application ready for submission.

- Conflict of Interest and other policies agreed and implemented

- Governance arrangements for VCSE advisory workstreams, thematic sub-groups and local VCSE networks Sub-groups agreed in accordance with MOU (Appendix 2). These include supporting local VCSE networks and reps to be able to feed into the parallel INT and LCP developments, as well as thematic VCSE sub-groups being developed to explore key NHS priority need areas (frailty, end of life, workforce development, etc).

- The VCSE Infrastructure grant scheme (a key element of our ICB funding) is now also in development and we will be distributing relevant grants to the sector as soon as possible. This will support VCSE to have the infrastructure in place to support the three shifts and the development of neighbourhood health.

- Better Care Fund discussions ongoing between VCSE Assembly and DCC/ICB commissioners to ensure VCSE potential is utilised as appropriate

- Evaluation model for VCSE Assembly effectiveness being developed in partnership with an independent organisation to provide outcome-based measures.

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CIOS Specific Infrastructure work

CIOS VCSE Infrastructure

12. Following confirmation of the 3-year strategic infrastructure contract to Cornwall VSF, we now attach our strategy to support delivery.

13. **Cornwall VSF** remains engaged with the neighbourhood health and wellbeing development and has supported system conversations about the opportunities presented by developing an integrated health and wellbeing partnership with VCSE as an equal partner. Working alongside Kernow Health CIC/GP Collaborative Board to explore opportunities to develop a similar governance model, VSF is working with the VCSE Strategic Alliance to shape this development.

Peninsula VCSE Delivery

WellFed Programme

14. Led by Volunteer Cornwall and with a small amount of funding from the ICAs and Public Health, this innovative offer provides fruit and veg on prescription, cooking education, peer support and health coaching. By connecting local community growing schemes with GP practices and face to face support, participants are supported to volunteer, connect with others, do more exercise and cook healthier food for their families. The programme is being evaluated by the University of Bath and early findings suggest a promising reduction in type 2 diabetes risk as well as reductions in weight and cholesterol. Further evaluation is ongoing with next stage data being available in 2027.

The Devon Dementia Strategy.

15. Following endorsement of the Dementia Strategy by the Board, a VCSE led Thematic Group has been set up role modelling a collaborative, inclusive model for a VCSE-led approach to service delivery. This consists of a VCSE strategy group supported by sub-groups to identify what is already working and where the gaps are. All work is undertaken in collaboration and close liaison with statutory sector colleagues. The work is on track to submit recommendations to the commissioning teams in relation to the implementation of the Dementia Strategy to be incorporated in the commissioning intentions document.

16. The ICB and VCSE are working together on a **small grants programme** to support the engagement for future neighbourhood working in Devon. It is proposed that the programme starts in September and will enable VCSE organisations to ask communities for their views and concerns.

17. Similarly, CIOS will be launching our **annual small grant scheme** later in the year delivered through our infrastructure partners Cornwall Community Foundation.

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VCSE Peninsula Challenges

18. Whilst the ICB investment is welcomed, the financial precarity of the VCSE sector continues to be of concern. The cost-of-living crisis has resulted in reductions in public donations which has a knock-on effect to grant making nationally and locally.

19. The vast majority of VCSE organisations who are funded through grants are reliant on annual confirmation of ongoing funding. There are only two examples (Community Gateway and People in Mind services) where contracts have been let for 5 years giving much needed financial security and sustainability. It is critical that our vision of protecting and increasing VCSE investment is underpinned by longer-term contracts or grants.

Recommendations

20. The South West Peninsula Board is asked to:

- **Endorse** the development of a Devon and Cornwall & Isles of Scilly NHS Physical Activity Shared Vision and supporting Commissioning Framework as part of the wider prevention and neighbourhood health agenda and in support of Cornwall & Isles of Scilly and Devon ICB Joint Forward Plan (1) (2). Specifically, the purpose is to:
  - Recognise physical activity as a strategic intervention in population health approaches, reducing inequality and for preventing and treating long term conditions
  - Highlight the contribution and potential of community, voluntary sector in this work, and partnerships with public health and public leisure services in delivering health outcomes through physical activity.
  - Support and build on a collaborative Devon and Cornwall approach across NHS, Local Authority Public Health, VCSE and Active Partnership organisations to strengthen the role of physical activity within health and care system (Appendix 1)

- **Endorse** the setting up of 'Peninsula Gateway Meetings' as a means of spreading best practice across Devon and Cornwall. This successful model has the potential to embed a better understanding of the VCSE potential in supporting delivery of the NHS plan across the peninsula. This will provide a safe place where strategic opportunities and systemic barriers can be openly discussed between senior ICB and VCSE Leads with the aim of developing a shared plan to capitilise on the VCSE potential whilst addressing the barriers. The outcomes of these conversations will augment data and insights which can be used to help

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shape ICB commissioning intentions regarding VCSE development and delivery of services across the whole peninsula.

- Continue to **support** the exploration of innovative ways of ensuring the VCSE are an equal partner in the governance processes, design and delivery of services and support.

- Continue to **push forward** with its ambition to increase investment into the VCSE with a focus on sustainable, longer-term funding support the left shift.

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South West Peninsula Board Meeting in public

Director of Public Health Update for Devon

Date of meeting	Date report produced
24 September 2026	10 September 2026

Author(s)		Report approved by	
Name and title:	Dr Lincoln Sargeant, Director of Public Health, Torbay Julia Chisnall, Consultant in Public Health, Torbay	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
This report provides an update from the perspective of the Directors of Public Health of the Devon Integrated Care System. Key highlights include a spotlight on inequalities and neighbourhood health developments in Torbay Council area.

Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

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Key recommendations and actions requested
NOTE the emerging local authority perspectives on neighbourhood health planning.
CONTINUE TO FOSTER COLLABORATIONS AT PLACE (local authority footprints) for development of neighbourhood health plans.

Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of services	All
Health inequalities	
Workforce	
Resources and finance	
Legal	
Digital and Data	
Engagement and consultation	
Other – sustainability etc	

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Directors of Public Health Update for Devon

Introduction and Background

1. This report provides an update from the perspective of the One Devon Directors of Public Health.

2. For the purposes of clarity, the Public Health Directors are from the Local Authority areas of Devon County Council, Plymouth City Council and Torbay Council.

3. The content of this report focusses on the areas of most significance in terms of key performance measures for each organisation as well as key achievements and events from the last period. The report also highlights any emerging issues or risks from the perspective of the Directors of Public Health.

Spotlight on inequalities and neighbourhood health developments

Torbay Council

4. Torbay and South Devon Local Care Partnership developed a South intelligence profile in 2025, combining the headline analysis from the Torbay and Devon Joint Strategic Needs Assessments, supplemented by input from Primary Care Networks, Trust and VCSE colleagues.

5. From this a South Health Inequalities Strategy was developed, drawing out the areas of highest need and agreeing a year one priority work programme.

6. During 2025-26, the Local Care Partnership sponsored work on frailty, high intensity Emergency Department use, and health and work. The health and work programme involved a primary care and VCSE collaboration identifying people with long term health conditions, furthest from the workplace, and developing pathways of support which feed into the Local Authority led *Connect to Work* programme.

7. South Primary Care Networks have been asked to identify priority projects to address areas of local inequality, which are now being taken forward under the neighbourhood development programme, using a test and learn approach which will feed into longer term system redesign.

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8. Building on the approach in Cornwall, Torbay Public Health are about to pilot 'prevention and inequalities opportunity packs' to support local primary care networks with population level analysis showing areas which would benefit from intervention or outreach.

9. The Torbay *Turning the Tide on Poverty* programme sits alongside these initiatives, taking an overview of deprivation based work within the Bay. Torbay is a second wave participant in the national *Coastal Navigators Network* focusing on diversion and support for the pre NEET (not in education, employment or training) cohort.

10. A new approach to community engagement is being developed by VSCE organisations, led by Torbay Communities and Local Motion, and sponsored by the Local Care Partnership and Turning the Tide. The approach takes learning from the Poverty Truth Commissions elsewhere in the cluster and will focus on building meaningful relationships with populations experiencing coastal and rural poverty, those who are ethnically minoritised, and people experiencing frailty, building the learning into the developing approaches to neighbourhood health.

Recommendations

11. The South West Peninsula Board is asked to:

- **Note** the emerging local authority perspectives on neighbourhood health planning.

- **Continue to foster collaborations at place** (local authority footprints) for development of neighbourhood health plans.

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South West Peninsula Board Meeting in public

Chair's Report of the Audit & Risk Committees meeting in Common – 07 September 2026

Date of meeting	Date report produced
24 September 2026	10 September 2026

Author(s)		Report approved by	
Name and title:	Lopa Patel, Non-Executive Member	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
This report provides information about the discussions that took place at the NHS Cornwall and Isles of Scilly (CIOS) and NHS Devon Audit & Risk Committees meeting in common on 07 September 2026.

Committees that have previously discussed / agreed the report, and outcomes of that discussion
N/A

Key recommendations and actions requested
NOTE the content of the Chair's Report.

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Outline the implications of matters covered in this report for the following	
Area	
Quality of Services	All
Health Inequalities	
Workforce	
Resources and Finance	
Legal	
Digital and Data	
Engagement and Consultation	
Other – sustainability etc.	

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NHS Cornwall and Isles of Scilly

NHS Devon

Chair's Report of the Audit & Risk Committees meeting in common – 07 September 2026

Introduction and Background

1. The Audit & Risk Committees (ARC) have been established by the South West Peninsula Board to enable the NHS Cornwall and Isles of Scilly (CIOS) and NHS Devon (NHSD) Integrated Care Boards to contribute to the overall delivery of organisational objectives by providing oversight and assurance to the South West Peninsula Board on the adequacy of governance, risk management and internal control processes.

Overview

2. The ARC met on 07 September 2026 and considered reports on financial governance; counter fraud; internal and external audit; governance, risk management and internal control. The Committee was quorate.

Financial Reporting

Cluster Losses and Special Payments

3. The Committee received an oral update on a significant special payment case relating to a former accommodation contract in Cornwall. Legal advice has been sought, NHS England has been notified, and Treasury approval will be required. Negotiations are underway to reduce liability, and a full paper will be circulated ahead of the December meeting.

4. No special payments were reported for NHS Devon.

Cluster Quarterly Contracting and Procurement Update

4. The Committee received an update on contracting and procurement activity, including immediate actions taken to strengthen the single action waiver (SAW) process. NHS CIOS will adopt the established NHS Devon SAW process to ensure consistent recording, risk assessment and oversight. SAWs will be reported to the Procurement Oversight Group and ARC. Members emphasised the need for stronger contract management, clearer accountability for legacy contracts, and improved forward planning through the Procurement Oversight Group. The Committee noted capacity constraints arising from the closure of

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the Devon Commissioning Support Unit (CSU) and the need to prioritise procurements once reduced central support takes effect in December 2026.

NHS Devon: Clinical Waste Procurement Update and Options Appraisal

5. ARC reviewed the clinical waste procurement update and noted that although the Committee cannot approve spend, it would scrutinise the process and associated risk. The Committee welcomed the detailed appraisal and recognised the complexity and litigious nature of clinical waste procurement. Members sought clarity on governance, liability under the SBS framework, and the need for robust documentation to mitigate challenge risk. The Committee agreed that the paper should also be submitted to the Devon Finance, Performance and Quality (FPQ) Committee to enable consideration of, and comment on, the associated expenditure.

Counter Fraud

NHS Cornwall and Isles of Scilly

6. The Committee received the Counter Fraud Update and Functional Standard Return for NHS CIOS. The report provided assurance on fraud prevention, detection and response activity, with no significant issues escalated. The Committee noted continued compliance with national standards.

NHS Devon

7. The Committee reviewed the NHS Devon Counter Fraud Annual Report and Functional Standard Return. Assurance was provided that NHS Devon continued to meet required standards, with active investigations and preventative work underway. No material concerns were raised.

Internal Audit

NHS Cornwall and Isles of Scilly

8. The Internal Controls Assurance Report highlighted that several longstanding recommendations remained open due to capacity constraints during organisational restructuring. Assurance was provided that controls were improving, though further work was required to embed new processes consistently across the cluster. Members noted that the robustness of contract management and procurement oversight would need to be strengthened to

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prevent recurrence of issues such as single action waivers being used outside expected parameters.

NHS Devon

9. The Committee reviewed the Internal Audit Progress Report for NHS Devon. It was noted that audit delivery for 2026/27 remained under pressure due to reduced capacity, alongside the impact of organisational change. Several audits were underway, but the Committee recognised that the current audit plan may not be deliverable in full. Members emphasised the need to prioritise audits that addressed the highest risks, including procurement, contract management, information governance, and health inequalities. The Committee discussed the importance of ensuring that new directors and commissioning leads were fully sighted on legacy contracts, renewal timelines and budget responsibilities, as gaps in this knowledge represented a material risk to internal control.

NHS Cornwall and Isles of Scilly Internal Audit Recommendations Update

10. The Committee received an update on outstanding internal audit recommendations for NHS CIOS. Progress had been made, though several recommendations remained open due to restructuring, recruitment delays and capacity constraints.

External Audit

NHS Cornwall and Isles of Scilly

11. Assurance was provided that external audit work remained on track, with no significant issues escalated at this stage. The Committee noted that Grant Thornton had maintained constructive engagement with the organisation throughout the transition period, despite capacity pressures. Members recognised that the stability of the CIOS financial position supported a smoother audit cycle, though the auditors highlighted that continued vigilance was required given wider system pressures.

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NHS Devon

12. KPMG highlighted that the Devon system remained under significant financial challenge, with multiple providers preparing recovery plans ahead of regional meetings. The Value for Money (VfM) assessment emphasised the importance of credible plans to recover performance in urgent and emergency care and Referral to Treatment (RTT) and noted that failure to demonstrate sufficient assurance could result in national escalation. The Committee discussed the implications of these risks for the audit opinion and recognised that the underlying financial position, combined with operational pressures, continued to represent a material VfM risk. KPMG confirmed that they were monitoring the impact of organisational restructuring, procurement capacity constraints and contract management weaknesses, all of which could influence future VfM conclusions.

Governance, Risk Management and Internal Control

Risk Management and Board Assurance Framework (BAF) Update

13. The Committee received a verbal update on the cluster BAF ahead of its submission to the Finance, Performance and Quality which highlighted new risks in relation to UEC, RTT and 62 day Cancer performance replacing a previous generic risk on constitutional standards.
14. The Committee was also provided with an update on the top financial risks which were aligned to the delivery of the financial plan, 2028/29 outturn position and transformation as well as the risks of CIP delivery.

Other Assurance Functions

Freedom to Speak Up (FTSU)

15. The FTSU guardian confirmed that new dual-guardian arrangements were now in place across NHS CIOS and NHS Devon. The transition to the new organisational structure had required a refresh of FTSU governance pathways, reporting lines and escalation routes. The Committee discussed the importance of ensuring that FTSU activity remained visible within the cluster's assurance framework, particularly given workforce pressures, organisational change and the need for strengthened psychological safety during periods of restructuring. The Committee took assurance from the update and recognised that further embedding of the new arrangements would be required as the cluster matured.

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Data Protection and Security Assurance Report (incl. SIRO Report)

16. The Committee noted continued compliance with statutory requirements despite capacity pressures within digital and information governance teams. The report highlighted the need for strengthened cyber security oversight across the cluster, with particular focus on ensuring clear lines of accountability between digital, cyber security and information governance leads. Members noted that the cluster transition had created temporary gaps in capacity and clarity and agreed that future iterations of the report should provide more explicit assurance on how risks were being managed across both systems. The Committee took assurance from the report but recognised that continued strengthening of cyber security governance and data protection oversight would be required as the cluster matured.

Annual Health and Safety Report

17. The update confirmed that estates-related responsibilities remained the largest component of the health and safety function, but that wider corporate obligations, particularly for lone-working staff in Continuing Healthcare and community-based roles, were also in scope. The Committee emphasised the importance of mental health and wellbeing within the health and safety remit, particularly given the dispersed workforce and pressures associated with organisational change. It was agreed that monitoring of work-related stress and sickness absence should sit with the People Committee, with Audit & Risk focusing on assurance around systems and controls.

Matters to Raise to the Board

18. There are no issues to escalate to the Board however, the Chair draws the Board's attention to:
    - Legal advice has been sought regarding a Special Losses claim and negotiations are underway to reduce the liability.
    - Internal audit work identified remediation themes relating to health inequalities. These risks should be more explicitly reflected within the Board Assurance Framework.
    - Internal audit delivery for 2026/27 remained under strain due to vacancies, restructuring and reduced resource within both the ICBs and Internal Auditors. Some audits may need to be rescheduled, scaled back or replaced with more targeted work to ensure assurance remained focused on the most material risks.
    - The Board will be provided with an update following the review of the contents of the internal audit plan

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Recommendations

19. The South West Peninsula Board is asked to:
    - **NOTE** the content of the Chair's Report.

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South West Peninsula Board Meeting in public

Chair's Report from the South West Peninsula People, Customer Experience and Equity Committee – 03 September 2026

Date of meeting	Date report produced
24 September 2026	10 September 2026

Author(s)		Report approved by	
Name and title:	Tarn Lamb Non-Executive Member	**Name and title:**	N/A

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

Executive summary
This report provides information about the discussions that took place at the South West Peninsula People, Customer Experience and Equity Committee Meeting on 03 September 2026.

Committees that have previously discussed/agreed the report, and outcomes of that discussion
N/A

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Key recommendations and actions requested
NOTE the content of the Chair's Report.
AGREE to further discuss and develop work on tackling health inequalities.
AGREE to include Population Health related Strategic risks within the Board Assurance Framework, as clarity on the organisation's population health risk would support a more focused and structured approach.

Outline the implications of matters covered in this report for the following areas (if none, please state)	
Area	
Quality of Services	All
Health Inequalities	
Workforce	
Resources and Finance	
Legal	
Digital and Data	
Engagement and Consultation	
Other – sustainability etc.	

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NHS Cornwall and Isles of Scilly

NHS Devon

Chair's Report from the South West Peninsula People, Customer Experience and Equity Committee – 03 September 2026

Introduction and Background

1. The South West People, Customer Experience and Equity Committee (the Committee) has been established by South West Peninsula Board to enable NHS Cornwall and the Isles of Scilly Integrated Care Board and NHS Devon Integrated Care Board (the Partner ICBs) to consider and seek assurance in respect of the experiences of people (staff, patients and the public) in relation to the work of the Partner ICBs, ensuring that the Cluster seeks and acts on insights to improve equity within the NHS workforce and the delivery of health outcomes.

Overview

2. The Committee met on 03 September 2026 and was quorate. This paper provides a summary of key items discussed at the meeting of the South West Peninsula People, Customer Experience and Equity (PCEE) Committee.

Integrated Care Board Workforce

Organisational Change Update – Timelines and Process

3. An update on Phase 2 organisational change was provided. Final structures and revised People Impact Assessments were published on 19 August 2026, confirming 356 colleagues slotted into roles, 78 placed into ringfenced pools and 34 assigned to the redeployment pool. Managers were progressing ringfenced interviews, with further colleagues expected to enter redeployment and an estimated 55 – 60 staff likely to be without roles at the end of the process. Approximately 160 voluntary redundancy applications had been approved. Digital and Data and Human Resources (HR) teams remained scheduled for Phase 3. Staff continued to experience uncertainty and pressure due to reduced resources, though trade union relations had improved.

Organisational and People Development Framework

4. The Committee reviewed the Organisational and People Development Framework, which set out priorities for the next 12 months focused on reflective practice, independent scrutiny and mitigating the loss of senior leadership experience. A cluster-wide staff voice group and OD reference group had been

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established, and an external cultural review would be commissioned in Quarter 3. The Committee discussed the importance of senior leadership stability, clarity on expectations across the new geographical footprint, and realistic sequencing of OD activity given recruitment needs, political uncertainty and winter pressures. The Committee advised that the Framework and delivery roadmap must be appropriately prioritised and endorsed the proposed governance arrangements.

Customer Experience

Strategic Direction for the Experience, Engagement and Communications Function

5. The unified Experience, Engagement and Communications function aims to strengthen how local people's voices and experiences informed commissioning and service improvement. Although significant insight was already collected, it was not yet used systematically across the Peninsula. The function would shift from gathering feedback to analysing insight that could shape decision-making, with a stronger focus on lived experience, intersectionality and representation from deprived neighbourhoods. Members emphasised the need to map the organisation's engagement responsibilities against those of Providers to avoid duplication. The Committee supported the approach and the plan to feature the voice of local people into South West Peninsula Board discussions from September 2026.

Equity

Tackling Health Inequalities

6. The Committee discussed the emerging framework for understanding, prioritising and evaluating health inequalities activity across the Devon and Cornwall systems. A mapping exercise was underway to identify existing work, learning and gaps, and the upcoming Peninsula Health Inequalities Workshop in October 2026 would support development of a shared approach. Members agreed that integration of NHS CIOS and NHS Devon provided an opportunity to adopt a more strategic, intelligence-led approach with clear long-term ambitions such as reducing the deprivation gap and extending healthy life expectancy. The need for a small set of long-term metrics, fair and transparent funding approaches, and increased upstream investment in prevention was emphasised.

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Matters to Report to the Board

7. The Committee agreed to raise the following matters to the Board's attention:

- The Committee supported the strategic direction of the Experience, Engagement and Communications Function and bringing the voice of local people into Board discussions from September 2026.

- The Committee agreed to recommend to the South West Peninsula Board that further discussion and development on tackling health inequalities is required. Following this process, Population Health related Strategic risks should be included within the Board Assurance Framework, as clarity on the organisation's population health risk would support a more focused and structured approach.

Recommendation

8. The South West Peninsula Board is asked to:

- **Note** the content of the Chair's Report.
- **Agree** to further discuss and develop work on tackling health inequalities.
- **Agree** to include Population Health related Strategic risks within the Board Assurance Framework, as clarity on the organisation's population health risk would support a more focused and structured approach.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

Chair's Report of the Cornwall and Isles of Scilly Finance, Performance and Quality Committee - 10 September 2026

Date of meeting	Date report produced
24 September 2026	14 September 2026

Author(s)		Report approved by	
Name and title:	Martin Sykes, Non-Executive Member	**Name and title:**	N/A

Executive summary
This report provides information about the discussions that took place at the Cornwall and Isles of Scilly Finance, Performance and Quality Committee Meeting on 10 September 2026.

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

To which Board Assurance Framework risk does the report relate?
All

Brad Hyden 17/09/2026 20:08:20

What are the recommendations arising from the report?
NOTE the content of the Chair's Report.
NOTE the Committee's limited assurance regarding provider financial recovery and system performance.
NOTE the continued focus on maternity improvement activity and quality oversight across the Peninsula.

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
N/A

What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	All
Quality and Safety	
Equality, Diversity and Inclusion	
Finances and Use of Resources	
Regulation and Legal Requirements	
Information Governance (DP and Cyber)	
Transformation and Innovation	
Environment and Climate Change	

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Chair's Report of the Cornwall and Isles of Scilly Finance, Performance and Quality Committee – 10 September 2026

Introduction and Background

1. The Cornwall and Isles of Scilly (CIOS) Finance, Performance and Quality Committee met on 10 September 2026 to review governance, finance, performance and quality matters across the Cornwall and Isles of Scilly system and wider Peninsula arrangements.

Overview

2. The Committee received updates across governance, workforce, quality, performance and finance and reviewed a number of areas subject to enhanced oversight. The Committee welcomed progress in strengthening assurance and governance arrangements but noted continuing concerns relating to provider financial recovery, operational performance, service sustainability and growing financial pressures, including the impact of prescribing costs and national drug price concessions. Particular focus was given to the delivery of recovery plans, urgent and emergency care performance and maternity quality oversight

Governance

3. The Committee reviewed progress on the development of the Board Assurance Framework (BAF) and endorsed proposed revisions to strengthen the articulation and oversight of strategic financial and performance risks. Members supported the separation of financial and performance risks, recognising that this would improve visibility of risk drivers, mitigating actions and accountability arrangements.

4. The Committee noted ongoing work to align the BAF with organisational objectives and discussed whether the framework sufficiently reflected risks associated with prevention, population health improvement and longer-term transformation. Members requested further consideration of how these areas could be strengthened within the strategic risk profile and reflected in future iterations of the framework.

5. The Committee emphasised the importance of ensuring that transformation, financial sustainability, service performance and population health objectives are appropriately reflected within future assurance and reporting arrangements, enabling the Board to maintain effective oversight of the principal risks to delivery of its strategic objectives.

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Finance

6. The Committee reviewed the Month 04 financial position and received satisfactory assurance regarding the ICB's forecast year-end position, with NHS CIOS remaining on track to deliver plan. The Committee noted a number of emerging financial pressures, including prescribing costs, national drug price concessions and increasing demand in specific commissioned services. The Committee supported ongoing national discussions regarding funding arrangements for drug price concessions, recognising the significant impact of these pressures across the system.

7. The Committee received limited assurance regarding delivery of provider recovery plans and noted ongoing risks associated with recovery trajectories, reliance on future savings delivery and the need for greater assurance regarding the implementation of provider mitigations.

8. The Committee noted that enhanced financial oversight arrangements, including increased provider scrutiny and planned deep-dive reviews, are being implemented to strengthen accountability and support delivery of recovery objectives.

Performance

9. The Committee reviewed workforce performance across CIOS providers and noted that expenditure remained below plan overall, although this continued to be offset by significant reliance on temporary staffing and higher than planned bank usage. The Committee noted the need for further understanding of the underlying drivers of workforce performance and for strengthened assurance regarding workforce trajectories, productivity and affordability.

10. The Committee received updates on system performance and noted continuing challenges in urgent and emergency care, diagnostics and elective recovery. The Committee recognised the potential impact of these pressures on delivery of constitutional standards, waiting list trajectories and access to elective recovery funding. The Committee received limited assurance regarding performance delivery and noted the need for continued oversight and corrective action.

Quality

11. The Committee endorsed the proposed Peninsula quality governance arrangements and welcomed progress in developing a more integrated approach to quality oversight across CIOS and Devon. The Committee noted the importance of clear escalation routes and robust assurance processes as the arrangements continue to mature.

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12. The Committee reviewed the quality report and received limited assurance regarding current quality risks. The Committee noted a number of areas subject to enhanced oversight and highlighted the importance of timely provider

responses to quality concerns, clear action planning and effective escalation arrangements.

13. The Committee also considered the implications of the Amos and Ockenden maternity reports. The Committee welcomed the planned self-assessment process and subsequent gap analysis and noted the importance of maintaining focus on maternity safety, governance, workforce sustainability, inequalities and learning. The Committee recognised the importance of continued oversight of maternity services across the Peninsula, including implementation of actions arising from the Amos and Ockenden reviews.

Matters for Escalation to the Board

14. The Committee recognised continuing risks associated with provider financial recovery plans and the importance of maintaining robust assurance regarding the delivery and sustainability of recovery trajectories. Enhanced oversight arrangements are being implemented and will require ongoing monitoring. Clarity is sought on the role of the ICB vs NHSE regarding performance escalation. A future Board to Board with RCHT was discussed as a potential escalation should the Chief Executive consider this useful.

15. The Committee also highlighted ongoing performance challenges across urgent and emergency care, diagnostics and elective recovery. The Committee noted the potential implications for delivery of constitutional standards and wider system objectives and considered these areas to require ongoing Board attention.

16. The Committee further highlighted the importance of maintaining oversight of maternity services and implementation of actions arising from the Amos and Ockenden reviews. The Committee noted the need for continued focus on maternity safety, inequalities and service resilience across the Peninsula.

Recommendations

17. The South West Peninsula Board is asked to:

- **Note** the content of the Chair's Report.
- **Note** the Committee's limited assurance regarding provider financial recovery and system performance.
- **Note** the continued focus on maternity improvement activity and quality oversight across the Peninsula.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

Chair's Report of the Devon Finance, Performance and Quality Committee – 13 August 2026

Date of meeting	Date report produced
24 September 2026	09 September 2026

Author(s)		Report approved by	
Name and title:	Graham Clarke, Non-Executive Member	**Name and title:**	N/A

Executive summary
This report provides information about the discussions that took place at the Devon Finance, Performance and Quality Committee Meeting on 13 August 2026.

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

To which Board Assurance Framework risk does the report relate?
Nil

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What are the recommendations arising from the report?
NOTE the content of the Chair's Report.

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
N/A

What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	All
Quality and Safety	
Equality, Diversity and Inclusion	
Finances and Use of Resources	
Regulation and Legal Requirements	
Information Governance (DP and Cyber)	
Transformation and Innovation	
Environment and Climate Change	

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Chair's Report of the Devon Finance, Performance and Quality Committee – 13 August 2026

Introduction and Background

1. The Devon Finance, Performance and Quality Committee (the Committee) met on 13 August 2026 to review governance, finance, performance and quality matters across NHS Devon and the wider Devon system. Particular focus was given to financial recovery, workforce affordability, assurance arrangements and maternity services.

Overview

2. The Committee received assurance across a broad range of areas and welcomed progress in strengthening financial, workforce and quality oversight arrangements. Members noted continuing risks associated with provider financial recovery plans, workforce affordability and maternity service resilience.

Governance

3. The Committee reviewed progress on the development of the Board Assurance Framework (BAF) and noted further work to clarify the distinction between ICB, provider and system responsibilities within the risk structure. Updated risks and supporting narrative will return to the Committee in September 2026 to support strengthened Board assurance arrangements.

Finance

4. The Committee took satisfactory assurance regarding NHS Devon's Month 03 financial position, with the organisation remaining on plan and forecasting a break-even position for the year. However, members noted ongoing financial pressures and a reduction in assurance against the financial risk domain due to emerging cost pressures and limited financial flexibility.

5. The Committee reviewed system financial recovery arrangements and noted that all organisations remained forecast to deliver plan. Members highlighted ongoing concerns regarding reliance on non-recurrent savings and the delivery risks associated with recovery programmes, particularly within University Hospitals Plymouth NHS Trust.

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Performance

6. The Committee reviewed developments in contract management and provider oversight arrangements and noted that further work is required to strengthen assurance and accountability across the system.

7. Members also reviewed workforce performance and affordability, welcoming improvements in workforce reporting while recognising the risks associated with workforce reduction programmes, staff wellbeing and continued reliance on temporary staffing in some areas.

Quality

8. The Committee considered the implications of the recently published Amos and Ockenden reports and welcomed plans to undertake a cluster-wide gap analysis to assess local arrangements against the recommendations. Members noted the importance of maintaining focus on maternity safety, workforce sustainability, governance, accountability and the reduction of health inequalities.

9. The Committee received an update on the temporary changes to maternity service provision at North Devon District Hospital. Members were assured that appropriate arrangements had been put in place to maintain safe care, including enhanced oversight and revised pathways to support women and families accessing maternity services.

10. The Committee noted the importance of continued monitoring and oversight of the temporary arrangements and reflected on opportunities to strengthen escalation, assurance and early warning processes across the system. The Committee took satisfactory assurance that quality risks had been identified and were being appropriately managed, whilst recognising the importance of ongoing oversight of maternity services and delivery of improvement actions.

Matters for Escalation to the Board

11. The Committee agreed to escalate to the Board:

- Risks associated with provider financial recovery plans, including reliance on non-recurrent savings and delivery assumptions.
- Ongoing risks relating to the delivery of University Hospitals Plymouth NHS Trust's financial recovery programme.
- The temporary suspension of maternity services at North Devon District Hospital and wider maternity improvement work arising from the Amos and Ockenden reports.

Recommendations

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12. The South West Peninsula Board is asked to **note** the content of the Chair's Report.

NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board Meeting in public

Performance Exception Report

Date of meeting	Date report produced
24 September 2026	17 September 2026

Author(s)		Report approved by	
Name and title:	Paula Curnow, Head of Performance, NHS CIOS Jason Burn and Ben Chilcott, Directors of Finance	**Name and title:**	Gary Heneage, Chief Finance Officer

Executive summary

This report brings together performance reporting for both Devon and Cornwall and Isles of Scilly Integrated Care Boards as a move towards the development of a comprehensive integrated performance report. The focus for the report is on the operational performance delivery, finance and quality.

Both NHS Devon and NHS Cornwall and Isles of Scilly have seen material challenges in delivering **2026 / 2027 operational planning standards** during June 2026 and July 2026. This is despite many of the trajectories being re-based during the latest planning round and the summer months historically seeing an improvement.

Of specific concern is the continued distance from plan for 4-hour ED performance and the number of over 12 hour waits within **Urgent and Emergency Care (UEC)** at the Royal Cornwall Hospitals Trust (RCHT) and Torbay and South Devon FT (TSD). Both trusts remain significantly off plan in July 2026. University Hospitals Plymouth Trust (UHP) are performing better than plan based on their planning re-submission. RCHT and UHP have 12hour recovery plans that will be regularly reviewed with NHS England and the ICB.

Ambulance handover delays remain significantly worse than plan at RCHT and UHP, with the only trust consistently delivering near-30-minute delays being Royal Devon University Healthcare FT (RDUH).

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Elective Referral to Treatment (RTT) 18-week performance and list size are close to plan in Month 03.

The number of people waiting over 52 weeks remains significantly worse than plan for RDUH, TSD and UHP.

Cancer delivery remains variable. 62 days is the most challenged with TSD having the largest variance and therefore impacting on the Devon overall position who are the furthest ICB behind plan nationally.

Diagnostics over 6-week waiters are off plan at all organisations with the largest variance at RCHT which is 16% off plan. However, both RDUH and TSD have significantly more patients waiting above 6 weeks.

In **quality**, the recent CQC inspection of Royal Cornwall Hospital Trust West Cornwall Hospital resulted in an improved rating of Good (previously Requires Improvement) and the temporary maternity unit closure at North Devon District Hospital due to significant staffing pressures.

Additional risks remain within maternity services due to ongoing neonatal and obstetric workforce pressures, quality escalations with mental health settings particularly Sowenna ward (Cornwall Partnership FT) and Langdon hospital (Devon Partnership Trust), while urgent and emergency care partners continue to focus on reducing corridor care, improving patient flow and maintaining safe, high-quality care across both the NHS CIOS and NHS Devon systems.

In **Primary Care** NHS CIOS closed the River Dental Practice following a Quality Assurance visit in April 2026. A new provider for those in 'active treatment' is in place and contact with patients currently in active treatment is underway.

NHS CIOS's immediate priority remains those patients whose treatment was interrupted by the closure. **Finance position** for the ICBs as at Month 4 is on plan at break even. CIP delivery is on schedule except for slippage in the timing on running cost savings. The report details some of the grip and control assurance processes we have implemented to maintain control and oversight.

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N/A

What is the purpose of the report?	
ASSURANCE	☒
DECISION	☐

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To which Board Assurance Framework risk does the report relate?
• Delivery of Constitutional Standards and Recovery Priorities • Delivery of System Financial Sustainability • Avoiding Quality and Safety Failures in Commissioned Services

What are the recommendations arising from the report?
NOTE the performance issues reported as exceptions.
NOTE the quality issues highlighted.
NOTE the current cluster financial position as at Month 04.
NOTE the progress of the four-level data framework to support the strengthening of the Contract Review Meeting cycle.

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
The Devon and Cornwall and Isles of Scilly Finance, Performance and Quality Committees – 10 September 2026 South West Peninsula Management Group – 16 September 2026

What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	The performance of NHS Integrated Care Boards (ICBs) directly shapes the health and well-being of local communities, especially in addressing health inequalities and disparities
Quality and Safety	The report outlines key areas that need system oversight and support and the summarised narrative highlights actions that can be taken to support the quality of services across the Devon and Cornwall systems.
Equality, Diversity and Inclusion	The report outlines key areas that need system oversight. An improvement of efficiency across the four key areas will need to ensure that it does not drive health

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	inequalities and appropriate EQIAs will support decision making.
Finances and Use of Resources	The report outlines key areas that need system oversight and support and the summarised narrative highlights actions that can be taken to support the financial position within Devon and Cornwall with the finite resources available.
Regulation and Legal Requirements	None
Information Governance (DP and Cyber)	None
Transformation and Innovation	None
Environment and Climate Change	None

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Performance Exception Report

Introduction and background

1. This report summarises the content and key actions being taken to deliver against the NHS Cornwall and Isles of Scilly (NHS CIOS) and NHS Devon Integrated Care Boards (ICB) operational priorities, constitutional standards, finance delivery plan and key quality markers.

2. The report includes system operational performance against the current metrics for the end of June 2026 / July 2026, within Urgent and Emergency care (UEC), Elective and Cancer care, Primary Care and Mental Health (MH) as articulated in NHS Oversight Framework (NOF) and the national constitutional standards where appropriate. Data is sourced from the NHS England planning-activity tool published through NHS Futures.

3. Summary level performance is shown in table 1 below. Red, Amber, Green rating is assessed against the annual operating plans, not overall level of performance. Table 1 reflects actuals and distance from plan based on the re-submitted planning submissions in July 2026.

Table 1 Summary of key measures against plan

	NHS CIOS		NHS Devon	
KPI	Actual	% Variance	Actual	% Variance
Total waiting list	63,968	(1.7)	146,449	(0.2)
RTT % Within 18 Weeks	72.8	0.3	64.9	(1.2)
% RTT > 52 Weeks	1.1	0.1	1.84	(0.2)
% > 6 Week Diagnostics	16.2	(7.0)	30.2	(6.5)
% FDS Cancer 28 Day	83.56	3.1	76	(5.3)
% Cancer 62 Days	74.6	(3.6)	64.7	(14.5)
% within 4 hour A&E Waits	-		-	
% within 12 hour A&E Waits	-		-	
Average Ambulance Handover	-		-	

	UHP		RDUH		TSD		RCHT	
KPI	Actual	% Variance	Actual	% Variance	Actual	% Variance	Actual	% Variance
Total waiting list	47,271	0.4	74,021	0.2	-	-	43,268	(2.0)
RTT % Within 18 Weeks	67.4	0.2	63.6	1.1	-	-	74.3	0.5
% RTT > 52 Weeks	1.5	(0.3)	2.1	0.1	-	-	1.07	0.0
% > 6 Week Diagnostics	19.9	(3.3)	38.3	(10.9)	25.4	2.3	18.7	(15.7)
% FDS Cancer 28 Day	86.26	6.1	80.4	(2.3)	57.1	(0.3)	82.8	2.3
% Cancer 62 Days	63.6	1.9	68.2	(6.1)	53.6	(29.1)	79.4	(2.6)
% within 4 hour A&E Waits	66.8	0.2	68.2	(0.8)	63.9	(11.1)	75	(8.5)
% within 12 hour A&E Waits	18.9	0.1	5.3	(1.7)	12.9	(7.7)	15	(6.0)
Average Ambulance Handover	01:41:30	(250.0)	00:27:51	(9.9)	00:46:50	(56.1)	01:03:32	(128.0)

UHP - University Hospitals Plymouth NHS Trust
RDUH - Royal Devon University Healthcare Foundation Trust
TSD - Torbay and South Devon Foundation Trust
RCHT - Royal Cornwall Hospitals Trust

RAG
On plan or better
Off plan, but within 2% of target
Off plan by more than 2%

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Operational Performance Overview

4. Narrative following outlines current position against trajectory for the key NHS Southwest Peninsula Board (SWPB) measures within the 2026 / 2027 Annual Operating Plan as at the July 2026 submission.

Urgent and Emergency Care (UEC)

5. July 2026 has remained broadly static with only some marginal improvement across key indicators compared to the June 2026 position. All measures are below NHS Devon and NHS Cornwall's planned positions for each provider except for UHP (University Hospitals Plymouth) which is better than their re-submitted trajectory for July 2026.

6. Across Devon and Cornwall, we continue to experience sustained challenges around patient flow, delayed discharges and high levels of hospital occupancy with even small improvements not being sustained for any significant period.

7. Royal Cornwall Hospitals Trust (RCHT) remain the strongest provider in terms of actual overall A&E 4-hour performance but are below plan alongside most of the acute trusts. The Trust, along with TSD (Torbay and South Devon), remain furthest behind plan at -8.5% and -11.1% respectively. UHP are the only provider on plan since their re-submission of their trajectories in July. All providers need to reach a minimum performance of 82% by March 2027; this is looking incredibly challenging for most Trusts based on the current levels of performance and lack of any significant improvement of late.

8. For type 1 performance, all providers are failing to meet the required standard set out in their plans except UHP.

9. Table 2 below shows the national benchmarked position for July 2026 (rankings). RCHT hold the best overall 4-hour position in the cluster, although their national position has deteriorated of late. Royal Devon University Healthcare Foundation Trust (RDUH) and TSD fare better with their 12-hour ranking, with UHP almost the worst in the country at 112th from 119 providers.

Table 2 National Rankings

July 2026 National Standings

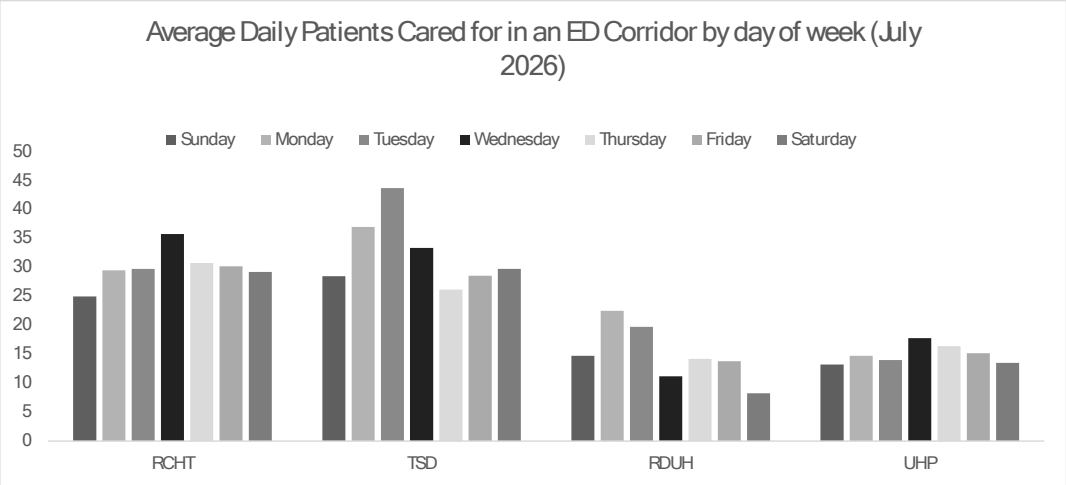
	% 4 Hour	% 12 Hour
RCHT	70th	102nd
UHP	116th	112th
RDUH	122nd	29th
TSD	127th	88th
Total Providers Reporting	139	119

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10. The newly published corridor care metrics shows patients receiving corridor care in Emergency Departments (EDs) in July, at snapshot times. TSD had an average of 32 patients and RCHT had an average of 30. Both UHP and RDUH were significantly under the national average of 19 with and average of 15 patients each.

11. Chart 2 shows the average daily patients cared for in an ED corridor setting by day of week. Peaks generally reflect patient flow and discharge constraints at the beginning of the week except at UHP which is consistent.

Chart 2 Average daily patients cared for in an ED, July 2026, by day of week.



12. Ambulance handover delays remain significantly challenged at most providers, with the only trust consistently delivering near-30-minute delays being RDUH. UHP remain the worst performer, only delivering an average of 01hrs 41mins against a plan of 29 minutes and a worsening position.

Actions to address - Cornwall

13. Recent work across the Urgent, Emergency and Integrated Care (UEIC) Programme, UEC Steering Board, Clinical Validation of Findings (CVoF) programme, Getting it Right First Time (GIRFT) review, winter planning preparations and ongoing pathway development has identified a number of priority areas requiring coordinated action ahead of winter 2026 / 2027.

14. Whilst admission avoidance remains an important priority, there is increasing recognition that improvement will require a balanced focus across:

- admission avoidance and alternatives to attendance;
- specialty streaming and internal acute flow;
- discharge and occupancy;
- community capacity;
- diagnostics and pathway enablers;
- escalation and operational grip

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15. The recent Getting It Right First Time (GIRFT) team visit identified opportunities relating to Urgent Treatment Centre (UTC)-manageable attendances, streaming and alternative pathways. Whilst these findings provide a useful challenge and support continued focus on admission avoidance and ensuring patients access the right service first time, the review was based on a partial, single-day snapshot and included a number of acknowledged caveats.

16. A number of priority themes have been established with clear questions that require system focus.

Referral to Treatment (RTT)

17. Based on the recent plan re-submission of the percentage with 18 weeks trajectory, NHS CIOS are on plan and NHS Devon behind. It should be noted that TSD have reporting issues due to the implementation of the electronic patient record EPIC.

18. In terms of total waiting list, Cornwall remain significantly above plan by over 1,000 patients. The high denominator will be supporting the percentage within 18 weeks position. Devon's overall wating list is also above plan.

19. Managing waiting list size alongside appropriate utilisation management and the optimisation of care pathways will be a focus as a strategic commissioner.

20. The over 52-week waiters remain significantly worse than plan for NHS Devon with over 2,000 people waiting longer than planned.

21. The table 4 below shows the national rankings for the two key RTT measures for June 2026. RCHT remains in the upper quartile for % within 18-week performance. RDUH remains the lowest ranking of our providers. The ranking of the over 52-week waiters has deteriorated for all acute providers.

June 2026 National Standings

	% 18 Weeks	% >52 Weeks
RCHT	32nd	81st
UHP	68th	102nd
RDUH	106th	123rd
TSD	75th	112th
Total Providers Reporting	150	150

Actions to address:

- Acute provider Contract Review Meeting process has been re-established to manage performance.
- Both Devon and Cornwall providers have well established outpatient transformation programmes.

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Diagnostics

22. The percentage of waiters within 6-weeks has not been met for either commissioner for first quarter of the year. Diagnostic activity levels are also below plan for June 2026.

23. At provider level, RCHT remain furthest behind % > 6 weeks plan with a variance of nearly -16%, closely followed by RDUH at -11% adrift. Key modalities with high volumes and long waiting times at cluster commissioner level include:

    - 2,600 (26%) patients waiting beyond 6 weeks for a NOUS (Non-Obstetrics Ultrasound) in Devon and 3,000 (35%) waiting for more than 6 weeks for an MRI (Magnetic Resonance Imaging).
    - 645 (23%) patients waiting beyond 6 weeks for an Echocardiology in Cornwall.

24. Overall, year to date diagnostic activity is on plan for NHS CIOS, but 5% (7,800) tests below plan for NHS Devon. However, whilst RDUH and TSD are closer to plan, they have significantly more patients waiting above 6 weeks. Table 5 shows the national rankings against the over 6-week diagnostic wait target. All out providers are in the lower half of the rankings. Most ICB's in the country are behind plan as of June 2026.

Table 5 National Rankings

June 2026 National Standings

	% > 6 Weeks
RCHT	84th
UHP	90th
RDUH	132nd
TSD	105th
Total Providers Reporting	153

Actions to address:

- Echo activity at RCHT significantly under plan with all activity now retrospectively recorded. Booking now being delivered in house to ensure capacity maximised. An ICB level strategy for echoes is being developed in order to maximise the capacity already within the system. There is additional capacity with the InHealth CDC, so we should not have any case of a patient waiting over 6 weeks
- Sleep studies saw further improvement in activity since purchase of additional machines, but still off target
- Need to ensure we maximise the independent sector Community Diagnostic Centre capacity (CDC) to support delivery of the standards. Action plan now in place to track and progress all actions.

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- Evidenced Based Design (EBD) survey has opened for all live Devon and CIOS sites. For Devon, only Torbay and Exeter will partake, once the target number of responses have been reached sites are open to bid for additional development funding.

Cancer

25. 62 days cancer target was missed this month by both NHS Devon and NHS Cornwall. The 28 day faster diagnostic target was also missed for both NHS Devon and NHS Cornwall in April 2026 and May 2026 but met in June at NHS CIOS.

26. Table 6 below shows the provider national standings as at June 2026. TSD are by far the worst performer in the cluster and are close to being the worse nationally. NHS Devon is the furthest behind plan in the country.

Table 6 National Rankings

June 2026 National Standings

	Cancer 28 Days	Cancer 62 Days
RCHT	39th	34th
UHP	16th	106th
RDUH	70th	87th
TSD	128th	127th
Total Providers Reporting	130	133

Actions to address:

- RDUH recovery plan shared with NHSE, issues relate to medical staffing and anaesthetic capacity.
- At Torbay and South Devon (TSD), Commissioners have continued to challenge to ensure a fully functioning tele dermatology service is live, action plan received with support from Peninsula Cancer Alliance.

Primary Care

27. All key primary care indicators are on plan as at June 2026. It should be noted that the data used to measure the percentage of patients identified by their practice as clinically urgent who are managed on the same day may have some data quality issue.

28. NHS CIOS closed River Dental Practice following a Quality Assurance visit in April 2026. A new provider for those in 'active treatment' is in place and contact with patients currently in active treatment is underway. To date, around 540 patients whose NHS orthodontic treatment was active at the time of the closure have been identified and transferred to the new provider.

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29. We are continuing to validate patient information to ensure that no one is missed. Patients are being prioritised according to clinical need. Urgent cases are already being addressed, and additional clinics are planned throughout September to support ongoing care. This has been a complex process because complete patient records were not available following the closure. As a result, the ICB has had to identify and validate patients using information from a number of different sources. NHS CIOS's immediate priority remains those patients whose treatment was interrupted by the closure. We are also developing arrangements for patients who had not yet started treatment and will provide further updates to affected families as soon as these arrangements are confirmed.

Mental Health

30. The following reported metrics for mental health align to the targets detailed within the 26/27 operating plan.

31. Inappropriate Out of Area placements for NHS Devon are 10 people against a target of 10 as at the end of July and for Cornwall and Isles of Scilly off trajectory by 3 placements with 26 people.

32. NHS COIS Talking Therapy outcomes for Reliable Improvement and Reliable Recovery are above target with data showing reliable improvement 70% against a target of 69% and 52% for reliable recovery against a target of 51%. In relation to completed course of treatment, providers are on track against the locally agreed target of 6,501. This is below the NHSE desired target.

33. For NHS Devon performance for reliable improvement and reliable recovery in June 2026 are currently below target with reliable recovery 48% against a target of 51% and reliable improvement 65% against a target of 69%. In relation to completed courses of treatment, NHS Devon is below target. The reported deterioration in completed courses of treatment, reliable recovery and improvement for Devon is in line with previously identified risks and forecasts. NHS Devon working collaboratively with providers to manage these challenges and deliver improvement actions aimed at strengthening performance over time.

34. June 2026 shows Average Length of Stay (ALOS) for Older Adult services in Devon has reduced from 90 days in April to 86 days, but is above the plan of 72 days. Average Length of Stay (ALOS) for Adult Acute and PICU services has increased from 53 days in March 2026 to 55 days in June 2026. The average length of stay in Adult Acute and PICU mental health beds in Cornwall and Isles of Scilly is 37 days.

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35. NHS Devon May 2026 data demonstrate continued positive performance in Individual Placement Support (IPS) and Perinatal Mental Health Services, both of which remain above their respective target positions.

36. Whilst not contained within the operating plan for 26/27, two other areas of achievement to highlight are in relation to access to transformed community mental health services and the national standard for Early Intervention in Psychosis (EIP) - both of which are monitored by NHS England (NHSE).

37. Access to transformed Community Mental Health Services in Devon remains significantly above target, with 22,595 people accessing services by May 2026 against a target of 8,700.

38. Access to Children and Adolescent Mental Health (CAMHs) services in Devon remains significantly above target, with 21,908 people accessing services by June 2025 against a target of 15,754.

39. Children with an eating disorder seen within 4 weeks (Routine access) has fallen July 2026 to 75% and is off target 95%.

40. Early intervention in Psychosis (EIP) performance in Devon continues to exceed the national standard, achieving 65% against the 60% target. Cornwall and Isles of Scilly is also above target.

Quality

41. Across the Devon and Cornwall cluster, the Quality Assurance Meetings (QAMs) maintain overall oversight across commissioned services with a joint Peninsula QAM that launched on 1st September 2026. Clinical Quality Review Meetings (CQRM's) in place in CIOS and the roll out of CQRMS in NHS Devon has commenced in August 2026.

42. In NHS CIOS improvements reported in several key areas including acute stroke care, fracture neck of femur outcomes, patient flow within urgent and emergency care, and reductions in historic harm review backlogs. A CQC inspection report for RCHT West Cornwall Hospital has been published rating the service as Good. Most independent sector and diagnostic providers reported no significant quality concerns. However, ongoing workforce shortages across acute, community, mental health and maternity services continue to impact performance and service delivery, requiring sustained monitoring and support.

43. Several services remain under enhanced quality oversight, including the system stroke pathway, Bodmin Renal Unit, mental health out-of-area placements, and Sowenna adolescent mental health services. Of particular concern, a recent CQC inspection of Cornwall Partnership Foundation Trust mental health services identified patient safety issues, resulting in a Section 29A Warning Notice and increased scrutiny of improvement actions. Maternity

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services continue to manage risks associated with neonatal and obstetric staffing pressures, while system partners remain focused on reducing corridor care, improving patient flow, and ensuring safe, high-quality care across all settings.

44. In NHS Devon improvements reported in several key areas including closure of the UHP Mortality QIG following sustained improvement to Summary Hospital-level Mortality Indicator (SHMI), continued implementation of CQRMs across Devon, and collaborative improvement activity across healthcare-associated infection and patient safety priorities.

45. Several services remain under enhanced quality oversight including system wide children's complex care pathway; system wide long waits for community paediatric assessments autism / neurodiversity pathways; system wide urgent and emergency care including corridor care; Langdon Hospital; fire safety risk at Torbay Hospital and EPIC implementation at UHP. Of particular concern, temporary maternity unit closure at North Devon District Hospital. Due to significant staffing pressures the decision was taken because safe staffing levels could not be consistently maintained and followed consideration of the associated clinical risks.

Finance

System Financial Performance - M4

46. The key system finance metrics for the Cornwall and Devon cluster are summarised in the table below which highlights the areas where organisations within the system are off plan.

M4 Financial Performance Metrics	DPT £m	CFT £m	RCHT £m	RDUH £m	TSD £m	UHP £m	CICB £m	DICB £m
YTD (M4) Finance variance	0.0	0.0	-0.4	0.0	0.0	0.0	0.0	0.0
FOT Finance variance	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
YTD CIP delivery variance	-0.1	0.5	-0.5	-0.5	0.0	-8.0	-1.9	1.9
CIP Unidentified	0.0	0.0	0.0	0.0	0.0	3.4	0.0	0.0

47. The Devon system financial plan was revised in Month 3 to align to a system-wide break-even trajectory, with consequential adjustments to both RDUH and UHP.
48. The Peninsula system is reporting a year-to-date deficit of **£35.5m**, broadly in line with plan, with a modest adverse variance of **£0.3m**, primarily driven by the position at RCHT.
49. Within this position, Devon is reporting a deficit of **£30.7m**, while Cornwall and the Isles of Scilly is reporting a deficit of **£4.8m**. Delivery requirements increase significantly in the second half of the year to achieve the planned break-even position.
50. Current forecasts indicate delivery of the agreed break-even plan; however, a number of material risks remain and the delivery of this is looking highly unlikely.

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51. The most significant risks relate to **UHP**, with an unmitigated net risk of approximately **£70m** (excluding any potential loss of Deficit Support Funding), and **RCHT**, with an unmitigated net risk of **£10m**.
52. Financial deep-dives are being undertaken with all organisations through the Financial Oversight Group to strengthen delivery plans ahead of the second-half regional assurance meetings.
53. A further risk remains to the cluster ICB position from unfunded primary care prescribing price concessions, currently estimated at **£12.0m**

ICB Financial Performance

54. The cluster ICB's are forecasting a combined breakeven position and have delivered to plan at month 4. The combined forecast net expenditure for the cluster is £5.0bn, which is set against a plan (resource limit) of £5.0bn, delivering a forecast breakeven position. The year-to-date position for the cluster ICB's similarly is breakeven in totality at this stage in the year, forecasting net expenditure of £1.7bn.

55. Whilst the ICB's have agreed a breakeven plan for 2026/27, this is in the context of a very challenging planning round where all NHS providers have also agreed breakeven plans (albeit with non-recurrent support), based on material levels of efficiencies and a significant level of risk.

56. The acute NHS contracts in Devon have moved to the national payment scheme this year, which adds volatility risk to the acute expenditure.

57. Whilst the performance management of the providers sits with NHSE under the ICB blueprint, the delivery of the contracted positions and performance is closely linked, and the assurances process put in place include:

    a. NHSE provider oversight meetings – work ongoing to ensure these are not duplicative and align with the other assurance forums locally.

    b. Financial Oversight Groups (FOG) – assurance on the financial aspects of the provider (and ICB) positions, with a view to maintaining momentum, and early warning and action on unwarranted variation.

    c. Contract Review Meetings (CRM) – newly refreshed assurance meetings to drive adherence to contract, address variation or contract underperformance through improvement and assure commissioned performance targets are robust.

    d. Investment panel – to extent existing arrangements with greater grip and control over any investment in excess of £50k.

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58. The ICBs at month 4 reported an unmitigated net financial risk of £14.0m (Devon £7m, CIOS £7m[1]) in relation to prescribing, mental health and continuing care. This is an increase of £3m from month 3 due to further indications that there is pressure in concessionary drug pricing for 'no cheaper stock obtainable' [NCSO].

59. The combined ICBs CIP efficiency programme totals £70.2m (excluding intra system NHS efficiencies contractualised by the ICBs but delivered by the providers). At month 4 the YTD position is on plan at £20.8m. The delay in the delivery of the phase 2 restructure has been offset by delivery of the pathway transformation work in NHS Devon (this is due to the profiling and not additional CIP above plan). The ICBs are forecast to deliver their plans by 31 March 2027.

Next Steps

60. A new four-level data framework is evolving, designed to ensure that performance information is presented in a structured, proportionate and actionable manner. The framework will enable users to progress from high-level strategic oversight through to detailed operational analysis, ensuring that attention is focused on those areas of greatest risk, variation or opportunity.

61. The diagram below illustrates each level of the data framework and has been specifically designed to:

- Reduce information overload.
- Focus attention on material risks and opportunities.
- Support early identification of variation.
- Provide a consistent evidence base for escalation and decision-making.
- Provide a consistent and trusted information source across commissioning, contracting, finance and business intelligence functions.

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[1] At month 4 CIOS reported an unmitigated net financial risk to NHSE of £4m, however after submission the net risk was increased to £7m due to the impact of no cheaper stock obtainable (NCSO) for drugs.



62. Development of the framework will be undertaken using a phased approach. Initial versions of Level 1 (System Oversight), draft shown below, and Level 4 (Detailed Diagnostic Analysis) are expected to be available by mid-September and will be presented to the Strategic Commissioning Hub for review and refinement. Subject to agreement, these components will be incorporated into the October Contract Review Meeting cycle, providing both a high-level view of contractual performance and the detailed analytical capability required to investigate emerging issues and support evidence-based decision making.

63. Subsequent development of Levels 2 and 3 will build on this foundation, introducing threshold-based prioritisation and enhanced diagnostic reporting to help identify areas requiring focused attention and further strengthen the intelligence available to commissioners and contract managers. As the framework matures, outputs from Levels 2 to 4 will be used to develop Key Lines of Enquiry (KLOEs) for individual providers, services or pathways, with clear ownership, governance routes, reporting arrangements and timescales for delivery. This will ensure that identified risks, variations and opportunities are translated into structured actions and monitored through the appropriate

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governance forums, providing clear accountability and a transparent route from intelligence to intervention.

		RCHT	RDUH	TSD	UHP	ICB Total
Activity	Daycase	volume variance + (%) (IAP v ACM)				
	OP First					
	OP FU					
	ELIP					
	A&E					
	Virtual Wards					
	UCR					
	SDEC					
	GP Referrals Growth	%				
Performance	RTT 18%	Performance variance against plan (OP Plan v NHSE Statistics)				
	RTT Waiting List					
	52 weeks					
	Cancer 28 FDS					
	Cancer 31day					
	Cancer 62 day					
	Diagnostics					
	A&E 4 hr					
Finance	Daycase	£ variance + (%) (IAP v ACM)				
	OP First					
	OP FU					
	ELIP					
	A&E					
	Drugs					
	Devices					
	Other					
	Total					
Contracting\DQ	Uncoded	Volume + (%)				
	Routine clockstops < 14 weeks					
	Multiple OP same patient same day					
Rating	Red	x/29 (%)				
	Amber					
	Green					

Thresholds	
On track	Between -x% and x% of plan
Over	>x% variation
Under	<-x% variation

Recommendations

60. The South West Peninsula Board is asked to:

- **NOTE** the performance issues reported as exceptions in the main report.
- **NOTE** the quality issues highlighted
- **NOTE** the current cluster financial position as at Month 04
- **NOTE** the progress of the four-level data framework to support the strengthening of the Contract Review Meeting cycle.

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NHS Cornwall and Isles of Scilly

NHS Devon

South West Peninsula Board meeting in public

Board Assurance Frameworks – NHS Cornwall and Isles of Scilly and NHS Devon Integrated Care Boards

Date of meeting	Date report produced
24 September 2026	17 September 2026

Author(s)		Report approved by	
Name and title:	Molly Marcu, Chief Governance Officer	**Name and title:**	N/A

Executive summary
Further to the July Board update this report provides an overview of both Devon and Cornwall ICB's Board Assurance Framework. This iteration of the report highlights two updates in relation to the finance and performance risk position as well as an overall update on progression with the development of the document. The Board is asked to note and receive the updated iteration of the Board Assurance Framework.

If this paper needs to be presented at a private meeting, please state why and mark as CONFIDENTIAL:
N/A

What is the purpose of the report?	
ASSURANCE	☐
DECISION	☒

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To which Board Assurance Framework risk does the report relate?
N/A

What are the recommendations arising from the report?
NOTE and **APPROVE** the proposed revisions to the Board Assurance Framework in relation to financial performance
NOTE and **APPROVE** the proposed revisions to the Board Assurance Framework in relation to operational performance
NOTE and **APPROVE** the proposed steps to enhance and further embed the Board Assurance Framework

Has the issue been considered by any relevant groups previously? If so, what were the outcomes of that consideration?
N/A

What are the implications of the matters covered in this report for the following areas?	
Area	
Residents and Communities	None from the report itself. The report contents reflect principal risks on residents and communities and their mitigations.
Quality and Safety	None from the report itself. The report contents reflect principal risks on quality and safety and their mitigations.
Equality, Diversity and Inclusion	None from the report itself. The report contents reflect principal risks on equality, diversity and inclusion and their mitigations.
Finances and Use of Resources	None from the report itself. The report contents reflect principal risks on finances and use of resources and their mitigations.
Regulation and Legal Requirements	None from the report itself. The report contents reflect principal risks on regulation and legal requirements and their mitigations.
Information Governance (DP and Cyber)	None from the report itself. The report contents reflect principal risks on information governance (DP and Cyber) and their mitigations.

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Transformation and Innovation	None from the report itself. The report contents reflect principal risks on transformation and innovation and their mitigations.
Environment and Climate Change	None from the report itself. The report contents reflect principal risks on environment and climate change and their mitigations.

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Board Assurance Frameworks – NHS Cornwall and Isles of Scilly and NHS Devon Integrated Care Boards

Introduction and Background

1. Integrated Care Boards (ICBs) are required to prepare public statements to confirm that they have done their reasonable best to maintain a sound system of internal control to manage the risks to achieving their objectives. This is achieved by the Chief Executive providing a signed Annual Governance Statement, which covers the risk management and review processes within the organisation. The evidence to support this Statement is supported by the Board Assurance Framework, referred to hereafter as the BAF.

2. The BAF is based upon the identification of an organisation's strategic objectives (in essence, its strategic aims and goals), the principal risks to delivering them, the key controls to minimise these risks, with the key assurances of these controls identified.

3. These are monitored by Executives to resolve issues or concerns and to improve control mechanisms. The BAF should be regularly considered by the Board to seek assurance that strategic risks are being managed appropriately.

BAF update overview

4. The document has been updated to reflect the actual and emerging risks in relation to the achievement of the financial and operational at system level. This follows on from the agreement to dissect the financial sustainability risk in order to refine and highlight the sources of the risk exposure in order to enable the robust identification, implementation and tracking of associated mitigations.

5. This also coincides with the identification of significant risk exposure in month 4 which are set out in the next section of this report.

6. The Board Assurance Framework will also be aligned with the goals identified within the business plan, pending the articulation of strategic goals by the Board at an imminent workshop in Quarter 3.

7. Ahead of this workshop, the People, Customer Experience and Equity Committee recommended the adoption of a strategic objective on population health, which will also aid the identification of strategic population health risks.

8. Lastly, work will be undertaken in the intervening period to identify quality risks, drawing from recent work carried out as part of the draft quality strategy which will be presented to the Board in a future meeting.

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Proposed amendments to existing BAF risks

9. The system finance report highlights significant net financial risk exposure for both Cornwall and Devon for the 2026/27 financial plan delivery.

10. The most significant risks relate to **University Hospitals Plymouth NHS Trust (UHP)**, with an unmitigated net risk of approximately **£70m** (excluding any potential loss of Deficit Support Funding), and **Royal Cornwall Hospitals NHS Trust (RCHT)**, with an unmitigated net risk of **£10m**.

11. At the time of writing further work is planned or underway to identify robust mitigations to address this gap, partly through the work of the System Financial Oversight Group and also operationally through the ICB Cluster Chief Finance Officer's oversight and assurance reporting to the Finance, Performance and Quality Committee as well as to the South West Peninsula Board.

12. Amongst other key actions, the immediate focus of the group is to carry out further deep dives of each Trust's cost improvement plan.

13. At the time of writing a deep dive had been carried out on 16 September 2026 for UHP, which has the most significant CIP gap of £70m of unmitigated risk.

14. Further detail on potential and actual mitigations is set out in the system financial report as part of the Board 's agenda.

15. Based on this information and the interdependency between the CIP, deficit support funding and financial plan delivery, it is proposed that the sustainability risk is replaced by the following risks:

System financial risks

16. ***Failure to identify and implement robust cost improvement plans by providers and the ICB in Devon and COIS resulting in the potential loss of deficit support funding and non-delivery of the 2026/27 system financial plan***
    - *Initial risk score (20)*
    - *Current risk score (20*)

17. ***Failure to achieve break even by the end of 2028/2029 due to an inability to implement effective system transformation and delivery of recurrent CIPs.***
    - *Initial risk score (20)*
    - *Current risk score (20)*

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ICB financial risks

18. ***Failure by Devon and Cornwall &Isles of Scilly ICB to deliver the 2026/27 financial plan, and the statutory duty to breakeven) due to a shortfall in the cost improvement plan and mitigations to emerging risks***
    - *Initial risk score (16)*
    - *Current risk score (16*)

Performance risks

19. As reported previously in the July BAF update, a piece of work has been undertaken to highlight the areas of significant risk exposure in relation to performance against the constitutional standards. These areas are also consistent with the performance exception report, which also features on this agenda.

20. ***Adverse patient experience, exposure to patient harm and regulatory intervention due to material lapses in urgent and emergency care performance in Devon and COIS.***
    - *Initial risk score (20)*
    - *Current risk score (20)*

21. ***Adverse patient experience, exposure to patient harm and regulatory intervention due to material lapses and long waits associated with RTT performance in Devon and COIS***

22. Further work is required in relation to demand management in order to mitigate these risks effectively.

23. ***Risk of patient harm and adverse patient experience due to prolonged poor performance on the 62-day cancer target***
    - *Initial risk score (20)*
    - *Current risk score (20)*

June 2026 National Standings

	Cancer 28 Days	Cancer 62 Days
RCHT	39th	34th
UHP	16th	106th
RDUH	70th	87th
TSD	128th	127th
Total Providers Reporting	130	133

24. The data above shows significant improvements are required in the system in relation to the cancer 62-day target and that this is an area of significant risk, with TSD in the bottom rankings nationally.

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Current Risk Position Summary– NHS CIOS AND DEVON

25. Detailed information for NHS CIOS and NHS Devon respectively is provided at **Annex A** and **Annex B**, to this report.

26. The table below presents a high-level summary of the current risk position across the nine strategic risks for NHS CIOS for the current financial year:

Risk Theme	Board Committee	Executive Lead	Inherent Risk Score	Latest Residual Risk Score	Tolerance Level (Residual Risk Score)
Safety and Quality System Failure in Commissioned Services	CIOS FPQ	CCO	16	9	6 - 10 (Safety) 11 - 15 (Quality)
Workforce Sustainability and Resilience	SWP PCEEC	CPO	20	16	11 – 15
Failure to Reduce Health Inequalities	SWP PCEEC	CPHO	20	16	11 – 15
Digital, Data and Analytics Capability* (Incl. Cyber Security and Information Governance)	CIOS ARC	CPHO	20	16	6 – 10
Provider and System Alignment	SWP PC	CPTO	20	16	11 – 15
Reconfiguration and Service Change	SWP NHIC	CSCO	20	16	16 – 20
Progress in and focus on Cornwall	CIOS FPQ	CCO	16	12	16 – 20

Board Committee Abbreviations:	
CIOS FPQ	*Cornwall Finance, Performance and Quality Committee*
Devon FPQ	*Devon Finance, Performance and Quality Committee*
SWP PCEEC	*South West Peninsula People, Customer Experience and Equity Committee*
CIOS ARC	*NHS Cornwall and Isles of Scilly Audit and Risk*
SWP PC	*South West Peninsula Planning Committee*
SWP NHIC	*South West Peninsula Neighbourhood Health Improvement Committee*

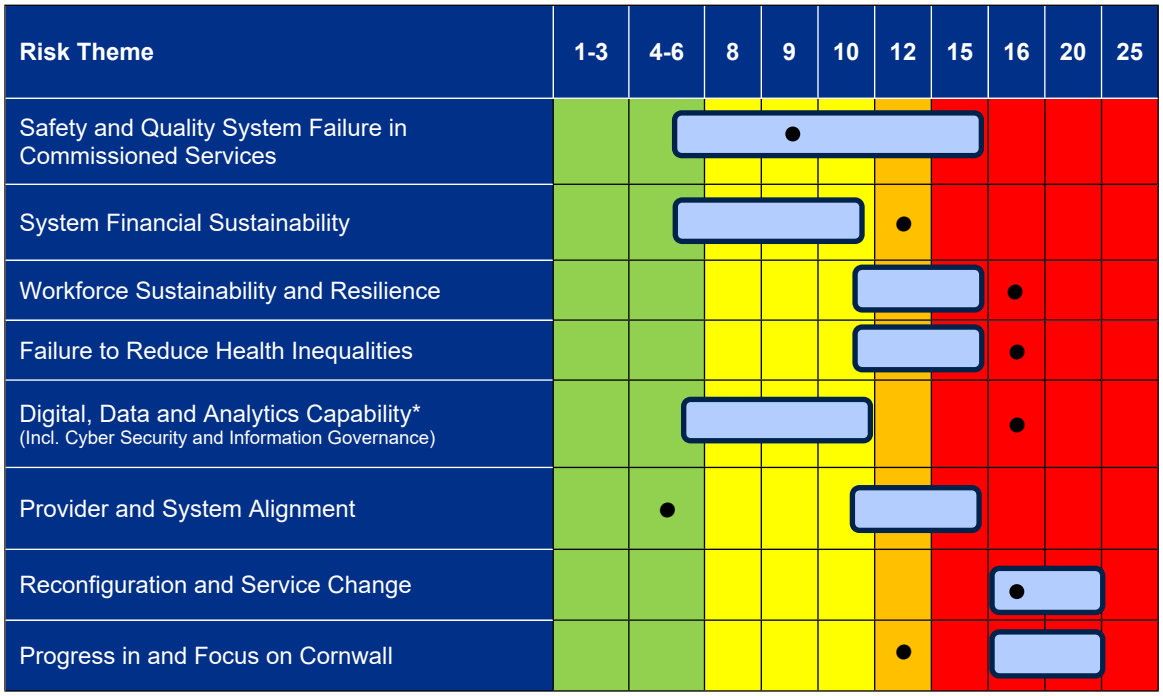
Executive Lead Abbreviations:	
CCO	*Chief Clinical Officer (Chief Nursing Officer)*
CFO	*Chief Finance Officer*

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CPO	*Chief People Officer*
CPTO	*Chief Place and Transformation Officer*
CPHO	*Chief Population Health Officer*
CSCO	*Chief Strategic Planning & Commissioning Officer*

27. The heatmap below provides a comprehensive overview of the likelihood and impact associated with NHS CIOS's strategic risks, illustrating the current overall risk profile. Of the nine BAF risks, based on their residual risk scores, four are assessed as exceeding the agreed risk tolerance level, four are considered to be within or below tolerance, and one is yet to be scored.

NHS CIOS Board Assurance Framework – Status



Current Risk Position Summary – NHS Devon

28. Detailed information for each BAF risk is provided at **Annexes A& B**, to this report.

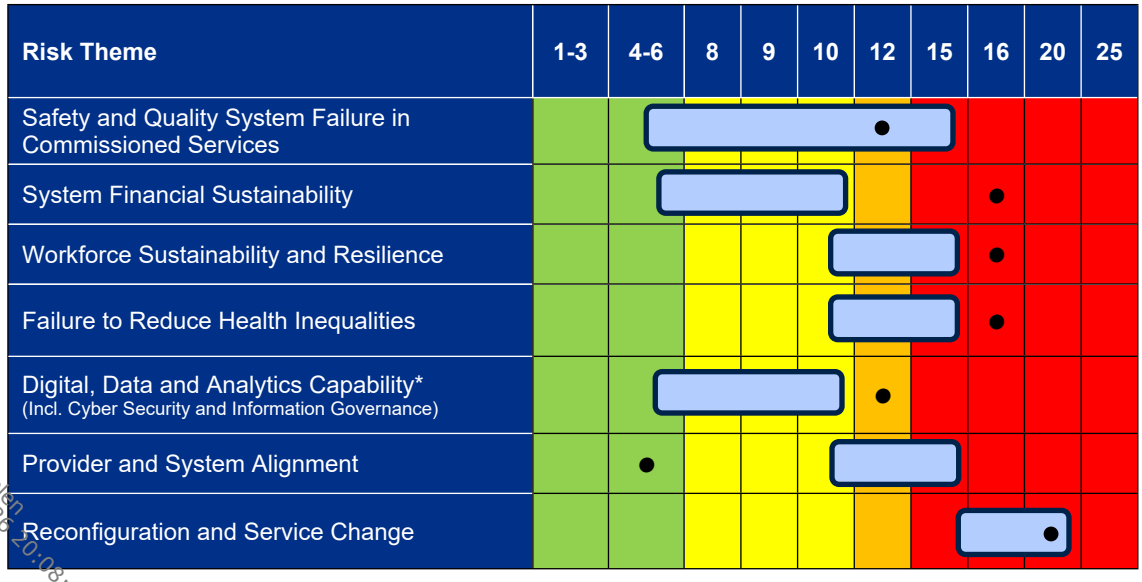
29. The table below presents a high-level summary of the current risk position across the eight strategic risks for NHS Devon for the current financial year:

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Risk Theme	Board Committee	Executive Lead	Inherent Risk Score	Latest Residual Risk Score	Risk Appetite	Tolerance Level (Residual Risk Score)
Safety and Quality System Failure in Commissioned Services	Devon FPQ	CCO	16	12	Low (Safety) Moderate (Quality)	6 - 10 (Safety) 11 - 15 (Quality)
Workforce Sustainability and Resilience	SWP PCEEC	CPO	20	16	Moderate	11 – 15
Failure to Reduce Health Inequalities	SWP PCEEC	CPHO	20	16	Moderate	11 – 15
Digital, Data and Analytics Capability* (Incl. Cyber Security and Information Governance)	NHS Devon ARC	CPHO	12	12	Low	6 – 10
Provider and System Alignment	SWP PC	CPTO	20	16	Moderate	11 – 15
Reconfiguration and Service Change	SWP NHIC	CSCO	25	20	High	16 – 20

30. The heatmap below provides a comprehensive overview of the likelihood and impact associated with NHS Devon's strategic risks, illustrating the current overall risk profile. Of the eight BAF risks, based on their residual risk scores, four are assessed as exceeding the agreed risk tolerance level, three are considered to be within or below tolerance, and one is yet to be scored.

NHS Devon Board Assurance Framework Heatmap – Status



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Recommendations

31. The South West Peninsula Board is asked to:

- **Note and approve** the proposed revisions to the Board Assurance Framework in relation to financial performance
- **Note and approve** the proposed revisions to the Board Assurance Framework in relation to operational performance
- **Note and approve** the proposed steps to enhance and further embed the Board Assurance Framework

Appendices

Annex A: NHS CIOS BAF 2026/27
Annex B: NHS Devon BAF 2026/27

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